IOWA DEPARTMENT OF HUMAN SERVICES

DIVISION OF BEHAVIORAL, DEVELOPMENTAL, AND PROTECTIVE SERVICES FOR ADULTS, CHILDREN AND FAMILIES



CHILD AND FAMILY SERVICES REVIEW
IOWA STATEWIDE ASSESSMENT

Submitted to:
U.S. Department of Health and Human Services

March 2003

Child and Family Service Review
Iowa Self-Assessment
Executive Summary
March 2003

The pages that follow contain an aggressive and complete review of Iowa's child welfare system. This is a self-evaluation, which is the first step in the federal government's new method of reviewing the performance of a state's child welfare system. The final step is a program improvement plan to be agreed upon in the summer of 2003.

There are several caveats to keep in mind as you read this document:

- This appraisal is unvarnished. There are parts of Iowa's child welfare system that do not meet expectations, and the Department of Human Services makes no effort to conceal them. There are areas of success. We acknowledge them with pride, but in the same sentence we insist on improvements.

- The DHS welcomes this evaluation process. We make difficult decisions for at-risk children and we provide services the best we know how, but we must have results. If children are not safe, if they don't have permanent families to grow up in, then we have not succeeded.

- Child protection is the work of a community. The Iowa DHS is a central player in helping at-risk children, and the DHS coordinated this report. But the DHS cannot do this alone. We are in direct and daily partnership with courts, medical staff, prosecutors, providers, communities and parents, whose efforts are essential.

- Iowa's performance in child welfare has been stunted by budget cuts during times of additional demand. These have resulted in cuts both to critical early intervention and treatment services as well as reductions in casework staff. Resources provide time to do the work. Resources – used resourcefully – are essential. Without resources, positive results are impossible.

For many, the main question answered by this document is whether Iowa meets new – and very demanding – federal standards. The answer is, some are met, some are not, and some are pending further review. By definition, all states must be "in substantial compliance" with all standards in order to meet the new threshold. None has done so, nor will Iowa. Thus, the DHS will work with partners in Iowa and with federal officials to prepare and implement a program improvement plan in the summer of 2003.

A word about how we have come to this point. In March 2000, the U.S. Department of Health and Human Services issued new regulations for the evaluation of child welfare programs that receive federal funding. Traditionally, reviews were focused primarily on whether a state complied with procedural requirements, such as case file documentation. Now, states are evaluated on the results of services, plus capacity to create positive outcomes.

The first step for each state is a self-evaluation, which is this document. It is the result of a year of data collection, surveys, intensive reviews of specific cases, and focus groups and interviews with judges, juvenile court officers, foster families, providers, advocates, parents, and others. A preliminary draft was made public January 7, 2003. Iowans and federal reviewers suggested improvements to the draft, and many are reflected in this final version. The next step is an intense

weeklong evaluation by state and federal investigators at three sites in which about 50 random cases are examined. That part of the examination will take place in Polk, Woodbury, and Linn counties. It is scheduled for the week of May 19-23. The federal report is then issued, followed by the two-year program improvement plan. There are penalties for states that fail to follow their improvement plan.

Results of the Self-Evaluation

The evaluations are in two broad areas, outcomes and systemic factors. Outcomes measure the results in safety, permanency, and child and family well-being. Systemic factors measure the capacity and usefulness of the state's "infrastructure" that enable social workers, judges, and providers to do their jobs. The threshold is extremely high. For measurements of safety and stability, states must reach the 75th percentile (top 13 of all 50 states) on all six data indicators as reported in the Adoption and Foster Care Analysis and Reporting System.

Generally speaking, Iowa does well in systemic areas. In the outcomes area, some results are encouraging and some point to weaknesses.

Some outcomes use data indicators. Here is a summary:

- The national standard requires that fewer than 6.1 percent of children in the child welfare system experience a second confirmed report of neglect or abuse within six months of the first confirmed report of neglect or abuse. The Cedar Rapids area meets that standard, thanks in part to the new Community Partnership approach that surrounds at-risk families with resources. Five other areas (Ames, Charles City, Spencer, Ottumwa, and Waterloo) are starting this system. But statewide, Iowa's rate of 11.2 percent is not in substantial compliance.

- The national standard requires that fewer than 0.57 percent of children in foster care experience another maltreatment. For the time period of the review, Iowa's rate was 0.89 percent (8.9 children per 1,000 children in foster care).

- Most children taken from the home for safety reasons are reunited with parents after services are provided. The national standard requires that at least 76.2 percent of all these children be reunited with in a year. Iowa exceeds this level at 82.3 percent.

- Of children who cannot be reunited and are adopted, at least 32 percent must be adopted within two years. Iowa exceeds this standard at 48.8 percent. A record number of special-needs children were adopted (781) in FY02.

- Of children reunited with their families, no more than 8.6 percent may re-enter the foster care system within a year. Iowa's rate of 26.6 percent does not meet that standard. This shows that in many cases, the child welfare system needs to do more to equip families to handle the pressures of reunification.

- Of all children in the child welfare system in a 12-month period, at least 86.7 percent must have two or fewer placements. Iowa exceeds this rate at 88.4 percent.

In systemic areas:

- Iowa's statewide system of keeping track of at-risk children and their histories meets or exceeds federal requirements. It is assessable to appropriate staff, but it needs to be more user friendly.

- The case review system is strong and is getting stronger. There is a written case plan for every child in foster care. With the growing use of family team meetings, which take advantage of the strengths of a family and community, the plans are improving.
- Iowa meets the federal requirement for periodic review of every child in foster care. There is a strong partnership between the judiciary, the DHS, and the Foster Care Review Board.
- Iowa has a rich array of services, including strong prevention and permanency programs, although these have been impacted by recent budget cuts.
- Iowa has a statewide and focused plan to recruit and support adoptive and foster parents, although there remains a shortage of families willing to fill these roles.
- Iowa's system of quality assurance for children in foster placements does not meet our expectations. While Iowa is one of only two states to voluntarily conduct scores of intensive "look backs" at specific cases, we nevertheless do not have a comprehensive quality assurance program in place. Challenges include a diminished budget.
- DHS social worker training is excellent but limited. Budget cuts have forced sharp cuts in offerings.

Iowa's child welfare system has many outstanding strengths, chief among them being the dedication of staff, providers, and the judicial system. There are also obvious areas for improvements. We appreciate the input received after a draft of this document was made public in January 2003. We look forward to the remaining steps in the evaluation process – especially working with our partners to implement program improvements that help us get even better outcomes for the children and families we serve.

IOWA STATEWIDE ASSESSMENT

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STATEWIDE ASSESSMENT

Name of State Agency

State of Iowa
Department of Human Services
Division of Behavioral, Developmental and Protective
Services for Adults, Children and Families

Period Under Review: March 01, 2002 to May 19, 2003

Federal Fiscal Year for Onsite Review Sample: Federal Fiscal Year 2002
Period of AFCARS Data: March 01, 2002 to September 30, 2002
Period of NCANDS Data: March 01, 2002 to September 30, 2002

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OVERVIEW

It is the mission of the Department of Human Services (DHS) to meet the unique needs of individual Iowans and help them become healthy, safe, stable and self-sufficient. The success of this mission depends on our partnership with families, the federal, state, and local governments, as well as community providers. Working cooperatively with others, the child welfare system strives to produce positive results for over 20,000 children.

The Iowa Department of Human Services is experiencing some of the most difficult times we have experienced in decades. The difference between FY01 actual expenditures and FY 03 Child Welfare allocations is $27,538,012, which includes decreases in flexible funds through decategorization. This represents a 14% decrease in our child welfare budget. In addition, as of 12/31/02, Field Operations experienced a reduction of over $11 M, resulting in the elimination of 116 positions and holding another 135 positions vacant. Cost saving measures instituted during FY 02 have directly affected the availability and accessibility of department staff and services. Budget reductions have had the following affect:

- Major staff reductions and increasing caseload size
- The department was reorganized in response to decrease in resources, resulting in dislocation of staff and loss of quality assurance functions
- Thirty-four [34] local DHS offices were converted to less-than-full-time status
- The majority of flexible funding available for prevention programs and community grants was diverted to fund the delivery of basic child welfare services
- Waiting lists for group care were established in some areas - resulting in increased pressure on shelters and foster homes
- Seventeen [17] adult and child psychiatric treatment beds at the state mental health institutes were closed
- The capacity at the two juvenile facilities was reduced by twenty-two [22] beds
- Because of increased caseloads and reduced funding for purchasing child welfare services from private agencies, the department is significantly reducing the provision of voluntary child welfare except in situations in which there is an identified safety risk for the child

Over the same period of time, private providers also experienced a reduction in funds available to purchase services resulting in the closing of 78 programs including 157 group care beds - in the 18 months prior to July 2002. As of October 2002, private providers have laid off nearly 500 staff statewide.

Economic factors, which have forced a cutback in state revenue, are the same factors that have caused more and more Iowans to seek our services.

- Child abuse reports increased 15% in calendar year 2001- confirmed victims increased 22% and both these factors fueled increased demand for ongoing services to protect and treat child victims.
- Child in Need of Assistance [CINA] filings- the legal requests for Juvenile Court intervention for children experiencing child welfare problems in Iowa- increased by 30% in 2001; this increase also resulted in increased demand for child welfare services
- Juvenile Court filings related to delinquency allegations increased by 5% in 2001 which resulted in an increased demand for DHS funded services for delinquent youth
- Iowa experienced a 1.5% increase in the number of children in family foster care - resulting in an increase in child welfare expenditures

Because of limited funding, the legislature placed a cap on group care placements. The impact has been:

- A waiting list for residential treatment
- Development of alternative services to address the needs of children who are awaiting residential placement
- An increased focus on standardized criteria for determining the need for residential treatment.

Despite the limitations on residential foster care placements, Iowa still has one of the highest rates of placement of children under 12 in residential placements.

In addition to critical funding issues, emerging social issues are effecting the work that we do in the child welfare system. A sample of 770 children's cases indicated that in 42% of the cases, parental drug involvement contributed to involvement in the child welfare system.

Despite these challenges, department staff, private child welfare providers, and Juvenile Courts continue to strive to meet our goals of assuring safety, permanency, and well-being for the children we serve.

Child Welfare Partners

The Department of Human Services administers services statewide with a central administrative office in Des Moines, eight [8] Service Areas across the state, and local DHS offices in each of the 99 counties either full time or part time. While the Department of Human Services administers child welfare services, the child welfare system is comprised of key partnerships, without which we could not achieve safety, permanency and well-being for the children we serve. Our key statewide partners include, but are not limited to:

- Court Improvement Project
- County Attorneys and Guardian ad Litems
- Community Initiative for Native Children and Families
- Department of Public Health, Division of Substance Abuse
- Department of Education
- Faith community
- Foster and Adoptive Parents, Iowa Foster & Adoptive Parents Association
- Iowa Child Advocacy Board [Court Appointed Special Advocates & Iowa Foster Care Review Board]
- Iowa Coalition Against Domestic Violence
- Iowa Youth Connection Council
- Juvenile Court Judges
- Juvenile Court Officers
- Magellan Behavioral Health
- Prevent Child Abuse Iowa
- Schools of Social Work
- Service Providers and the Coalition for Family & Children's Services
- Youth Law Center

Funding reductions have necessitated staff reductions and restructuring that has had a significant impact on the effectiveness of the department in partnering with community stakeholders. These

reductions in turn impact the budgets of our partners by resulting in decreased referrals to providers and increased referrals to voluntary organizations. The impact noted in focus groups held by the DHS this spring and summer, included the following:

- Disruptions in relationships between the department and the community because of staff changes
- Elimination of the Human Service Area Administrator (HSAA) position has resulted in loss of the recognizable DHS presence in many communities
- Less time for department involvement in community planning and collaboration meetings
- Loss of knowledge of community services and informal support systems due to staff reassignments
- Some department staff have been reassigned to jobs they are unfamiliar with
- Increased caseloads due to reduced hiring/less staff and greater demands for services
- Increasing pressure to make rapid decisions in response to the budget reductions has at times resulted in the department having to make decisions independently, without the time for dialogue with external stakeholders that could result in greater community understanding of the reasons for specific departmental decisions

Field Service Areas

Iowa is a state-administered child welfare system. Child welfare funds are allocated to Service Area Managers (SAMs) in 8 field service areas for results based accountability. Field service areas deliver services to individuals and families as defined by policy within a single statutory and regulatory framework.

State Offices: DHS state office is comprised of six divisions, and Field Operations. Each division, service area and unit is under the supervision of a deputy director or the director. The divisions are:

Division of Behavioral, Developmental and Protective Services for Families, Adults and Children (DBDPS) – The Division is responsible for providing leadership, collaborating with state level partners, developing policy and budget, and setting performance standards in the following program areas.

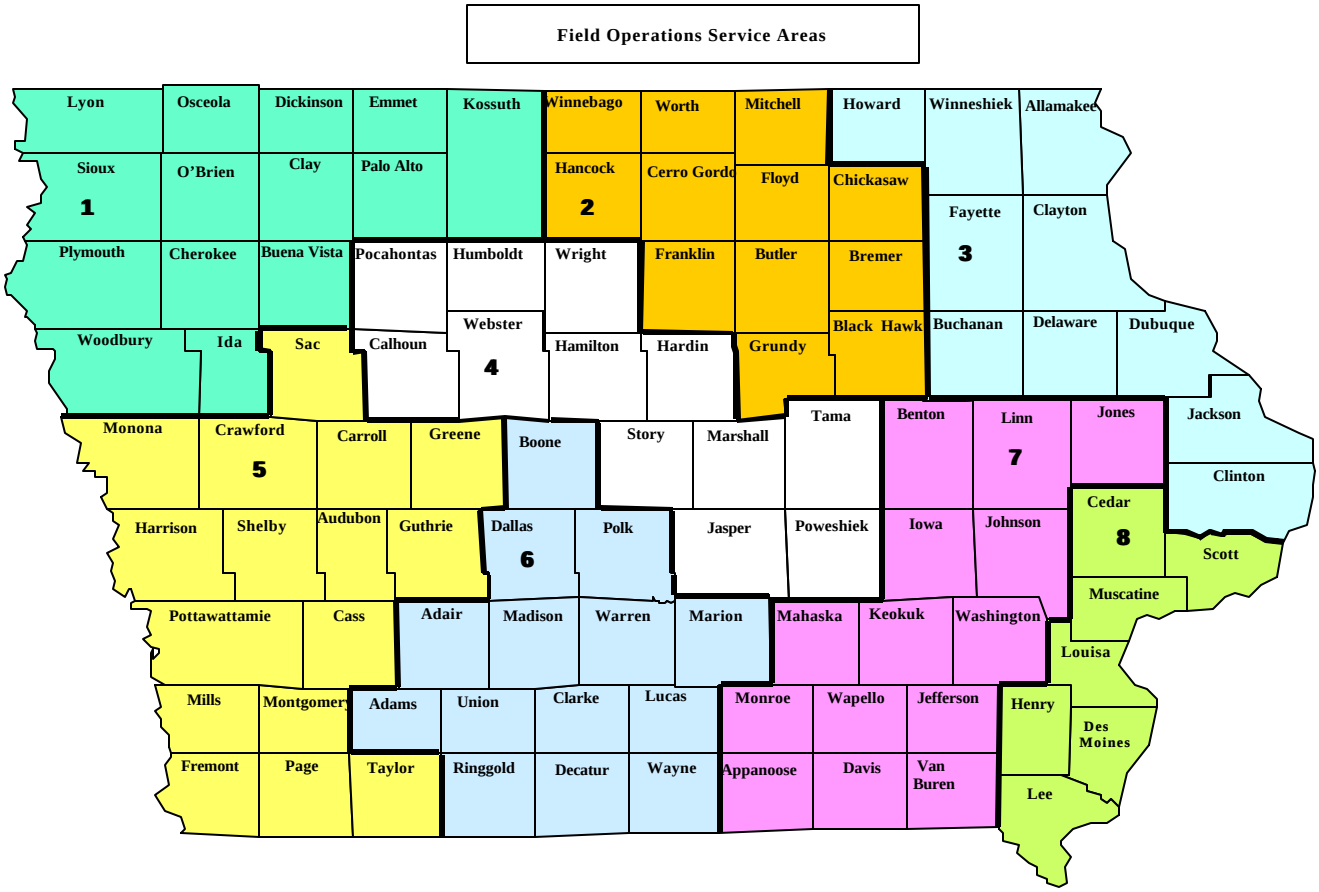
- Child welfare
- Juvenile justice
- Services to children and adults with developmental disabilities
- Services to children and adults with mental health issues
- Child care programs

DBDPS is responsible for the following state plans.

- IV-B Child and Family Services Plan
- IV-E Foster Care and Adoption
- Child Care
- Mental Health Block Grant

DBDPS also operates the child abuse registry, and administers the Interstate Compact on the Placement of Children.

DBDPS is the lead division on the Child and Family Service Review.



Field Operations – A continuum of Child Protective and child welfare services are provided through DHS offices located in each of Iowa's 99 counties. These local offices are grouped into 8 service areas, each one headed by a Service Area Manager (SAM). These SAMs are responsible for the development and maintenance of community partnerships as well as the delivery of services. DHS is an umbrella agency and the SAMs are also responsible for the provision of Food Stamps, Medicaid and Temporary Assistance to Needy Families (TANF)/Family Investment Program (FIP) benefits.

- Field Operations Support Unit (FOSU)—FOSU is a centralized unit that provides overall support to Field Operations/the 8 service areas, including operational budget management, customer services, policy clarification, case/clinical consultation, staff training, performance monitoring, quality assurance technical assistance, etc.

Division of Results Based Accountability (RBA) –responsible for providing leadership and technical assistance to DHS to facilitate the transition to a performance based organization. This includes providing tools, information and expertise to assure that results are measurable and tracked.

Division of Fiscal Management –provides financial services and fiscal administrative support including the development and expenditure of the DHS programmatic and operational budget.

Division of Data Management –provides automated systems to support the delivery of services, the payment of bills and the collection of required data elements. This division also provides technology services to all DHS staff and customers.

Division of Financial, Health and Work Supports -provides economic and health assistance to individuals and families to enable them to become self-sufficient stable and maintain their health. Economic assistance programs include TANF, childcare assistance and food stamps. Health assistance includes Medicaid and the state's Children's Health Insurance Program (CHIP).

Division of Child Support, Refugee Services, and Targeted Case Management –3 bureaus provide services directed at improving self-sufficiency for target consumers who are owed child support, are refugees, or are eligible for target case management. The Child Support Bureau is also responsible for foster care recovery.

Bureau of Managed Care and Clinical Services and Bureau of Long Term Care - responsible for program related policies for Medicaid including managed care.

Facilities - 4 Mental Health Institutes provide services to persons with mental illness and substance abuse problems. 2 State Resource Centers provide services to persons with mental retardation and developmental disabilities. 2 juvenile facilities serve youth adjudicated as Child In Need of Assistance (CINA) or delinquent. The Civil Commitment Unit for Sexual Offenders provides treatment for persons committed as sexually violent predators.

Child Welfare Practice

The department directly provides child welfare case management and child protection assessment. Services provided by the DHS child welfare case managers include working with families to assess risk and treatment needs and to develop a comprehensive case plan, purchasing needed services and monitoring to ensure desired results are achieved. In addition, DHS case

managers place children in foster care, provide crisis interventions, supervision, treatment, follow-up and monitoring of case activities. All other services are purchased from private providers or provided by other community entities.

Service supervisors in Field Operations play a variety of critical roles in the child protection and child welfare systems.

- Provide on-the-job training to help staff to develop needed competencies
- Review and approve decisions related to acceptance of child protective referrals
- Review and approve child protective assessment to ensure children are safe and that the reports are accurate and timely
- Review and approve case permanency plans and case work decisions to ensure children are safe, the plans are built on a strength-based assessment and the plan will facilitate the achievement of desired outcomes
- Provide clinical consultation to staff
- Make guardianship decisions for children under DHS guardianship
- Help workers balance and prioritize workload demands
- Perform personnel functions including employee performance evaluations, disciplinary actions, etc.
- Establish strong working relationships with community partners

Through a 28E Agreement, the Judicial Department, Juvenile Court Services manages cases for children adjudicated delinquent. The agreement requires compliance with federal requirements and the same reporting criteria for juveniles in foster care and those receiving DHS funded in-home services.

Private providers, through purchase of service agreements with the department, provide services to families referred by the department. The services they provide include family preservation, family centered services, family foster care, therapeutic foster care treatment, and group care. The department contracts with Iowa Foundation for Medical Care (IFMC) to determine service necessity for RTSS treatment services in accordance with the Early Periodic Screening, Diagnostic and Treatment (EPSDT) Medicaid Program.

The department has partnered with the Chief Juvenile Court Officers of Iowa's eight Judicial Districts to secure legislative funding for several specific services designed to promote community-based interventions with juvenile offenders. Funding for these programs - monitoring and outreach, life skills development, supervised community treatment, and school-based supervision - is contained in the department's appropriation but allocated by the department on a formula basis to the eight Judicial Districts to support these programs.

Child Welfare Policy

Child welfare policy is defined by the Code of Iowa, Iowa Administrative Rules, and Iowa Department of Human Services Employees Manual. Child welfare practice policy can be found in **Title 18** Child Welfare **Chapter A** Child Welfare General Requirements. Administrative Rules and the Employees Manual are available to the public on line at http://www.dhs.state.ia.us/policyanalysis/.

Safety, Permanency, and Well-Being

Child welfare services in Iowa are focused on the outcomes of safety, permanency, and well-being for children and families. The department's goal is to assist children and their families to effectively achieve and maintain real change that will ensure their safety and well-being.

The safety of children is the paramount concern that guides all child welfare services. Focus on child safety begins at the first contact the family has with the department and continues during the entire case process – from assessment through service provision, placement, and permanency planning decisions.

Iowa's statutes strongly promote a permanent home for children who cannot return safely to their own homes. Statutes establish time frames for making permanency-planning decisions, and for initiating proceedings to terminate parental rights.

Permanency planning is required at every level of DHS intervention. When placement of the child is considered, the department must:

- Prevent unnecessary out-of-home placements
- Facilitate the quickest safe return of the child to the home when placement is necessary
- Arrange for the most permanent alternative home for the child without delay when timely reunification is not possible
- Provide ongoing case review and oversight

According to Iowa law, efforts to place a child for adoption or with a guardian can be made concurrently with reasonable efforts to reunite a family to expedite permanency for children.

The Iowa child welfare system focuses on results and accountability**.**

SYSTEMIC FACTORS

Statewide Information System Capacity

One

1. Discuss how effectively the State is able to meet the State plan requirement that it operates a statewide information system that can determine the status, demographics, location, and goals for all children in foster care in the State. In responding, consider the accessibility of this information to State managers and local staff and the usefulness of the information in carrying out the agency's responsibilities.

Description

Iowa has a fully operational, statewide SACWIS that is able to meet the state plan requirement that it operates a statewide information system that can determine the status, demographic, location, and goals for all children in foster care in the state. Iowa's SACWIS has been in statewide operation since July 1995. Further, Iowa has undergone two federal SACWIS compliance reviews: an initial review in August 1998, and a follow-up review in July 2000. . Iowa is in the final stages of federal review for SACWIS acceptance. The system design has been reviewed and has been deemed to meet the requirements.

Iowa's SACWIS consists of two main components, Family and Children's Services (FACS) and Statewide Tracking and Reporting (STAR). FACS is the child welfare case management and payment system for the department. It is responsible for tracking the services provided to approximately 12,000 children at any specific point in time and automates issuance of over $220 million annually to foster and adoptive parents and other child welfare providers. FACS also serves as the data source for information used by field budget staff. STAR is responsible for tracking the intake, assessment and findings for over 25,000 child abuse investigations annually.

These two mainframe systems share a common platform (CV) with separate menus for specific child welfare and child protective screens. The system design supports the capability to share common records as well as a single database record shared by both systems. Additionally, the database design supports direct access to specific screens from one system to the other, making the two systems seamless to users.

CWIS receives daily and monthly response time reports for each system. The response time measured at the mainframe usually averages $10/100^{th}$ of a second. Local offices access the SACWIS through DHS's WAN, with connectivity occurring via T1 lines. Response time at local offices differ due to a number of variables including network load, switch and /or router performance and local phone company T1 line performance.

Iowa's SACWIS:

- Is available at all DHS locations to every DHS staff person needing access Monday through Friday from 5:30 A.M. to 7:30 P.M. and system maintenance and batch processing activities are done overnight and on weekends
- Contains a highly discreet security protocol which controls view and update access down to specific individual screens for each worker

- Supports inclusion of information about juveniles case managed by Juvenile Court Officer (JCO) under the Judicial Branch (DHS workers enter information as Juvenile Court System does not have direct access)
- Is used for tracking in routine case management activities by line staff
- Is used by managers to monitor caseloads and budget
- Provides standardized performance reports at the state and service area level for monitoring of the Federal Child Welfare Outcomes and state identified performance measures
- Provides standardized and ad-hoc reporting for key foster care and adoption data

For children in foster care the following data is collected at a minimum:

- Client demographics
- Worker information
- Household composition
- Service(s) provided
- Invoices for services submitted, invoices approved and/or paid
- Funding sources
- Current status and status history
- Location
- Case permanency goals

For individuals receiving child protective services, data collected in the STAR system includes:

- Time and nature of the alleged abuse
- Decision to accept or reject
- Reporter, if known
- Alleged perpetrator
- Worker, if case is referred for assessment
- Dates of parental notifications
- Appeals data, if any
- Final disposition of assessment including any referrals for services
- Timeframes for completing steps in a child abuse assessment to ensure compliance with mandated state and federal timeframes

Iowa's SACWIS generates over one hundred reports providing a variety of information which is routinely used in case management activities by line staff and by managers. As of January 2002, ten staff members were designated as Quality Assurance Management Analysts for field staff support. One person in each of the eight service areas and two centralized staff have been designated as the link between data findings and practice. Reports from SACWIS are used by staff to analyze practice patterns and inform practice change.

Statewide Assessment

In a recent DHS staff survey conducted to gather information for the statewide assessment, workers were asked to respond on a scale of 1-5, with 1 being "strongly disagree" and 5 being "strongly agree", to how well they agree with the following statements pertaining to the statewide information systems.

1.) The FACS information system allows me to identify, track and report information on children and families.

2.) The STAR information system allows me to identify, track and report information on child abuse assessments.

The average response fell in the neutral area, between "somewhat effective" and "somewhat ineffective" for both, with the response to the STAR system being somewhat higher than for FACS.

Participants in DHS staff focus groups indicated the following:

- That we do well at tracking things involving payment issues but not as well at accessing information on which to base case decisions
- There is plenty of information but it is not readily available in a useful format
- Line workers would like more case specific information
- Administrators would like more localized information on service customers and trends
- The FACS system is antiquated, difficult to learn and difficult to change
- The priority setting process for changes to the system is slow and is a barrier to getting data
- That lack of access to our system by the Juvenile Court Officers is detrimental

Staff dedicated to the information system has been cut by 40% in the last year and one half. Funding for information system improvement and change has been non-existent in the past several fiscal years. Maintenance of the existing system has been a struggle. In response to areas of the information system that staff have indicated as problematic, DHS has developed a prioritized list of Field requested changes to the CWIS System. This list provides an estimate of the hours and dollars needed for each of the 20 changes. Examples of requests follow:

- Split the IVE screens to eliminate delay in entering foster care placement information
- Review all generated reports and alerts and stopping production of those not currently used
- Programming to allow for direct deposit of foster/adoptive parents checks
- Purging and expunging old data
- Automate correction of bad data

It is evident that our SACWIS system needs to be more "user friendly." Of the two SACWIS components, FACS was the first to be developed. Issues we are aware of regarding user friendliness are two fold:

1) Navigation through the system in a mainframe environment does not offer most of the user supports that are available in a client server or web environment. To address this concern, we designed STAR to incorporate an intake process that more logically leads the worker through the entries necessary to start the child abuse assessment process. Additionally, for the last three years it has been our plan to develop task specific GUI front ends to several of the common child welfare processes that would incorporate such client server or web features as imbedded HELP text, links to policy manual, the ability for staff to create notes, etc. We know performing this enhancement would greatly improve the understanding of and ease of use of our current systems. Unfortunately, ongoing budget reductions have prevented implementation of this initiative.
2) The design of FACS and STAR both incorporate the use of a large number of codes for specific field entry. In the original SACWIS design, look-up tables were created only for screens and fields for which a worker had the ability to make an entry. Therefore, if workers had view-only access, they could not access the HELP text for a field. In the past two years the programming standards have been changed to require the creation of HELP text for any field using codes. We are also going back and retrofitting view-only fields to allow HELP

text look-ups. Again, due to budget restraints, the process of going back through our systems has been slow. It is estimated that work has been completed on about 60% of the system.

Summary

Strengths

- Substantial progress toward achievement of federal certification of Iowa SACWIS
- Successfully submit federally-mandated AFCARS and NCANDS reports
- Successfully pass penalty inducing quality checks
- Completed detailed data mapping analysis of our AFCARS to promote accuracy in our information system
- Detailed mapping of NCANDS in process
- Client-server application , FACS+ Adhoc, that allows staff to query SACWIS data to "slice and dice" financial, service and demographic data
- Intranet data warehouse application which allows staff to drill into incidents of child abuse by location, type and victim demographics
- Prototype data warehouse application that contains data from a variety of DHS systems allowing workers to tie together the web of exposure clients have to DHS programs
- Staff in the Results Based Accountability Division are using a variety of SACWIS data to answer questions and to provide insight to a number of challenging issues
- Iowa is establishing statewide consistent tools for assessing risk and safety for clients. The data from these assessments is being collected within our IT system and will be utilized to identify and verify the most effective and efficient service "packages" for a specific type of assessment
- Implemented electronic invoicing with a subset of our largest child welfare providers, thereby providing more timely payments to providers and decreasing data entry work required of DHS staff

Opportunities for Improvement

- Iowa's information system need to become more "user friendly"
- A full scale review of the alert system needs to be made to assure that alerts are presented in a prioritized and useful manner
- Increased staffing at all levels. Focus group participants reminded us that the reliability of data within our system varies with the time caseworkers have available for updating. Staff at all levels need time to utilize information generated by the system and staff dedicated to working on the information system is essential to maintaining a useful and up-to-date system.
- Provide accessible, timely, and accurate data to child welfare partners
- Improve the consistency, description, and use of data
- Provide training for management and supervisory staff on the use of data

Case Review System

One

1. How effectively is the State able to meet the requirement that each child in care under the State's placement and care responsibility have a written case plan with all the required elements?

Description

Iowa is able to effectively meet the requirement that each child in foster care have a written case plan with all the required elements.

Case Plan policy is specified in Iowa Code Section 232.2: 441 IAC 130.7(234), 185.5(1) "c" and 202l15(234) states that to be acceptable, the case plan must:

- Be based on the functional assessment - a strength based functional assessment guide is available to workers to assist with the assessment interview. Newly implemented case plan training will address integration of assessment information into case plan development, implementation and follow-up
- Reflect the views and preferences of the family
- Be directed toward the achievement of strategic goals and success of the family to live safely
- Be coherent in design
- Be achievable
- Be prudent in the use of family and professional resources
- Be culturally appropriate
- Be modified frequently, based on changing circumstances, experience gained, and progress made

The individualized case permanency plan is an integrated service plan that:

- Focuses on immediate safety for the child and family as well as the long-term view addressing permanency and self-sufficiency
- Unifies cross agency efforts
- Identifies an array of supports and services necessary to fulfill the permanency plan requirements
- Incorporates all of the federal case plan requirements, including:
  - A summary of how the plan protects the child's health and safety
  - Plans for carrying out the court order placing the child in care
  - The type and appropriateness of the placement and services for the child
  - The care and services to be provided to the child, biological parents, and foster parents
  - How the care and services will meet the child's needs while in care and will facilitate the child's return home or other permanent placement
  - A summary of the child's available health and education records
  - The plan for independent living services when a child is 16 or older
  - Specific actions expected of the parents, guardian, or custodian for DHS to recommend that the court terminate the child's out-of-home placement and end DHS involvement with the family
  - A description of the reasonable efforts made to prevent placement
  - The child's permanency planning goal
  - The projected date the child's permanency planning goal will be achieved

- An explanation why this placement is in the child's best interests if a child has been placed a substantial distance from the parent's home or in a different state
- A summary of efforts to find an appropriate relative placement if the child is placed in foster care documentation that the placement is the least restrictive, closest to the child's home and school consistent with the child's best interest; and includes the child's health and educational records
- Is signed by team members participating in its development
- Is completed within 60 days of placement and is reviewed every six months, or more frequently if the strategies are not effective or have achieved the intended result
- Is submitted to the court

Statewide Assessment

Data collected through focus groups, surveys, and interviews found that most in the following groups of respondents indicated that children in foster care have a written case plan with all the required elements.

- Judges
- DHS Service Area Managers
- DHS supervisors
- DHS casework staff
- Iowa Citizen Foster Care Review Board representatives
- Court Appointed Special Advocates
- Consumers
- Foster Families
- Private Providers

However, there was general agreement from the same respondents that the case plan is too complicated and not family friendly or professional friendly. There was also general agreement that many case plans are not developed with the family. Due to high caseloads, it is difficult for case managers to spend the amount of time necessary to develop a well thought out plan. Respondents also felt that the plans tend to be deficit focused and designed to meet federal regulations rather than serve as a working document that provides a road map for families and professionals.

In response to the consistent evaluation of the present case plan as being too cumbersome and complicated, DHS implemented a new, simplified case plan for intact families in September 2002. Initial feedback from this new case plan has been very positive. The case plan for children in out-of-home placement is being revised based on the feedback on the intact family case plan. The new plan for children in out-of-home placement will use the same simplified family plan format, plus have a section for documenting compliance with all the federal requirements.

The majority of FTMs are coordinated, facilitated and funded by the department. DHS staff who coordinate FTMs include case aides, case managers, Decat coordinators, supervisors, and dedicated FTM facilitators. Average time of the FTM varies. In a survey areas reported meetings lasting from 20 minutes to 6 hours.

In a recent survey, staff identified barriers to holding FTM more frequently. Indicators related to time difficulties were most prevalent. These include: scheduling, staff time, lack of time, and the time intensive nature of FTMs. Other barriers were attitudes of staff and families, team members placing responsibility of "success" on the child, identification of positive supports, logistics,

juvenile court, family issues, resources, training for team and facilitator, and lack of key players attending.

Further study of the FTM process in Iowa has been recommended. The possibility of implementing the FTM process statewide will be addressed in the Program Improvement Plan (PIP).

The Court Improvement Project received an adoption opportunity grant to offer child welfare mediation. Mediation is presently offered in 6 metro sites. Cases in conflict are the primary referrals. Establishing and maintaining a neutral environment by the mediator has been successful in engaging families to participate in case planning.

Summary

Strengths
- Children in foster care have a written case plan with all the required elements
- Family Team Meetings are being used in many areas to develop individualized case plans, assuring involvement of the family and child, as well as other important team members
- Child Welfare mediators are assisting DHS by working with families to develop case plans
- An automated case plan face sheet has been developed and implemented
- A new and simplified case plan has been designed and implemented for intact families
- A new and simplified case plan has been developed for children in out-of-home placement

Opportunities for Improvement
- We need to complete implementation of the new case plan for out-of-home cases
- Assure that foster families receive available information, including case plans, regarding children in their care
- Need for involvement of parents and non-custodial parents in case planning
- Need to evaluate the effectiveness of Family Team Meetings and Mediation and explore possibility of implementing statewide

Two

2. How effectively is the State able to meet the case review system requirement that parents of children in foster care participate in developing the child's case plan? In responding, consider their participation in activities such as identifying strengths and needs, determining goals, requesting specific services and evaluating progress related to their children.

Description

DHS policy requires that the case permanency plan is to be developed by a team that includes the family and others identified by the family and the social work case manager. The team is to identify:
- Goals
- Strengths
- Needs
- Concerns
- Strategies
- Responsibilities

- Resources
- Schedules

for coordinated provision of assistance, supports, supervision, and services for the child and family to be successful and live safely without external supervision. The plan is to be developed at a family team decision-making meeting (FTDM), using a strengths-based approach and focusing on safety, permanency, and well being of the child and family.

Statewide Assessment

496 DHS staff were asked the following survey questions. The percentage of those responding to each of the 5 available responses is indicated.

- 65% of the respondents agreed or strongly agreed with the statement that family members are involved in case planning
- 60.5% agreed or strongly agreed that family members are engaged
- 66.2% agreed with the statement that people in my service area work effectively with families

23 Juvenile Court Services (JCS) staff responded to the following survey questions. The average response on a scale of 1 to 5, with 1 being strongly disagree and 5 being strongly agree, is indicated in bold print following the question.

Survey Questions	**.**	**JCS Survey**
Family members are involved in case planning for the cases I work.	**.**	**3.7**
Family members are engaged in services for the cases I work.	**.**	**3.9**

In a focus group conducted with members of the State Child Advocacy Board we were told that involvement of parents and foster parents in care planning varies based on worker experience level, attitudes and comfort level in involving these parties.

The Director of the Child Advocacy Board indicates that citizen volunteer reviewers report that parental involvement, other than simply signing the case plan, in development of the case plan is the exception and not the rule. According to him, this is even more so in JCS cases where volunteers report that the absence of a case permanency plan is not unusual.

Clearly, "family member involvement" may be perceived differently by those involved. This issue was highlighted in a previous Quality Service Review where it was found that in a number of cases reviewed, families reported that they were not involved in the case planning process, even when the family was present at the meeting(s) and signed the plan. The interpretation was that case plans were more or less presented to families for their agreement rather than developed with family input. It appeared that families went along with the plan because they felt there was no other choice. This speaks to the need to improve family engagement (building trust-based relationships) and the need to strengthen the functionality of the service planning process which includes the need to make the case plan more "user friendly" for families.

All but one of the participants in parents' focus group of five [5] stated that they had not been involved in development of their child's case plan. They had, in most cases, received the plan after it was developed. The one exception was a person who had been involved in a family team decision-making (FTDM) meeting. She indicated that this was a very positive experience. She

felt that she had been listened to and that her input had been used in developing the case plan for her child.

During the past three years, there has been an initiative to implement family team decision-making (FTDM) meetings as a means to engage families and involve them in the case planning process. There are parts of the state where FTDM is used routinely. However, in some counties or service areas very few family team discussions occur. At a minimum, families meet with their social worker, private provider or attorney to review the case plan, and sign the case plan as acknowledgement that it has been discussed with them. Some judges surveyed indicated that upon their inquiry with families, families report that they have not seen the case plan.

A variety of funding mechanisms has been used to assist in the implementation of family meetings. However, implementation of family meetings has been beyond the resources of many parts of our state. There are individual counties and areas where family team decision-making is enthusiastically utilized. Clark Community Partnership for Protecting Children has supported development and expansion of Family Team Decision-Making in Iowa sites.

Iowa has used Quality Service Reviews (QSR) as one way of reviewing how successful we are at engaging families in service planning. Over the last year, 160 cases were reviewed using the QSR protocol – although not a statistically valid sample, these case generally reflect the state as a whole. QSR findings indicated that 79% of the cases reviewed had at least minimally adequate child and family engagement, with 32% of cases having optimal status. In 16% of cases reviewed, child and family engagement was identified as an area for improvement. The QSR reviews identified workload as one of the barriers to family engagement in that high caseloads prevented workers from having time to develop trusting relationships with families. When families were successfully engaged, they reported feeling respected by DHS workers and generally very satisfied with the services they received. Another barrier to engagement identified by QSR were frequent changes in DHS and provider staff.

An American Humane Association (AHA) report to Iowa also recommends the FTDM to help families be more invested in in order to positively affect permanency. Often, as a result of participation in the QSR process, local offices will identify FTDM as an action step and add training for implementation of FTDM as well.

For the past five years, in addition to family team decision making, mediation has been offered in selected sites in the state, offering mediation on cases involved with the courts where conflict over the case plan has occurred.

The National Resource Center on Family Centered Practice at the University of Iowa evaluated the Iowa Mediation for Permanency Project. Results indicate:

- Cases move forward when parents have been included as partners
- Where mediation has been implemented, families are engaged
- 97% of the participants have been very satisfied with their participation and would utilize mediation again
- Relationships between the participants improved
- Use of mediation results in a more positive attitude toward families and a better collaborative practice environment

Summary

Strengths
- DHS and JCO staff support the idea that families should participate in the ongoing case planning process
- DHS and JCO workers report that in most cases the case plan is discussed with the parent and is signed prior to being submitted to court
- An initiative to implement Family Team Decision Making (FTDM) in case plan development began three years ago and the intention is to use FTDM statewide
- FY04 budget request includes increased dollars for FTDM training and implementation
- FTDM is emphasized in Iowa's Quality Service Review (QSR)
- Some local DHS offices have included QSR and training for implementation of FTDM in their action steps following participation in a QSR
- The Court Improvement Project under the Alternative Dispute Resolution grant offers both mediation and FTDM in selected sites in the state
- Clark Community Partnership Project sites have implemented FTDM
- Evaluation of the use of FTDM and mediation in Iowa indicates that a more positive attitude toward families and a better collaborative practice environment has resulted
- Include and treat families as equal planning team members

Opportunities for Improvement
- Provide more opportunities for staff to engage families, including use of FTDM, by reducing workload and/or increasing staffing levels
- Provide training to all staff on family engagement and FTDM
- Expand use of FTDM and mediation to increase family involvement in developing case plans

Three

3. Citing any data available to the State, discuss how effectively the State is meeting the requirement that the status of each child in foster care be reviewed periodically, i.e. at least every 6 months, by a court or by administrative review.

Description

Iowa meets the federal requirement for periodic review of each child in foster care. State policy requires that when a child is in foster care placement, a case review of the status of the child must be conducted no less frequently than once every six months. The review must determine:

- The safety of the child
- The continuing necessity for and appropriateness of the placement
- The extent of compliance with the case plan
- The extent of progress that has been made toward alleviating or mitigating the causes necessitating placement in foster care
- A likely date by which the child may be returned to and safely maintained in the home or placed for adoption or legal guardianship

There are three options for foster care case permanency review:

- Court hearing
- Foster care review board review
- DHS administrative review

Court Review: The review by the court is a formal hearing, in most cases, with appropriate notice to all interested parties. There are occasions, in some parts of the state, when a paper review by the court occurs because of uncontested agreement by the parties about the progress and goals of the case. In these cases, updated reports are submitted by the state agency staff, as well as service providers. Foster parents, parents, and others involved may submit reports as well.

State statute requires that a court hearing must be held at least every 12 months to review the status of the child's case permanency plan.

In the Court Review, DHS does not have its own attorney. The county attorney represents "the state." In most cases this works well but in some cases the county attorney may disagree with DHS which results in the county attorney representing his/her view of the "state's interest" rather than DHS's view, leaving DHS unrepresented.

Iowa Citizen's Foster Care Review Board (ICFCRB) Review: A non-judicial method of 6-month review is through use of the foster care review boards. The Iowa Legislature created the Iowa Citizen Foster Care Review Board in 1984 to ensure that planning for permanency would occur for all of Iowa's children. Three stated purposes of the foster care review board are:

- To review the cases of children in their own communities to assure that each child has a specific and appropriate goal for achieving a permanent home in a timely manner
- To establish a registry of placements of children in foster care to assist in the formulation of policy and the allocation of resources
- To prepare and disseminate foster care data with recommendations on how the foster care system can be improved

The ICFCRB process sets up local boards of citizen volunteers who review each foster care case in their area every six months. They invite involved parties to meet with the board to discuss the progress toward meeting the permanency goal for the child and any barriers they are experiencing in reaching the goal. Based on the information provided, the local board makes recommendations to DHS staff and the Juvenile Court Judge. These recommendations are advisory in nature, with DHS deciding whether to act on the recommendations and the judge having the final decision-making responsibility for the case. In its 2001 annual report, the Foster Care Review Board reports that when a Foster Care Review Board conducts reviews in a community, DHS does not conduct an administrative review. As illustrated in the following map, ICFCRB presently serves 53 of the 99 counties in Iowa. By the end of FY 2003, ICFCRB anticipates serving 56 counties. In FY 2002, ICFCRB conducted 3,931 reviews. ICFCRB projects to hold 4,262 reviews in FY 2003.

DHS Administrative Review: If neither a court or ICFCRB review is done, a DHS state administrative review occurs. However, it is infrequently needed due to the regularity of judicial hearings. DHS Administrative Reviews are used if the review timeframe of the court or ICFCRB will not meet the 6-month review requirement. Service areas determine the need for and are responsible for the scheduling of these reviews.

Each type of review must be open to the parents of the child, the foster parents, pre-adoptive parents, and relative caregivers of children in foster care, with notice and adequate opportunity to be heard.

Statewide Assessment

AFCARS data indicates that in 2002 an average of 92% of cases had timely reviews**.** The following chart shows % of reviews on time by reviewer:

% Reviews on Time	2002
DHS Administrative Foster Care Review	91%
Iowa Foster Care Review Board	93%
Court Review	92%
Average	**92%**

The state has had no penalties due to lack of timely hearings for more than three years.

There was a divergence of opinions by respondents to surveys, interviews, and focus groups regarding the effectiveness of the 6-month review efforts.

Court Review: Stakeholders involved in surveys, interviews and focus groups generally felt that court reviews are held in a timely fashion and are recognized as effective.

496 DHS staff were surveyed regarding their work with the Juvenile Court.

- 74% of the DHS staff view judges as supportive of their efforts to meet the needs of children served
- 86.8% reported that the court considers their recommendations
- 82.6% feel that the Guardian Ad Litem considers their recommendations
- 51.5% find the Guardian Ad Litem's reviews and recommendations effective
- 23.2% of DHS respondents find Court Appointed Special Advocate reviews and recommendations effective
- 83.7% of the respondents feel that the County Attorney considers their recommendations
- 54.3% of the DHS staff responded that they were able to get a case scheduled when needed
- 68.9% feel that court hearings are effective

23 JCS staff responded to the following survey questions. The average response on a scale of 1 to 5, with 1 being strongly disagree and 5 being strongly agree, is indicated in bold print following the question.

Survey Questions	.	JCS Survey
Judges are supportive of my efforts to meet the needs of the children I serve.	.	**4.3**
The court considers my recommendations when making decisions.	.	**4.5**
The Guardian Ad Litem (child's attorney) considers my recommendations when making decisions.	.	**4.2**
The County Attorney considers my recommendations when making decisions.	.	**4.5**
I am able to get a case scheduled for hearing when needed.		**4.0**
Court hearings are effective in achieving safety and permanency.	.	**3.9**

The 2000 Iowa Foster Family Satisfaction Study found foster parents' cumulative experience with Guardian Ad Litems (GALs) performance statewide as not overly positive. Foster parents were asked to rate how helpful the GAL had been to them in fulfilling their role as a foster parent, when taking into consideration their total experience with GALs. On a four-point scale, 67.3% of the respondents placed GAL's helpfulness on the bottom half of the scale. A significant amount, 38.4%, said they were not at all helpful. Generally, foster parents perceived that GALs more often than not attend hearings but rarely, if ever, obtain firsthand knowledge through interviews or visits to the foster home to obtain information.

In focus groups and interviews other comments were made:

- Individual judges won't terminate parental rights until an adoptive home has been found
- Some jurisdictions order long-term foster care rather than require more permanent options
- Some judges do not consider the recommendations of the state agency staff
- Some Judges allows continuances which increases the likelihood of missing the review or permanency deadline
- There is inconsistency in the availability and quality of representation for parents and children
- The effectiveness of a court review is impacted by:
  - -Which judge is hearing the case
  - -How long the judge has been involved with the case
  - -Whether the judge serves on a rotation system or is permanently assigned to the area

Practice varies as to frequency of review. Although state statute only requires a review hearing every 12 months, most judges set hearings every 6 months. In many cases where there is instability in the case, where it is requested by state agency, or there is high turnover of state agency staff, some judges will have review hearings as frequently as 1 and 3 months to assure safety and well-being of the children.

Judges participating in a focus group reported positive effects of having a 6-month review hearing rather than relying on ICFCRB or administrative reviews. They see the review as an opportunity to:

- Obtain current information
- Acknowledge positive changes
- Reinforce the work that has been done and that which is yet to be done
- Hold professionals accountable to timelines and responsibilities laid out in the case plan
- Assure that the families have an opportunity to identify if the services they are receiving are adequate or if there are other services they believe would be more effective

The judges indicated that effectiveness of the hearings was dependent on the judge's familiarity with the case and their responsibilities in juvenile cases, thoroughness of information supplied by the agency staff and private providers, and the availability of resources to assist families. They felt that judges who fill in or rotate into juvenile court infrequently might be less effective.

Based on best practices of the American Bar Association, information from the National Council of Juvenile and Family Court Judges and input from judges and DHS and JCS surveys and focus groups, one judge presiding over one case can best assure an effective hearing. In most of the populous counties of the state there is consistency in presiding juvenile judges. However, the rural areas must rely on an infrequent juvenile hearing schedule with judges who do not routinely

hear juvenile cases. With support from the Iowa Supreme Court, the Court Improvement Project (CIP) developed standardized training for all judges on the responsibilities of presiding over juvenile court cases. This training was provided to all judges in the state. An update training including changes in the juvenile code, clarifications of IV E eligibility and the final rule offered from the federal government and education on essential clinical issues such as attachment, mental health issues of children and visitation planning was conducted in December 2002.

In addition, there is annual attorney training sponsored by the Iowa Court Improvement Project (CIP) in collaboration with Drake Law School Legal Clinic and taught by Iowa judges. The training addresses the differing roles of prosecuting attorneys, Guardian Ad Litems (GALs), and parents' representatives, in the Juvenile Court arena; federal requirements of permanency; timeliness of hearings; and reasonable efforts. CIP will be initiating a taskforce to address quality of representation within the year.

ICFCRB Reviews: Stakeholders involved in surveys, focus groups and interviews found reviews performed by the Iowa Foster Care Review Board less effective than a court review.

496 DHS staff were surveyed regarding their experience with Citizen Foster Care Review. Only 9.6% of the DHS respondents reported that they find Citizen Foster Care Review Board reviews and recommendations effective.

23 JCS staff responded to a similar question. The average response on a scale of 1 to 5, with 1 being strongly disagree and 5 being strongly agree, was 1.8.

Although survey participants rated ICFCRB reviews as not very effective, the IFCRB indicates that is different than information collected through an anonymous survey conducted at the end of reviews. According to their records, 1200 surveys were received from DHS caseworkers. 60% said they received new information at the review that will assist them; 78% stated that the review helped them understand what steps each person or agency would need to take in order to achieve permanency and 84% reported that the review was helpful. The ICFCRB also notes that children, families, and foster parents surveyed also found the reviews beneficial. These differing results indicate a need for increased collaboration and information sharing in order to improve the case review system and make it useful to customers and service providers alike.

In many areas where ICFCRB local boards are active, there is likelihood of reviews both by the court and the local board. Participants in focus groups and/or interviews with DHS staff, judges and JCS staff often repeated the assessment that court hearings are definitive thus essential, while the local board reviews are not essential and redundant. This was also supported by an independent review done by the American Bar Association. With reduced budgets, less staff, and higher caseloads, the issue of duplicate reviews is of concern to many.

The Iowa Citizens Foster Care Review Board routinely solicits feedback from the following that attend Citizen Review Board meetings:

- Child's Attorney
- Parent's Attorney
- CASA
- DHS
- Guardian Ad Litem
- Juvenile Court Services
- Parent

- Foster Parent or Placement Representative
- Provider Agency
- Youth

Included in the ICFCRB 2001 Annual Report were the following questions:

1. Did this review provide new information that will assist you?
2. Did this review help your understanding of what steps each person or agency should take in order to achieve the permanency goal?
3. Do you feel this review was helpful?

In all categories, those responding yes to the above questions exceeded 60% except for Juvenile Court Services, which consistently answered in the 20% range. The report does not indicate the number of participants in the survey in each category or the total.

Feedback from focus groups and surveys of state agency staff, judges, Juvenile Court Services staff, and most other professionals respondent groups conducted as part of the CFSR statewide assessment, however, were that the reviews are ineffective and redundant. Special concerns expressed were:

- Time requirements to attend another meeting
- Making multiple copies of the case files
- The repetitive nature of the review when most judges do 6 month reviews
- Ineffective, uninformed, or unreasonable recommendations

DHS staff are encouraged to participate in foster care review activities and do so routinely. DHS and the ICFCRB have a good relationship. However, staff feel that in many cases the activities of the ICFCRB are duplicative of the Court Review.

DHS Administrative Review: In the DHS and JCS survey, staff found DHS Administrative Reviews to be less effective in helping them to achieve safety and permanency for their cases than a court review but more effective than IFCRB reviews.

Only 27.4% of DHS staff report DHS Administrative Reviews and recommendations effective in helping them achieve safety and permanency for cases.

On a scale of 1 to 5, with 1 being strongly disagree and 5 being strongly agree, 23 JCS staff responded to the same question.

Survey Question		**JCS Survey**
DHS Administrative (foster care) Reviews and recommendations are effective in helping me to achieve safety and permanency for my cases.	**-**	**2.0**

Summary

Strengths

- 92% of foster care cases meet the federal requirement for periodic review
- Utilization of Iowa Foster Care Review Board to meet periodic review requirements
- Court review process considered effective by most stakeholders

- Strengthened relationships between DHS and JCS staff and judges
- Strong Court Improvement Project which provides standardized training and updates for judges as well as annual training for attorneys
- Strong commitment by Juvenile Judges to their role in assuring safety and permanency for children in foster care
- Development of a communication plan between DHS, JCS, and judges for areas needing improvement. Quarterly meetings are being held with Judges, SAMs and BDPS represented.

Opportunities for Improvement

- Work with Juvenile Courts and Foster Care Review Boards to reduce duplication of periodic 6 month reviews in cases in which both bodies conduct a 6 month review; and increase effective use of Juvenile Courts, Foster Care Review Boards and caseworker's time
- Reduce duplication of effort in reviewing foster care cases
- Reduce use of rotating judge system in rural areas of the state
- Improve collaboration and information sharing with review partners

Four

4. Citing any data available to the State, discuss how the State meets the requirements that permanency hearings for children in foster care occur within prescribed time frames. Discuss the effectiveness of these hearings in promoting the timely and appropriate achievement of permanency goals for children.

Description

Iowa has policy and prescribed procedures to assure that permanency hearings for children in foster care occur within prescribed time frames. Iowa Code Section 232.104 on permanency hearings states:

> The time for the initial permanency hearing for a child subject to out of home placement shall be the earlier of the following:
>
> - For temporary removal order, for a child who was removed without a court order, or for an order entered for which the court has not waived reasonable efforts requirements, the permanency hearing shall be held within twelve months of the date the child was removed from the home
> - For an order entered for which the court has waived reasonable efforts requirements, the permanency hearing shall be held within thirty days of the date the requirements were waived. Chapter 232.102 [12] of the Iowa Code contains a list of aggravated circumstances

DHS has provided updated training to field staff regarding code changes and their responsibilities to assure timely hearings. The Child Welfare Information System (CWIS) has a flag to alert individual workers two months prior to the need to request a petition for a permanency hearing.

Statewide Assessment

We do not currently have the Information Technology staff capacity to monitor compliance with timelines for permanency hearing. However, respondents report that there is substantial progress in meeting timeliness requirements. DHS, JCS, and Judge focus group participants indicated that some county attorneys are unwilling to file the petition for a permanency hearing.

The Supreme Court of Iowa has put a priority on cases that involve custody and placement of children.

- District resources are being used to move juvenile cases forward in a timely manner
- Training is being provided by the Court Improvement Project (CIP) to judges who do not regularly preside over juvenile cases
- An updated Juvenile Court Benchbook CD with the current juvenile code has been developed and distributed all judges and county attorneys
- Training and a resource manual have been provided to Juvenile Court Officers

The Court Improvement Project presented identified delays in resolving appeals of terminations of parental rights decisions as a significant barrier to achieving permanency for children in foster care. As a result, they developed recommendations for rules of appellate procedure changes to the Supreme Court, which were approved and implemented in January 2002. There has been a substantial reduction in the average time from time of appeal to decision, initially averaging 367 days to less than 90 days. 90% of the cases utilizing the new procedure were settled in an average of 47 days.

Participants in focus groups, surveys, and interviews identified an emerging best practice that has been credited with much of the improvement for timely permanency hearing. Setting the date of the permanency hearing at the end of the dispositional or six month review hearing and including that date and time in the court order effectively assures that permanency hearings are held within the 12 month time frame required by law.

Judges rated very high the effectiveness of the review to move children to permanency. Their responses were quite similar to the six-month review process. They feel the hearings are effective in keeping everyone:

- On track
- Mindful of the permanency goal
- Evaluating the reasonable efforts
- Informed of the responsibilities of all parties

The judges expressed concern that the 12-month timeframe was detrimental to families who might be making good progress, but where the primary issue was substance abuse of one of the primary caretakers/parents.

In a recent random sample review of 100 out-of-home placement cases, the most current cases reviewed were timely. Cases that originated 3+ years ago were far less likely to be in compliance. In addition, ICFCRB reports substantial improvement in permanency for the cases they reviewed.

Summary

Strengths

- Court Improvement Services has developed several monthly reports and is working on developing a tracking mechanism for permanency hearings
- Increase in Termination of Parental Rights (TPR) filings
- Increases in adoptions and guardianships
- CWIS system flag to alert workers of need to request permanency hearing
- Supreme Court priority placed on juvenile out-of-home cases and cases on appeal

- Judge, county attorney, and JCO training provided by CIP
- Updated Benchmark CDs provided to judges, county attorneys and JCOs
- Emerging best practice to schedule permanency hearing at the end of the dispositional or 6 month review hearing
- DHS caseworkers provided update training
- Rules developed, presented and recommended to the Iowa Supreme Court were approved and implemented resulting in substantial reduction in average time from time of appeal to decision

Opportunities for Improvement

- Continue training efforts
- Adopt best practice in all districts
- Need to develop capacity to monitor on-going compliance with requirement for permanency hearings
- Increase compliance rate
- Improve data availability to judges and county attorneys

Five

5. Citing any data available to the State, discuss how the State meets the requirement to provide foster parents, pre-adoptive parents, and relative caregivers of children in foster care with notice of and an opportunity to be heard in any review or hearing held with respect to the child in their care

Description

Iowa has policy and prescribed procedure providing caregivers of children in foster care with notice of and an opportunity to be heard in hearings held with respect to the child in their care. Iowa Code Section 232.88 Summons, notice, subpoenas, and service reads:

> "After a petition has been filed the court shall issue and serve summons, notice, subpoenas, and other process in the same manner as for adjudicatory hearings in cases of juvenile delinquency. Reasonable notice shall be provided to persons required to be provided notice under Section 232.37, except that notice shall be waived regarding a person who was notified of the adjudicatory hearing and who failed to appear. In addition, reasonable notice for any hearing under this division shall be provided to the agency, facility, institution, or person, including a foster parent, relative or other individual providing pre-adoptive care, with whom a child has been place."

DHS policy also directs staff to provide notice of administrative reviews conducted by DHS.

Iowa Code Section 237.20 Local board duties, requires that at least 10 days before the case review the ICFCRB provide notice to:

- The person, court, or agency responsible for the child
- The parent or parents of the child unless termination of parental rights has occurred pursuant to Section 232.117
- The foster care provider of the child
- The child receiving foster care if the child is fourteen years of age or older. The child shall be informed of the review's purpose and procedure, and of the right to have a guardian ad litem present

- The guardian ad litem of the foster child
- The department
- The county attorney
- The person providing services to the child or child's family

The notice shall include a statement that the person notified has the right to representation by counsel at the review.

Statewide Assessment

A 2000 Iowa Foster Family Satisfaction Study indicates that foster parents are not consistently receiving adequate notice of hearings and reviews across the state. Additionally, this survey indicated that many foster parents are not receiving a copy of the court order that indicates their responsibilities. However, in a focus group of foster parents held in late 2002, overall, the response was positive to being included in the hearing notice distribution and in having the opportunity to provide personal information and feedback to the court. This response may indicate an improvement in practice since 2000. Other things we heard from the focus group were:

- Some foster parents are concerned about appearing at hearings where parents may be present
- Some foster parents chose to submit reports or letters to the court rather than attend
- Some foster parents said they received notice, but they were not encouraged to actively participate, either by the case manager, the licensing agency, or the court personnel
- Some welcomed the opportunity to appear, especially when :
  - No CASA had been appointed
  - They felt the child's best interest was not being represented
  - The parent had intimidated them
  - When the foster parent and the parent had developed a positive working relationship

Summary

Strengths

- Iowa has policy and procedure for providing notice to alternative caretakers
- .Alternative caretakers view inclusion in the court process positively
- Alternative caretakers may submit written reports or letters to the court in lieu of appearing

Opportunities for Improvement

- Improved professional support of alternative caretakers' participation in hearings regarding children placed with them Provide training for foster parents and workers on the role of foster parents in the court system
- Assure that alternative caregivers consistently receive notice of court hearings and copies of court orders resulting from those hearings
- Implement measures to track compliance with notice of foster parent provision
- Implement measures to assess the quality, comprehensiveness and content of case plans

Quality Assurance System

One

1. Discuss how the State has complied with the requirement at section 471 (a)(22) of the Social Security Act to develop and implement standards to ensure that children in foster care placements are provided quality services that protect their health and safety, and any effects of implementing the standards to date.

Description

The Iowa Department of Human Services has implemented standards to ensure that children in foster care are provided with quality services that protect their health and safety. These standards are reflected and formalized through the State of Iowa Code, Iowa Administrative rules (IAC), Department's Employee Policy Manual, state handbooks for service providers and staff training. All department materials such as code sections, administrative rules, and employees manuals on child welfare, are available to employees and the general public online through the State of Iowa Website, www.dhs.state.ia.us.

DHS has established the following standards to ensure that children in foster care placements are provided with quality services that protect their health and safety.

- **Health assessment**. Administrative rules and employees' manual requires that workers assess the child's need for medical, psychiatric, and psychological services as part of determining the child's need for foster care placement. Manual requires that this include a medical exam either before the child is placed or within 14 days of placement. Manual also requires the worker to provide the foster care provider with the results of the physical examination and specific information about the child's medical needs and the child's immunization record.
- **Safety and risk assessment**. Employees' manual clearly states that child safety is paramount throughout the duration of the case, and that safety assessment must focus both on the current and future situation. Policy requires safety analysis at specific case milestones, including within 5 working days after case assignment, whenever circumstances suggest that the child's safety may be in jeopardy, when considering commencement of unsupervised visits, immediately before returning a child home, and immediately before closing a service case. Policy also provides guidance regarding risk factors that may place a child in immediate danger of moderate to severe harm. DHS is currently piloting the use of a standardized safety assessment tool and a risk assessment tool to strengthen the safety assessment and planning processes.
- **Placement selection**. Administrative rules and employees' manual require that workers select a specific foster care placement based on the best interests and special needs of the child.
- **Case plan focus on health and safety**. Administrative rules and employees' manual require that the caseworker develop a case plan within 60 days of a child's placement into foster care. The case plan must include a safety plan and a crisis plan, as well as services to address the child's physical and mental health needs. The plan must include the child's immunization record.
- **Foster care worker visits**. Administrative rules and employees' manual require that workers visit the child regularly to review the child's status and progress. If a child is placed in

family foster care and the placement is supervised by DHS, policy requires that visits be monthly, not to exceed 35 days (with some exceptions). If a child is placed in a foster family supervised by a private agency, visits must be at least every 60 days. If a child is placed in group care, visits must be at least every 90 days. These frequencies are based on the need to prioritize DHS caseworker workload and an assessment of what other supports and checks are in place to ensure child safety beyond the worker visit. In private agency foster homes, the private agency worker is visiting more often, so they are able to monitor child safety and provide support to the foster family. In group care, a number of different staff interact with the child and are available to monitor child safety.

- **Services for child**. DHS requires that the worker arrange for services for the child, based on the needs of the child.
  - Medical services, including annual medical examinations; annual dental, eye and ear examinations; routine treatment of any illnesses; immunizations against common contagious diseases; routine diagnostic laboratory procedures; and emergency medical care as needed. The worker is also required to give the foster care provider information about the plan for the child's medical care.
  - Mental health services, including testing, counseling and treatment based on the specific needs of the child.
- **Unauthorized absence of a child from placement**. DHS policy requires that the caseworker make a diligent search for any child who has runaway. The worker is also required to notify the child's parents, policy (or sheriff's office), the child's guardian, and the court within 12 hours of knowing that the child has run away.
- **Transporting a child**. DHS policy provides guidelines to ensure the child's safety in situations in which a worker is transporting a child.
- **Case Review**. DHS administrative rules and manual require that a case review must be conducted no less frequently than once every six months for children placed in foster care. The case review must address the safety of the child.
- **Licensing and Certification Standards**. DHS also has established licensing and certification standards for providers to ensure that children in foster care placements are provided with quality services that protect their health and safety can be found in policy
- **Child abuse investigation in foster care facilities**. DHS has also established specific standards for child abuse investigations involving allegations that a foster parent or facility staff have abused a child, including notice and coordination with the licensing authority.

Statewide Assessment

In calendar 2000, DHS solicited comments on the foster care standards in administrative rules from staff, provider agencies, state boards, interested individuals and other state agencies. Several comments suggested that there was some confusion between the rules that covered payment, licensing and services. Another person suggested that we should revise the standard on the frequency of caseworker visits to children in foster care and to foster parents; as well as clarify caseworker responsibilities towards foster families and towards children in out-of-home placements, especially concerning medical information. Another person recommended that we clarify caseworker responsibility to search for relatives as placement options. DHS did revised Employees Manual in 2001 to add clarity regarding expectations and procedures for seeking relative placements and to ensure placements with non-licensed relatives meet basic state safety standards. One responded commented that the rules were good as they are.

Stakeholders involved in surveys, interviews and focus groups generally felt children are safe and received adequate care. JCO, DHS, and FCRB staffs indicate that children's physical and mental

health needs are being met. However, a 2000 Iowa Foster Family Satisfaction Study - Retention Analysis indicated that foster families often do not receive medical and mental health information. This survey also indicated that there is a direct correlation between retention of foster parents and the caseworker home visit frequency.

496 DHS staff were surveyed regarding their practice.

- 92.3% of DHS respondents agreed that they regularly access the safety of children on their caseload
- More workers do not visit children at least once a month than do. Only 27% indicated that they visit face-to-face monthly
- Again, more workers do not visit children placed out of state once every 12 months than do. Only 37.2% indicated that they visit once every 12 months
- An overwhelming 92.6% of respondents repeated they know where all children on their caseload are. It should be noted that the survey did not give guidance as to how runaways should be dealt with. It is very possible that many of the disagree and neutral answers were given with runaway cases in mind
- 83.8% of DHS workers feel that the physical health needs of their clients are met
- Workers are less confident that dental health needs are met with only 61.9% agreeing or strongly agreeing
- Regarding mental health services, 65.4% reported that children receive services to meet their mental health needs
- Only 12.1% reported they had adequate time to complete their required duties

24 JCS staff responded to the following survey questions. The average response on a scale of 1 to 5, with 1 being strongly disagree and 5 being strongly agree, is indicated in bold print following the question.

Survey Questions	**. .**	**JCS Survey**
I assess the children's safety on a regular basis throughout the life of the case.	**.**	**4.5**
I visit face-to-face with children that I have placed in out-of-home care at least every month.	**.**	**2.7**
As their caseworker, I visit children placed out-of-state at least once every 12 months.	**.**	**4.2**
I know where all children on my caseload are.	**.**	**4.7***
Children receive services to meet their physical health needs.	**.**	**3.7**
Children receive services to meet their mental health needs.	**.**	**3.8**
I have adequate time to complete my required duties.	**.**	**3.3**

DHS policy requires that the caseworker make a diligent search for any child who has run away. The worker is also required to notify the child's parents, policy (or sheriff's office), the child's guardian, and the court within 12 hours of knowing that the child has run away

Summary

Strengths
- Iowa policy reflects a strong set of standards to ensure that children in foster care placements are provided quality services that protect their health and safety
- DHS is in the process of implementing more standardized risk and safety assessment tools for identifying and directing treatment of behavioral health needs

Opportunities for Improvement
- High caseloads present a challenge to our staff in terms of meeting state standards regarding visits with the child. This issue will be included in our PIP
- Frequent staff turnover within private child welfare providers is a challenge to maintaining high quality services
- Assure that foster parents are involved in planning for the health and safety of children in their care and that information is shared with them

Two

2. Discuss the effectiveness of the agency's quality assurance system in helping to ensure safety, permanency, and well-being for children served by the agency and their families in all jurisdictions of the State. In responding discuss the jurisdictions in the State covered by the quality assurance procedures, the capacity of system to evaluate the adequacy and quality of the State's child and family services system, and its capacity to produce information leading to program improvements.

Description

The department currently has no statewide comprehensive, coordinated quality assurance system that is designed to assess systemic factors and the effectiveness of the department's service programs. We recognize that this is a capacity that we need to more fully develop. Diminished budget resources, as well as the recent reorganization, have reduced our capacity for quality assurance and required conceptualization of new partnerships within the agency to build coordinated quality assurance capacity. However, the department has made positive strides to build this capacity by:

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- Developing discrete positions focused on quality assurance
- Continuing program quality assurance initiatives such as the Quality Service Review [QSR] process
- Moving forward with the child welfare utilization management project

Following is a description of the quality assurance components and activities that the department has in place to ensure safety, permanency, and well being for children served by the agency and their families.

Defining Outcomes. The department has established the following agency wide outcomes.
- ❑ Safe: Children and adults are safe in their homes and communities
- ❑ Stable: Children and families have consistency and continuity in their lives

- Healthy: Children are born and remain healthy and adults achieve and maintain good health
- Self-Sufficient: Children and adults have sufficient resources to meet and provide for basic needs

As part of Iowa's Accountable Government Act, the department develops an annual strategic plan focused on improving results for those we serve. Each operational unit (i.e., Division, Field Office, Institution) is also required to develop a set of strategies directed at the desired outcomes/results.

Practice Standards. The department established the following practice standards for child welfare in calendar year 2000, based on collaboration between the policy division, our local office field staff, providers and advocates. These standards are included in the employees' manual.

- Child and Family Engagement. Family members and substitute caregivers are active participants in the process by which service decisions are made. Parents and other caregivers are partners in planning, providing, and monitoring supports and services. The child actively participates in decisions made about the child's future.
- Service Team Functioning. The people who comprise the child and family service team collectively possess the technical skills, knowledge of the family, authority and access to the resources necessary to organize effective services for the child and family. The actions of this team reflect a pattern of effective teamwork.
- Functional Assessment. Existing assessments – both formal and informal – contain the current strengths, needs, and risks of the child and family. Assessments identify the critical, underlying issues that must be resolved for the child to live safely with his/her family, independent of outside supervision. All service team members use this information to have a 'big picture' understanding of the child and family.
- Long-Term View. The service team shares an explicit, guiding view for the child and family about what will enable them to live safely independent of outside supervision. The long-term view provides direction and support for making smooth transitions across settings, providers, and levels of service or care.
- Service Plan. The service plan is relevant to the child and family's needs and goals; modified promptly as goals are met and circumstances change; a coherent assembly of strategies, supports, services, and time-lines; a reflection of the choices of those who are expected to participate in and benefit from the services offered.
- Resource Availability. The array of home, school and community supports and services is adequate to assist the child and family to reach the level of functioning necessary for the child to make developmental and academic progress commensurate with the child's age and ability.
- Plan Implementation. The services and activities specified in the service plan for the child and family are implemented as planned, delivered in a timely manner, at an appropriate level of intensity.
- Family Support Network. The family is supported to perform essential parenting functions. The family is connected with informal supports to assist them to function independent of outside supervision.
- Services Coordination. There is effective coordination and continuity in the organization and provision of services. There is a single point of coordination and accountability to ensure that service plans are implemented, monitoring activities are conducted, and information is shared with service team members. Timely changes are made in strategies and supports across settings and providers.
- Successful Transitions. The service team plans with the child and family for the next major transition. This planning is consistent with the long-term view for the child and family.

- Tracking and Adaptation. The status of the child and family, service process and results are routinely followed and evaluated. Services are modified based on current knowledge to respond to the changing needs of the child and family.
- Effective Results. Planned education, therapies, services and supports result in improved functioning and achievement of desired outcomes for the child and family.

Organizational Support for Quality. As part of the January 2002 reorganization, the department established the following structure to support quality assurance and monitoring.

- Division of Behavioral, Developmental and Protective Services for Families, Adults and Children. The Division of Behavioral, Developmental and Protective Services for Families, Adults and Children is responsible for developing policy standards and setting outcomes/performance standards ("steering"), in collaboration with field staff and other stakeholders. The Division is also responsible for providing technical assistance related to evidence-based practice.
- Service Delivery Areas. The eight Service Area Managers and their staff are responsible for developing strategies to achieve the outcomes and performance standards ("rowing"). Each Service Area has a quality assurance staff position to serve as the point person for the SDA's quality assurance plan and activities. These staff are the link between data findings and practice, working between systems, people and practice to find ways to improve performance for the individual areas and for the entire state.
- Field Operation Support Unit (FOSU). The Field Operation Support Unit is responsible for providing support and technical assistance to line workers and supervisors through policy clarifications, case /clinical consultation, development of local procedures for implementing statewide policies, staff training, customer service-complaint resolution, etc. The FOSU also is responsible for developing performance measures and processes, working with service area staff to develop strategies for improvement of performance, sharing best practices from across the state, etc.
- Division of Results Based Accountability (RBA). The Division of Results Based Accountability is responsible for coordinating the department's strategic planning efforts. RBA also serves as a focal point for the departments efforts to achieve better outcomes for those we serve, and is responsible for developing mechanisms to better able to track and report outcomes. RBA is developing a "report card" that will report on key performance measures related to the department's 4 results areas. The RBA division works in partnership with state and local staff to develop baselines, objectives, strategic plans and outcome goals on a statewide basis.

Automated Information and Analysis of Quantitative Data. As noted in the section II-A, Statewide Information System Capacity, Iowa's SACWIS provides standardized performance reports at the state and service area level for monitoring of the federal child welfare outcomes and state identified performance measures; as well as standardized and ad-hoc reporting for key foster care and adoption data. The RBA Division is also developing the capacity to collect and report data from the standardized tools (safety, risk, and family needs) to improve the quality and use of services provided throughout the state.

The department currently has an on-going group of staff from the Division of RBA, the Division of BDPS, and the Field Office Support Unit who meet regularly to review and analyze the data on child welfare outcomes. The FOSU quality assurance staff person also works with the quality assurance staff in the Service Delivery Areas to link data and practice strategies to improve outcome performance.

The Division of BDPS has also provided data on the federal child welfare outcomes to Juvenile Judges relative to performance on the 6 federal outcomes broken out by the 8 Judicial Districts.

Case Reviews. Prior to budget reductions that necessitated the restructuring that occurred in January 2002, the department had staff in 5 regional offices that routinely read a sample of cases including child protective intakes, child protective assessments, intact family cases, and out-of-home placement cases. The 5 regional offices and these specialist positions were eliminated, however, in order to avoid having to eliminate positions of front line workers and supervisors. Currently, monitoring of compliance with safety related policies is largely the responsibility of the local service supervisors. Line service supervisors do review and approve all case plans. The department's Field Operations Support Unit is developing standardized reporting information to support this activity.

Beginning in calendar year 2000, the department also began using Quality Services Reviews (QSR) to review cases. Since January 2002, there have been 4 QSRs, 2 in Linn County which were part of the Clark Project for Protecting Children; 1 in Story County and 1 in the Ft. Dodge Area which were part of the statewide QSR process.

QSR is a structured, case review process - similar to the CFSR. QSR focuses on two areas:
- the status of the individual child and family related to safety, permanency and well-being; and
- the quality of system performance in terms of best practice principles (see Section E).

The process relies on a protocol that was adapted by Iowa practitioners from similar documents used in other states. Each review usually examines 10 cases. Each case is rated using 13 indicators for Child and Family status and 12 indicators for System Performance. The appraisal of the professional reviewers examining each case is translated into a score for each category of functioning using a 6-point scale.

Participation in QSR is voluntary. Since its inception, the number of counties/service areas requesting QSR has exceeded the state's ability to fund the process. Local administrators who request QSR do so to establish a baseline "look" at local practice and use this information to build and enhance partnerships among service providers and implement local practice development initiatives like the Community Partnership for Protecting Children. To date, the department has conducted eleven (11) reviews in 29 counties involving 160 cases. The department has not scheduled any QSR's during FY 2003, due to the need to devote the department's limited resources to preparation of the CFSR statewide assessment and onsite review in May.

Each review concludes with feedback to individual staff and to the local office on practice strengths and opportunities for improvement. The department uses information derived from QSR to support training, practice development and system change. The department has used QSR results in the following ways
- revised employee's manual to support the practice principles which are included in the protocol
- revised the training curriculum for new workers to expand training on engagement skills and the practice principles that form the basis of quality practice
- incorporated findings into supervisor training
- formed the foundation of new practice tools for caseworkers and supervisors – including a Discussion Guide for Reflective Practice for supervisors

- influenced practice change at the local level – e.g., implementation of family team decision making and safety assessment

Community Partnership for Protecting Children (CPPC) sites also use QSR to review practice and identify areas for change.

This year, we are using the dollar and staff resources that would normally go for QSRs for the CFSR. We will be developing an overall quality assurance system as part of our PIP.

As noted in the section on "Agency Responsiveness to Community", the Child Death Review Team also conducts case reviews, makes recommendations to agencies represented on the CDRT for changes in policy and practice that may prevent child deaths and makes recommendations to the Governor and General Assembly for interventions to prevent deaths of children in Iowa.

Consumer Input. The department assesses consumer satisfaction through the following processes.

- Family engagement is assessed as part of the QSR process
- The Iowa Foster and Adoptive Family Association conducts periodic satisfaction surveys with foster parents under contract with the department
- The department solicits youth input and feedback through the Iowa Youth Connections Council, the statewide organization of current and former foster care youth

The department is also exploring use of routine consumer satisfaction surveys.

Stakeholder and Staff Involvement. The department involves stakeholders and staff in the QSR process, both as peer reviewers, and through stakeholder and staff focus groups.

Statewide Assessment

Both internal and external participants in various focus groups were asked to comment on the department's quality assurance system. Many respondents noted that there appeared to be no established, coordinated methods by which the quality and effectiveness of child welfare services was evaluated and service quality improvement activities were undertaken. While several respondents were able to identify both internal and external activities or groups that assessed services, there were limited identified means and procedures through which the results of these activities were analyzed and recommendations for service improvements developed.

The department and JCS asked one question in the staff survey related to quality assurance. The percentage of those responding to each of the 5 available responses is indicated.

Survey Question

I use information available to me to improve results.				
Strongly disagree	Disagree	Neutral	Agree	Strongly agree
.6%	2.7%	14.8%	64.8%	17.2%

JCO response showed an average response of 4.4.

Summary

Strengths

- Quality Service Review (QSR) is used to assess service quality and to improve the quality of practice and outcomes for children and their families
- Data indicators of child welfare outcomes are used at all levels of the department, and are being incorporated in performance based contracting with service areas
- Department operational units are developing strategic plans to improve child welfare outcomes
- Stakeholders and partners are also using data on child welfare outcomes to guide practice strategies

Opportunities for Improvement

- We need to continue work on the development of our quality assurance system in order to more comprehensively and consistently focus on improved service delivery and improved results for families
- Utilization of foster parent feedback to direct best practice improvement and change
- Improve effort to include foster parents as part of the professional team

Staff and Provider Training

One

1. Citing any data available to the State on the numbers and timeframes of staff trained, discuss the effectiveness of the State's initial and ongoing training for all child welfare staff employed by the agency that includes the basic skills and knowledge required for their positions.

Description

Training Redesign – Competency Based Curriculum
The department undertook a training redesign project and implemented a new curriculum in state FY 01 for both the protective assessment worker and the ongoing social worker case manager. The training redesign committees interviewed urban/rural, experienced and new workers and supervisors throughout the state to determine the competencies that are necessary for workers to perform their positions and then validated these competencies against national listings. Using the identified competencies, a new competency based curriculum was designed, developed and implemented to provide a competency-based, job-oriented training. The competency-based training is sequenced for learners to build a foundation of knowledge that is built upon during the subsequent modules. The nine competency areas include:

- Child welfare practice values
- Legal aspects and IDHS policy
- Safety assessment and planning
- Assessment
- Planning for results
- Delivery of services
- Communication and documentation skills
- Use of technology to facilitate service provision
- Strategies to address personal stress and safety issues.

Contract Based Training
The department contracts with Iowa State University for the development and delivery of training for child protective assessment workers and social work case managers. Face to face training is designed to be skill building. Family Team Decision-Making courses were implemented during FY 02 as part of the service contract. There are six legal Internet delivered training modules developed by the American Bar Association. Iowa State University provides the training for social worker case managers and foster and adoptive families. Using the university match, the state training funds are expanded to provide additional training funds. All service training is open to Juvenile Court Services and providers. Approximately 25% of the participant numbers are non-DHS participants. Assessment training is available only to DHS employees.

Training Committee
Training for both classifications is over-seen by a Training Oversight Committee. The committee meets monthly to work on the development, evaluation and updating of the training curriculum. The current Training Committee has 27 members representing:

- DHS eight (8) geographical service areas
- DHS central office
- Child Welfare Information Systems and training contractors

The trainers are part of the Training Oversight Committee, therefore are involved in monthly discussions of how to meet the training needs of staff. The trainers read all their evaluations and make changes based on this feedback as well. Training is updated as policy changes.

Training Plans, Communication and Continuing Education Units (CEUs)
Employees are required to discuss their training plans annually with their supervisor via the PPEs- Performance Plan and Evaluation. A continuing education calendar form is available to employees to plan out their yearly training. A yearly training schedule is developed and monthly calendars are distributed to staff listing upcoming training courses and dates. Both the monthly calendar and yearly schedule are available to staff in the educational computer electronic folder. A course listing is available to staff on our Internet web-site and Intranet.

Staff register for courses via their area training coordinator upon approval of their supervisor. Course registrations are entered into the Iowa Interagency Training System, IITS. Reports can be generated from the system to review numbers and hours of training. Also, employees' personal training history is available in the IITS system.

Social work continuing education units are offered for most courses and are available at no charge to requesting participants who complete the course requirements.

Evaluation
Training is evaluated on Levels I, II & III of Donald Kirkpatrick's evaluation model. Dr. Donald L. Kirkpatrick, Professor Emeritus from the University of Wisconsin, is the author of the book Evaluating Training Programs: The Four Levels, which is the widely used evaluation model in training and development. Briefly, the four levels are:

- Reaction - how do participants feel about the program/customer satisfaction
- Learning - to what extent did the participants increase knowledge, improve skills and/or change attitudes
- Behavior - to what extent did their job behavior change
- Results - what final results occurred - quantity, quality, safety, sales, costs, and profits.

After each course, all participants rate the course, facility and trainer. Information from the 10-question evaluation is entered into an Access database and quarterly reports are generated. The aggregate totals for the ten questions routinely ranges above 4.5 on a scale of 1- 5 with 5 being the highest.

After the basic training courses, the trainers provide feedback to the student and their supervisor on the application of the training to the work responsibilities.

A survey was conducted with both new employees and their supervisors regarding the training redesign. The feedback was positive overall.

Child Protective Training

New protective workers attend a five (5) day basic training that is offered quarterly. One half day of this training is learning the STAR system – Statewide Tracking Assessment Reports. It is recommended that new child protective staff attend basic training prior to case assignment.

Web based CPTA courses	**Hrs to complete**
Overview of Child Welfare	2 Hours
DHS Manual	8 Hours
STAR Tutorial	2 Hours
Legal Fundamentals	2 Hours
Medical Fundamentals	2 Hours
Substance Abuse	2 Hours
Domestic Violence	2 Hours

	Length
CPTA Basic Training	5 days

Two Intermediate training courses for new child protective staff are recommended to be completed within 18 months of employment but no later than the end of the 2nd year of employment.

Application of Legal and Medical Issues To Child Protective Assessments - 3 Days
Impact of Domestic Violence and Substance Abuse on Child Abuse Assessments -- 2 Days

Both basic and intermediate training courses are tested. Participants need to score 75% correctly. This feedback is used to ensure that staff are learning the material as well as to enhance the training content. Advanced courses are not tested as they are considered professional development.

Experienced child protective staff were required to attend four (4) days of Child Protective Academy Training each year until the beginning of FY 03 when the requirement was dropped due to training budget cuts. Examples of courses include Successfully Handling Challenging Assessments, Current Issues in Child Abuse, Advanced Medical Issues, Expert Testifying, Decision Making.

During state FY 01, 1113 students received 273 classroom hours for a total number of training hours of 5,609. Preliminary statistics indicate approximately the same number of participants and training hours were provided in FY 02.

Due to the precipitous cut of approximately 75% of state dollars in the training budget for FY03, training has been significantly decreased for FY 03 as indicated below. The Council on Human Services FY 04 budget recommendation for training starts to build back the range of courses offered. Training issues will be addressed in the PIP.

Child Protective Training Academy	**FY 02**	**FY 03**	**FY 04**
Overview of Child Welfare - web	X	X	X
DHS Manual – web	X	X	X
Legal Fundamentals - web	X	X	X
Medical Fundamentals - web	X	X	X
Substance Abuse	X	X	X
Domestic Violence - web	X	X	X
Basic Training	X	X	X
Application of Legal & Medical Issues	X	X	X

Impact of Domestic Violence & Substance Abuse	X	X	X
Impact of Abuse on Child Development	X		X
Advanced Interviewing & Documentation	X		X
Successfully Handling Challenging Assessments	X		X
Sexual Abuse Allegations	X		X
Personal Maintenance	X		X
Current Issues in Child Abuse	X		
Advanced Medical Issues	X		
Assessing Medical Evidence	X		X
Impact of Trauma on Children	X		
Case Consultation	X		
Effects of Chronic Neglect	X		X
Expert Testifying	X		
Initial Data Gathering Strategies	X		
Facility Assessments	X		
Diversity Training	X		
Decision Making	X		

Social Work Case Management Training

Basic Training
Basic training for new social work case managers is organized around a series of nine (9 sequential modules which alternate between class room training modules followed by on-the-job modules that put training into practice and experience. The training design concept is for new staff to complete the first two modules before they are assigned cases. With downsizing of staff and reduction in training opportunities, this is not always possible. Starting in FY03, Basic Training Module 2 is offered every other month and the other classroom modules are offered quarterly.

Feedback
At the end of the first three classroom training modules – the trainers provide written feedback to the student as well as to their supervisor to assist with the transfer of learning from the classroom to the workplace.

Ongoing Service Training
Experienced workers have various course options available to them for advanced training. During FY 02, a major new series on Family Team Decision Making was implemented. Blended funding from Quality Service Review, Clark Foundation funding and service dollars made this series possible. There is no specified training quota for service workers after completing Basic Training.

Likewise, due to the 75% cut in state training dollars for FY03, training has been significantly reduced for FY 03 as indicated below for ongoing social worker courses. The Council on Human Services FY 04 budget recommendation for training starts to build back the range of courses offered.

Ongoing Social Worker Courses	**FY 02**	**FY 03**	**FY 04**
New Employee Orientation - OJT	X	X	X
FACS for New Workers	X	X	X

Services Nuts & Bolts - OJT	X	X	X
Strength Based Assessment	X	X	X
Strength Based Assessment - OJT	X	X	X
Achieving Permanency	X	X	X
Case Permanency Plan - OJT	X	X	X
Legal Aspects in Social Work	X	X	X
Testifying in Juvenile Court	X	X	X
Permanency & Termination of Parental Rights	X	X	X
Professional Development	X		
Introduction to Adoption	X	X	X
Foster Home Licensing	X	X	X
Caring in the Community for Children w/HIV	X		
Strategic Communication	X		
Concurrent Planning: Tools of the Trade	X		
Elements of Risk	X		
Creating Solutions with Teams	X		
Family Dynamics	X		
Children, Families & Illegal Drugs	X		X
Working with Native Americans	X		X
Psychological Testing: From Ref. To Interv.	X		
Transition for Foster Care to Adoption	X		X
Topics in Dependent Adult Abuse	X		X
Basics of Mental Illness	X		
CM/with Culturally Diverse Clients	X		X
Assessment in SW Case Management			
Social Work Ethics	X	X	X
Annual Adoption & Foster Care Conference	X		
Reducing Appeals on Terminations	X		
Foster Care & Beyond: Transitioning	X		X
Introduction to Training Redesign	X		
Promoting Excellence of Staff	X		
Intro to Family Group Decision Making	X		
Practice of Family Team Decision Making – 1	X		
Cultural Response-MEPA	X		
Helping Difficult Children	X		
Practice of Family Team Decision Making – 2	X		
Reactive Attachment Disorder	X		
Manager's Role in Family Team Decision Making	X		
Family Decision Making – Facilitation	X		
Advanced Appeals	X		
QSR/QA training	X	X	X
Keeping Kids Safe		X	
Coaching for Reflective Practice		X	X
Advanced Coaching For Reflective Practice			X
Elements for Case Success		X	
Best Practices for Child Welfare		X	X
Behavioral Assessment			X
Case Planning			X

Case Management/FTDM Facilitation			X
Protocols for Practice			X

A series of courses are being taken to each service area in the state as a means to manage within the reduced resources while continuing to impact practice initiatives. These courses are offered twice in each service area and cover the topics of: Safety and Safety Planning; Coaching for Reflective Practice; Promoting Case Practice Success; and Best Practice Forum.

BSW and MSW Education

Due to decreased training funds and staff resources, the undergraduate Bachelor of Social Work program between the Department and the University of Northern Iowa was discontinued January 31, 2002. Thirteen students benefited from this program. The IDHS Master of Social Work program with the University of Iowa will terminate after June 30, 2004 and is no longer accepting new students. Forty-six employees received their MSW and completed their two years – three years employment with the department after receiving their degree.

Statewide Assessment

In FY 01, a web-based survey was conducted with employees and supervisors on the new curriculum/training redesign to evaluate how the training assisted with actual job competencies. The feedback was positive and all responding employees said that the training content was utilized on the job.

This year, in addition to training surveys through the oversight committee, focus groups were conducted with the Training Oversight Committee, Juvenile Court Officers, Providers, Youth Advisory Board, and Judges for the Statewide Assessment. A survey of department staff including SW2s, SW3s, and SW supervisor staff was completed that queried training issues.

The Training Oversight Committee, a cross section of dedicated department staff who share decision making around training, described the following strengths and concerns about training.

Strengths:

- Training evaluation by internal and external groups informs training initiatives
- Transfer of learning is facilitated by the use of on the job training modules
- The training redesign survey reported an improved sequence for training, more relevance of training to the job, and use of feedback from classroom training to supervisors and employees

Concerns:

- Training is geographically centralized which requires some staff to travel up to 5 hours
- Supervisory support of training content and application is needed
- Supervisory training is limited
- Use of mentors and peer training has not been implemented yet
- Supervisory ratios and high staff caseloads limit involvement in training
- Some staff are given cases before they have completed the basic orientation training
- Intermediate and advanced course offerings are a need
- The department needs to encourage the active role of supervisors in the individual training needs assessment for social workers
- Including child welfare partner representatives is seen as a way to strengthen the oversight committee's work

The Youth Advisory Board, made up of youth in foster care told us:

- They are concerned that foster care social workers did not have the training necessary to understand the needs of foster children
- They feel that training should focus on:
  - matching placements to the child's needs
  - transition of children from home to foster care and from one foster home to another

Juvenile Court Officers and providers stressed:

- The importance of involving them as partners in decision making around training and the sharing of resources. Local partnership on training recommendations was lost with elimination of interdisciplinary and reasonable efforts training funds
- Concern that information about available training through DHS was not getting to all interested parties
- The need for increased opportunities for cross training opportunities

The 2000 Iowa Foster Family Satisfaction Study recommends initiating or increasing training:

- Assisting in the development of relationships between foster parents and workers, particularly with new foster parents and case workers
- Aiding the understanding of each other's perspectives, roles and purpose within the system
- Teaching and encouraging teamwork based on mutual respect
- Increasing the worker knowledge of specific disorders, syndromes, and behaviors, so they can support foster families dealing with children with these disorders in their homes
- Discussing factors effecting foster parent satisfaction: receiving rapid support in a crisis, training, respect from workers, good phone access, good matching caseworker's willingness to share information
- Joint training of staff and foster parents

In a survey, 496 DHS staff were asked the following survey questions. The percentage of those responding to each of the 5 available responses is indicated.

Survey Questions				
The training I have received meets the needs of my job.				
Strongly disagree	Disagree	Neutral	Agree	Strongly agree
7.9%	27.4%	30.6%	31.9%	2.2%
I understand the requirements of the Indian Child Welfare Act (ICWA).				
Strongly disagree	Disagree	Neutral	Agree	Strongly agree
14.2%	0%	28.7%	44.9%	12.3%

I understand the requirements of the Multiethnic Placement Act (MEPA).				
Strongly disagree	Disagree	Neutral	Agree	Strongly agree
20%	22.4%	24.4%	25.7%	7.5%

I understand the requirements of the Adoption and Safe Families Act (ASFA).				
Strongly disagree	Disagree	Neutral	Agree	Strongly agree
4.1%	7.1%	14.1%	47.3%	27.4%

23 Juvenile Court Services employees were asked on a scale of 1 to 5, with 1 being Strongly Disagree and 5 Strongly Agree, to provide their assessment of the following statement:

Question		JCS Response
The training I have received meets the needs of my job.		4.0

The changing needs of DHS staff responsibilities continues to be an ongoing need to keep current training available for DHS staff. With the diverse needs of staff from novice to expert level, the current limited funds do not provide for a wide variety of training.

Summary

Strengths

- New worker training for child protective workers and social work case managers has been maintained and is accessible to department staff
- Training has been made more accessible for more staff by providing DHS initiative driven training in the service areas.
- Comments are favorable regarding the new worker training and the training redesign as implemented
- Training available through the Clark Grant and the Family Team Decision Making/QSR Initiative is a strength of the training system.
- Training provides performance based practice skills necessary for best social work practices
- Joint training with domestic violence and substance abuse providers
- The Council on Human Services has requested funding for FY04 to reinstate many core courses that were eliminated due to budget cuts

Opportunities for Improvement

- Budget cuts have resulted in limited ongoing training course offerings
- Staff find it hard to find time to attend training due to rising caseloads
- Mentoring system as a strategy to develop and retain staff
- Develop a culture of continual learning
- Child welfare principles and best practices need to be incorporated into all departmental training
- Support DHS workers as subject matter experts to increase capacity of staff
- Need to implement a strategy to do an on going assessment of training needs
- Provide an increased opportunity for cross training
- Accessibility of training due to large geographic areas to be served continues to be a challenge. Examples of delivery of training via distance learning technology which have been implemented are:
    - Iowa Communication Network (ICN) Presentations
    - Distribution and sharing of training videos
- Staff need more training regarding Indian Child Welfare Act (ICWA) and the Multiethnic Placement Act (MEPA)
- Joint training of staff and foster parents
- Provide training and or joint training for private provider staff
- Training options for supervisory staff - there are limited training options designed specifically for supervisors

Two

2. **Citing any data available to the State, discuss the effectiveness of the State's training of current and prospective foster and adoptive families and the staff of State licensed or approved child care institutions that care for children in the State's care or responsibility that addresses the skills and knowledge base needed to carry out their duties.**

Description

Basic foster parent pre-service requirements were legislatively changed July 1, 2002 from 12 to 30 hours of approved pre-service training. DHS uses Child Welfare Institute's PS/MAPP curriculum for the 30 hour pre-service. All pre-service training requirements must be met prior to initial license. Treatment level foster parents are required to also complete 12 hours of treatment training and meet all certification requirements before being certified as a treatment level home for children requiring that level of care. Annual ongoing in-service training requirements for basic foster parent license require 6 hours in-service training prior to each annual renewal. Certified treatment level parents must complete and 6 additional hours to address the needs of treatment level children.

Regional/Service Delivery Area annual plans for recruitment and retention include training ideas. Previously recruiters helped to identify the training needs of foster parents and work to set up and deliver needed training. Recruiter positions were eliminated due to budget cuts. Foster parents have a minimum number of training hours they need in order to be re-licensed annually. This is met in a variety of ways i.e. group, individual training, videos, tapes, books, etc. Each training curriculum must be approved for a specific number of hours.

Every month pre-service training is offered at the 15 community colleges across Iowa. Training are team taught by a social worker active in foster and/or adoption and a licensed foster and/or adoptive parent of a child with special needs.
In FY '02:

120 Foster Parent Pre-service classes with 2,410 participants
76 Adoption Pre-service classes with 1,326
15 Treatment Parent Pre-service classes with 169 participants

Through a combination of information sharing and experiential activities prospective foster parents and adoptive parents are introduced to:

- The Child Welfare System
- The special needs of children who require placement in out of home care
- Parenting strategies
- Stages of child development
- Issues of grief, loss, attachment and bonding

After initial orientation, participants in the foster parent and adoption pre-service training choose courses they wish to take. Placement workers may recommend that currently licensed foster parents who exhibit some of the skills necessary for treatment parents attend treatment training and be considered for certification. All providers, both private and public, of foster care,

treatment care and/or adoption services are encouraged to attend the basic training required of the families.

During FY 03, "Partnering for Safety and Permanency: Model Approach to Partnerships in Parenting" (PS-MAPP) will be implemented for foster and adoptive parents training.

Each licensed foster parent is to be evaluated by the licensing staff to establish additional training needed to build upon initial pre-service or to enhance their skills. For certified treatment parents a written training plan is to be developed and reviewed with the family annually. Licensing/certification staff and purchase of service staff evaluate the providers' qualifications and training needs.

Each licensed foster parent must meet the ongoing in-service training requirements for the number of hours to maintain their license. There is no specific curriculum which all foster parent in-service training must follow, however licensing staff are to asses the individual training needs of the foster parent and recommend training from the topic areas listed in IAC 441-117.7(2). If a relative is receiving foster care payment to care for a related child, there is no difference in the licensing requirements for pre-service and in-service training.

Each year pre-service curriculum is revised to incorporate changes in departmental requirements as well as any legal decisions.

DHS contracts with IFAPA to provide in-service training.

Each month the ISU newsletter, Trumpeter, is sent to over 4,000 recipients -- including foster/adoptive families, DHS, private agencies, ISU and community colleges.

Statewide Assessment

Participants in a foster/adoptive parent focus group told us:

- The current foster parent training is inconsistently delivered
- Families are not adequately prepared to meet the needs of special needs children (85% of children placed have special needs)
- Lack of training is seen as a causal factor to the stability of placements
- Foster parents generally feel they are not prepared to handle the children, which is seen as a contributing factor in the rate of abuse in foster care
- There are dedicated case workers who try to help them but ongoing support is not provided because of high case manager case loads

In a survey, 496 DHS staff were asked the following question. The percentage of those responding to each of the 5 available responses is indicated.

Survey Question				
The foster parents that I work with are prepared to address the needs of the children they serve.				
Strongly disagree	Disagree	Neutral	Agree	Strongly disagree
4.4%	17.6%	28.5%	43.9%	5.6%

Summary

Strengths

- DHS Service Delivery Area annual plans for recruitment and retention of foster and adoptive parents include training ideas
- Good partnerships have resulted in good outcomes for foster and adoptive parent training**.**
- DHS contracts with IFAPA to provide orientation for potential foster and adoptive families, as well as in-service training
- Change in pre-service training, PS-MAPP training, has been initiated, which will allow foster parents and adoptive parents to be trained together which contributes to the continuity of care for children through transitions and promote stability of placement with this training. This training will also provide foster and adoptive parents with more experiential opportunities to help prepare them for fostering/adoption
- Each year pre-service curriculum is revised to incorporate changes in departmental requirements as well as any legal decisions
- DHS contracts with IFAPA for the KidSake recruitment and retention project
- Beginning next year,
    - Each licensed foster parents will be evaluated by the licensing staff to establish additional training needed to build upon initial pre-service or to enhance their skills
    - A written training plan will be developed and reviewed annually with certified treatment parents
    - Licensing/certification staff and purchase of service staff will evaluate the providers qualifications and training needs

Opportunities for Improvement

- High caseloads make it challenging for DHS workers to partner with foster and adoptive parents to provide PS-MAPP training
- Private providers also find it challenging to partner with foster and adoptive parents to provide PS-MAPP training, because of limited resources
- DHS needs to implement policies and practices, including reduced case loads, that allow for case workers to provide ongoing support to foster parents
- Training needs to be offered consistently across the state
- Special needs children continue to challenge our parent training system

Service Array and Resource Development

One

1. Discuss how effective the state has been in meeting the title IV-B State Plan requirement to provide services designed to help children safely and appropriately return to families from which they have been removed.

Description

When children are removed from their family and placed into foster care, the department case manager provides the following services. Employees' manual states that child safety is paramount when making decision regarding service provision.

- Assessing the reason for removal and what needs to occur for the child to safety and appropriately return to their family
- Developing the case plan in partnership with the child and family
- Arranging and coordinating for needed services and supports for the child and parents
- Supervising the placement and providing crisis intervention
- Monitoring case activities and providing reports to the court

The department contracts with private child welfare agencies for the following services designed to help children safely and appropriately return to families from which they have been removed.

- Family centered services
- Family preservation services
- Time-limited family reunification services
- Decat funded family reunification services
- Family foster care services
- Group care services

The department also arranges for and coordinates services funded through other agencies and programs, including the following.

- Substance abuse services for the parent, funded through the Department of Public Health and Medicaid
- Mental health services for the parent, funded through Medicaid or county mental health systems
- DHS staff work closely with Magellan Behavioral Health staff in accessing mental health and substance abuse services for children in the child welfare system

Family Centered Services. Family centered services include a comprehensive array of pre-placement services to prevent removal, as well as services to help children safety and appropriately return to families from which they have been removed. Approximately one third of the children receiving family centered services are placed out of home and receive family centered services to help them return home.

In FY 1993, the department added family centered services to Iowa's Medicaid State plan under the rehabilitation option. As a result, the department was able to significantly increase funding for family centered services. Between FY 1993 and FY 2002, funding for family centered services increased 169 % from $14.5 M to $39 M.

In FY 2002, the department provided family centered services to an average monthly population of 6,116 children. In FY 2003, the department's budget for family centered services would support services for an average monthly population of 5,998 children.

Family Preservation Services. Family preservation services are available 24 hours a day, 7 days a week, and are designed to stabilize a family in crisis. Services are provided for an average of 45 days. Approximately 20% of children receiving family preservation services are placed out of home and receive family preservation services to help them return home.

In FY 1993, the department added family preservation services to Iowa's Medicaid State plan under the rehabilitation option.

In FY 2002, the department provided family preservation services to an average monthly population of 40 children. In FY 2003, the department's budget for family preservation services would allow us to serve an average monthly population of 88 children.

Time-Limited Family Reunification Services. The department allocates the Title IV-B Part 2 funding for time-limited family reunification services to the 38 Decat clusters to fund reunification services. Following are examples of specific services funded at the community level.

- Families in Recovery Program, substance abuse services
- Parent Time Out and Crisis Child Care
- Individual, group and family counseling
- Transportation to or from services and activities above

Decatagorization [Decat] Funded Family Reunification Services. "Decat" services are locally designed and flexible services that are developed through a collaborative community process. Funding for these services comes from a small allocation specifically targeted for Decat services, as well as from any unspent child and family services funding the Decat project is able to carry over from the prior fiscal year. In FY 01, Decat projects carried over $23.5 million from prior fiscal years, and spent $17.4 M on "Decat" services; approximately 20 % of these funds were spent on services to safely and appropriately reunite children with their families. In FY 02, Decat projects carried over $14 M from prior fiscal years. Following are examples of specific services funded through "Decat":

- Parent/child visitation programs to enhance the intensity of parent/child contact and thus promote reunification
- Case facilitation services to develop flexible and individualized family reunification plans
- Intensive day treatment programs to provide a structured environment for youth returning to their families from out- of-home placement
- Social work case aides who worked with assigned social workers to maintain more frequent parental contact and parent/child visitation in order to better achieve reunification
- Specialized programs to provide consultation on substance abuse and domestic violence cases in order to better achieve reunification

Family Foster Care Services. Family foster care services include services to help children safely and appropriately return to families from which they have been removed, as well as services to address the safety and well-being needs of the child and support the foster family.

Group Care Services. Group care services include services to help children safely and appropriately return to families from which they have been removed, as well as services to address the safety and well-being needs of the child.

Prior to budget reductions in FY 01 and 02, the department also funded parent-child visitation projects to increase visitation between parents and their children, and family-to-family projects in which foster parents worked directly with birth parents to support reunification.

Statewide Assessment

The following information was gathered from focus groups, interviews and surveys.

During a focus group, providers expressed a desire for adequate funding for both family centered and family preservation services along with a relaxation in the regulations surrounding these services. They felt this would allow them more flexibility to meet the broad range of needs presented by children and families. Providers and department staff expressed concern that funding family centered and family preservation services through Medicaid has resulted in too much of a focus on the child and not enough focus on the family.

Due to funding cuts, providers have closed a number of existing family centered and family preservation programs throughout the state. Private agency staff salaries are very low which makes it difficult to attract and keep quality staff for the remaining programs. Both department caseworkers and providers expressed the need for better mutual communication around family needs in both these programs and stressed the importance and value of good needs assessments prior to referring a family for these services.

Also during a focus group, foster families identified their role in working with children's birth families as a strength.

One of the concerns identified during focus groups was the lack of resources to supervise visits between children in foster care and their parents. Department caseworkers do not have time to supervise visits because of high caseloads, and funding to pay for supervision is less available. As resources to supervise parent-child visits have decreased, visitation has become less frequent.

Focus group information indicates group care providers believe strongly in the importance of working with families while children are in group care. They emphasized the need for adequate funding and sufficient authorization in order to work effectively with families of children in placement. The following were frequently cited as barriers to routinely placing children close to their home neighborhood and school:

- Demand for group care
- Special needs of children being referred
- Number of group care beds being closed due to funding cuts

Because funding for transportation services has also been reduced or eliminated, families often find it difficult to visit children placed outside their home communities. Distance and funding also limit the availability of after care services which make effective reunification from group care placements more challenging.

Department staff were asked to rate the effectiveness of a range of services for children and families in the child welfare system. On a scale of 1 to 5, with 1 meaning very ineffective and 5 meaning very effective, staff rated family centered and family foster care services as 3.5 or higher

in effectiveness. Staff were also asked to rate the degree to which they agreed with the statement, "There are services available in my Service Area designed to promote reunification of children with their families" on a scale of 1 to 5, with 1 indicating strong disagreement and 5 indicating strong agreement. The average rating was 3.8.

Iowa has a long tradition of supporting and funding innovative home-based services for vulnerable children and families. This tradition dates back to the mid 70's when Iowa developed the Families, Inc. intensive home-based program and continued in 1987 when Iowa initiated family preservation programs and was one of the first states to have these services available statewide. Even now, in spite of the recent budget reductions, over $39 million is allocated for family centered services.

Over the past several years, there has been a decline in the number of referrals to providers for family preservation service. Factors which have driven this trend include the following:

- Management and staff often find the service too costly
- Some DHS managers and staff feel that the experience and quality of family preservation providers has diminished and that it is no longer as strong a service as it was when Iowa modeled its program after the Homebuilders model
- Most families served by family preservation programs have multiple problems requiring on going family centered services so in order to avoid disruption in service providers, staff will begin with the family centered provider
- Many family centered provider agencies have enhanced their capacity to respond quickly, and with intensive service contact if necessary, to crisis referrals from the department, thus reducing the need for family preservation services

Inquiries will be made into all of these issues as we further explore this issue and develop our Program Improvement Plan.

Generally, there is sufficient diversity in the types of child welfare providers available to serve a child and family to allow for individualization of approaches based on client needs. However, given the recent budget cuts, caseloads for department workers have increased dramatically and funding that used to be available, through the Wraparound Program and Decat funds, to support individualized approaches has been eliminated or dramatically reduced. The impact of these two factors is that department workers have less time to assess and identify the unique needs of each client and there is less flexible funding available to respond to client needs for services such as respite, mentoring, transportation for visits, support for concrete assistance such as rent, utilities, furniture, etc.

Summary

Strengths

- DHS has policy, procedure, and a comprehensive array of reunification services
- Reunification services are available to families
- Numerous successful collaborative efforts at the local and state level

Opportunities for Improvement

- Funding for family centered and family preservation services has been significantly reduced in the last two years, as has funding for innovative projects such as parent-child visitation and family-to-family
- Program rules for family centered and family preservation services need to be more flexible and to focus more on the family and not just on the child

Two

2. Discuss how effective the state has been in meeting the title IV-B state plan requirement to provide pre-placement preventive services designed to help children at risk of foster care remain safely with their families.

Description

The department directly conducts child protection assessments and provides case management. Services provided by the case manager include:

- Developing the case plan in partnership with the parents
- Arranging/coordinating and contracting for needed services
- Conducting Family Team Meetings
- Placing children in foster care
- Providing crisis interventions, supervision, treatment and follow-up
- Monitoring case activities
- As noted in the previous sections, all other services are purchased from private child welfare providers, funded through grants to community agencies, or arranged through other funding sources such as the Iowa Plan, Iowa Department of Public Health, schools, etc.

Following are the pre-placement preventive services designed to help children at risk of foster care remain safely with their families funded or coordinated by the department:

Family-Centered Services. Includes a comprehensive array of home-based interventions for children who have been abused or are at risk of abuse, and day programs for children requiring intensive community-based interventions due to serious behavioral and/or emotional problems. Family-centered pre-placement preventive services include services to:

- Prevent and alleviate child abuse, neglect and, delinquency and out-of-home placement
- Support children placed with non-parental relatives and in kinship arrangements
- Maintain reunification or other alternative permanent placement when children have returned home or been placed in a permanent setting

Family Preservation. Is intended to help children remain safely at home by providing services to rehabilitate families, prevent unnecessary out of home placements and increase family strengths to reduce the risk of child abuse and neglect. This program also serves as both a preventive and reunification service. Services are available 24 hours a day to stabilize families in crisis.

Services are time-limited services (with average service duration of 45 days). In FY02 an average of 90 families with children at risk were served per month; 86% of these children remained in their family home 90 days after completion of the program.

Due to increases in department caseloads and decreased funding to purchase family centered and family preservations services, the department has had to reduce the provision of services on a voluntary basis to children and families in which there has been a child abuse assessment and/or there is an identified risk of abuse or neglect.

Adolescent Monitoring and Outreach. Provides services to youth that are either at risk of entering group care or training school placement, and as an aftercare service for youth that are returning from group care or training school placement. The program is intended to divert these youth from at-risk behaviors and/or out-of-home placement by helping them maintain self-accountability for their activities in a community-based setting and to reduce recurrence of at-risk or delinquent behaviors.

In FY02, 73% of youth in the program did not re-offend and were diverted from group care or institutional placement. 12.2% of youth were placed out of the home during participation in the program. In FY 2002, approximately 1,772 youth were served monthly statewide. In FY 2003, funding was available to serve an average monthly population of 1,368.

Supervised Community Treatment. Provides supervised educational support and treatment during the day for children who are experiencing behavioral, or emotional problems. Youth served by the program are either at risk of group care or state training school placement, or are returning from group care or state training school placement. Community treatment programs may be co-located with a school, but may not fund the cost of the school's educational programming.

In FY02, 73% of youth did not re-offend following completion of the program and only 12.2% of youth were placed out of the home during participation in the program.

Child Abuse Prevention. This program is intended to reduce child abuse and neglect by providing services to families and children at key points in the family life cycle - e.g. immediately after the birth of a child, young parents with infants, families under stress regardless of age of parent or child. In FY 02, grants were awarded to local child abuse prevention chapters that provide a local match for the following programs:

- Crisis nursery services (8 counties)
- Parent education (40 counties)
- Respite care services (21 counties)
- Sexual abuse prevention (34 counties)
- Young parents (13 counties)

In FY02, these grantees provided instruction and services to 5,179 parents and adults and 41,726 children. The grant also provided 83,925 hours of crisis nursery and respite care services to 1,313 families.

Clark Partnership for Protecting Children. The Community Partnerships for Protecting Children (CPPC) focuses on individualized services to meet the specific needs of children at risk. Iowa currently has 5 CPPC sites: Ottumwa, Waterloo, Spencer, Ames, and Charles City – plus the original site in Cedar Rapids. In Community Partnership sites, CPS workers

convene a family team meeting with families, neighbors and local service providers that results in tailor-made plans designed to support the family and ensure the safety and well being of the children. These plans identify the specific activities to be carried out by parents, friends, extended families and other formal and informal supports. Each Community Partnership site also creates a network of agencies, neighborhood and community supports that are available to families before, during and after crisis occurs. Citizens and community members are directly involved in shaping the types of services and supports that are made available to families. Localities shape their own strategies and develop a range of services based on their own resources, needs and cultures. Community Partnership sites collaborate with the domestic violence and substance abuse community to provide more coordinated and comprehensive services to children and families.

Healthy Opportunities for Parents to Experience Success (HOPES)/Healthy Families Iowa (HFI). This is a comprehensive prevention program designed to promote healthy, safe, self-sufficient families. The Iowa Department of Public Health administers HOPES/HFI and services are delivered under a contract with Prevent Child Abuse Iowa. Trained family support workers provide home visitation services to families in order to:

- Promote optimal child health and development
- Improve family coping skills and functioning
- Promote positive parenting and family interaction
- Prevent child abuse and neglect, as well as infant mortality and morbidity

Protective Child Care. The department provides funding under the protective childcare program to enable children to receive safe and stimulating child care services in state regulated settings while their parents are receiving protective child welfare services resulting from a child abuse assessment or other report. In FY03, approximately $3.7 million state dollars were appropriated for these services. Case managers can enroll children in protective childcare services if they feel this service will reduce stress in the family and promote healthy child development.

Life Skills Development. Serves youth that are either at risk of or have been adjudicated delinquent. The program is intended to eliminate or reduce recurrence of at-risk or delinquent behavior by helping youth establish positive behavior and living patterns. In FY02, an average of 250 were served monthly. 80% did not re-offend after completing the program, and 97.4% were diverted from group care placement.

School-Based Supervision. The program provides juvenile court school liaisons on-site services at middle and high schools for children experiencing truancy or other behavior problems at home, at school, or in the community. Juvenile Court Services collaborates with local school districts to jointly fund the program. Youth who demonstrate disruptive behaviors at school or at home and are either at risk of or have been adjudicated delinquent are referred to the program by juvenile court services, the school, or the department. In FY02, the program served 4,429 youth in 232 schools. Schools reported a 60% reduction in disciplinary referrals.

Community Based Family Resource and Support (CBFRS). Three grant programs funded through the federal Child Abuse Prevention and Treatment Act (CAPTA):

- Support state efforts to develop, operate, expand and enhance a network of community-based, prevention-focused, family resource and support programs that coordinate resources among a range of public and private organizations

- Foster understanding, appreciation, and knowledge of diverse populations in order to effectively prevent and treat child abuse and neglect
- Provide parent education programs targeted at families with young children
- Provide respite and crisis child care services
- Support Parents Anonymous groups, which are parent education and support groups, across the state

Decategorization Funded Services. (Decat) Locally designed nontraditional services provided to strengthen and promote child well-being and family stability and to prevent the need for more intensive/intrusive, deep-end services. Through a collaborative process, local communities have the flexibility to identify service populations that may be most vulnerable or underserved and develop "localized" services to address their needs.

In FY01, decat projects spent over $17 M in the following service areas:

- 38.8% for prevention/early intervention programs
- 10% for after care services
- 39.4% for support of innovative services for children already involved with the child welfare or juvenile justice system

In FY02 Decat funds were severely constricted due to the need to focus limited child welfare budgets on core child welfare services.

Prior to budget reductions in FY 01 and FY 02, the department also funded wrap-around services to prevent group care placement.

Statewide Assessment

The following information was gathered from focus groups, interviews and surveys.

Stakeholder focus groups and department staff believe that the best permanency option for children is in their own homes, and the best permanency services are those which help children to remain safely and appropriately at home. There is wide support for programs that keep children at home as the primary goal. Focus group participants would like to see these programs expanded to serve all counties, and expressed concern that funding for these services, where they do exist, may be eliminated due to budget constraints.

There was general agreement among focus group participants that the department does a good job protecting children. The relationships between department staff and local law enforcement are good. Participants also expressed belief in the value of prevention and aftercare services in keeping children safe. There is wide spread belief in the importance of community involvement in keeping children safe. Initiatives like the Community Partnership for Protecting Children, and local Child Abuse Prevention Councils were seen as mechanisms to achieve this. There is also frustration that at-risk children may be 'falling through the cracks' because the department has limited its role to child protection.

Schools are an important part of the child protection and treatment team. In order for schools and the department to be truly effective partners, more two-way communication and a mutual understanding of each agency's role and function in protecting children is critical. School personnel expressed frustration about the lack of feedback they received from the department. Although department staff responded quickly to their reports of child abuse or neglect, school

personnel felt that the information they received from DHS limited their ability to deal effectively with the child and family.

School and juvenile court personnel report that school-based services provide vital crisis intervention for at-risk students in the school system. The program also provides schools with regular feedback about these students and gives them the flexibility to meet needs as they arise. The regular student contact this program provides helps to reassure at-risk students that someone cares about their future and keeps them from 'falling through the cracks'. In many cases, the school-based liaison was seen as the best information link between the school and the department, but communication was still identified as a problem, especially when several 'helpers' are involved with the same child. According to respondents, the program needs more and stable funding, expansion into more school districts and the ability to add a summer program.

Many stakeholders and department personnel view Decat as a strength of the system. Local Decat committees have been effective in bringing community stakeholders together as part of a shared process to develop a continuum of services tailored to meet local needs. Decat has also been the department's only source of flexible funds that can be used to meet family needs not covered by categorical programs. Focus group participants expressed great concern that Decat funds will be eliminated in the FY03 budgets, leaving the department with little flexibility to creatively meet the individual needs of children and families.

Families interviewed as part of the QSR process consistently reported having good relationships with both child protection workers and the ongoing caseworker. They frequently stated that they "liked" their caseworkers. The most common complaints expressed by these families were linked to the process of transitioning between the assessment worker and the case manager and the time delays between when services were offered and their actual inception.

During a focus group, providers expressed a desire for adequate funding for both family centered and family preservation services along with a relaxation in the regulations surrounding these services. They felt this would allow them more flexibility to meet the broad range of needs presented by children and families. Due to funding cuts, providers have closed a number of existing family centered and family preservation programs throughout the state. Both department caseworkers and providers expressed the need for better mutual communication around family needs in both these programs and stressed the importance and value of good needs assessments prior to referring a family for these services.

Various focus groups expressed the feeling that Monitoring and Outreach and Supervised Community Treatment represents the best opportunity for schools, juvenile court officers and department personnel to collaborate around the needs of delinquent and pre- delinquent youth. School personnel praised the effectiveness of school liaison officers in meeting the needs of both juveniles and their families. Juvenile Court Officers and school personnel in particular felt that these programs were of benefit to delinquent youth because services were provided in their own home schools and community. Of concern were the facts that these services were not available in all school districts or in all schools and the current funding cuts would lead to elimination of some programs.

Programs described in this section have been negatively impacted over the past few years because of budgetary problems in the state.

Summary

Strengths
- DHS has policy, procedure, and a comprehensive array of services providing preventive services
- Community-based services are available to families
- Numerous successful collaborative efforts at the local and state level
- School liaison officers are seen as very effective
- Innovative programming such as: Community Partnership for Protecting Children, Decategorization, HOPES, etc.

Opportunities for Improvement
- Fewer families receive voluntary services
- Funding for family centered and family preservation services is not adequate to meet the increased demand resulting from increased number of confirmed child abuse reports and court adjudications. Home based services may prevent child abuse and/or reduce the need for more costly out of home placements
- Improved communication with stakeholders
- funding for programs such as Monitoring and Outreach and Supervised Community Treatment has been reduced significantly in the last 2 years

Three

3. Discuss how effective the state has been in meeting the title IV-B state plan requirement to provide services designed to help children be placed for adoption with a legal guardian, or if adoption or legal guardianship is determined not to be appropriate for a child, in some other planned, permanent living arrangement.

Description

DHS policy requires that when timely reunification is not possible, the department must make reasonable efforts to expedite another permanency option. The department must provide ongoing case review and oversight until case outcomes are achieved.

The department provides the following permanency services.
- Adoption services
- Adoption subsidy and post-adoption services
- Transition services for youth aging out of foster care

Adoption Services. Provides permanent, stable adoptive placements for children whose parents' rights have been terminated. Over 80% of these children have a diagnosed physical, mental, or emotional disability and more than 50% are older than age eight. The department and contracted private providers deliver adoption services focusing on preparing, matching, supporting, and supervising the placement of children in adoptive homes. Each year foster parents adopt approximately 75% of children adopted. When their foster parents do not adopt a child, the foster parents help children with the transition to a new permanent home and provide valuable information about the child to both the caseworker and the adoptive parents.

Over the last 10 years, the number of department adoptions has increased from 138 in FY 1993 to 793 in FY 2002.

Adoption Subsidy and Post-Adoption Services. The department also provides financial assistance, medical coverage, and special services to families who adopt children with special needs. The department also contracts with the Iowa Foster and Adoptive Parents Association to provide post-adoption support for adoptive families, including adoption helpers who contact families after adoption finalization to offer support, support groups, and respite care. The number of families participating in the adoption subsidy program has also increased significantly in the last 10 years, from 1,323 in FY 1993 to 4,874 in FY 2002.

Transition Planning. The department coordinates transition planning through 8 staff positions out-stationed in local offices. Recent legislation directs department case managers to coordinate transition planning with representatives of the adult service system whenever the child might need services from that system after aging out of foster care. The department also funds an Aftercare Network – a coalition of 10 private child welfare providers that provides aftercare services for youth who have recently aged out of foster care. Aftercare services are available up to age 21.

Statewide Assessment

Participants in focus groups, interviews and surveys told us the following regarding adoption services:

QSR focus group participants (in meetings held between April 2000 and May 2002) view the state as successful in its adoption efforts. Many children are being placed in safe, permanent homes and the majority of placements are well matched and don't disrupt. Providers participating in focus groups held during April and May, 2002, noted that in the last year children seem to be moving from foster care to adoption at a slower pace as department caseloads rise. Data from our SACWIS system, however, indicates that the Department finalized the highest number of adoption ever in FY 2002.

QSR focus group participants (between April 2000 and May 2002) consistently report their belief that the Department meets the needs of younger children with fewer behavioral concerns quite well. Frequently, these children are placed initially into homes that could eventually adopt. They believe that state does less well with older and/or more behaviorally/emotionally disturbed children who typically move several times before permanency can be established. Unmet needs, in the opinion of these focus groups, include subsidized guardianship and expanded post finalization services. They see the department as also needing to improve the transitioning of cases from the case manager to the adoption specialist. Additionally they believe the state should begin to track the number of adoption disruptions more systematically. The department has just begun to track adoption disruptions. Between 1996 and 2002, our data suggests a disruption rate of between 1% and 3%.

The department came very close to implementing a subsidized guardianship program in April, 2001. However, the program was suspended before it was implemented due to reductions in state revenue.

Information obtained from QSR focus groups with juvenile court judges, county attorneys and attorneys functioning as guardians ad litem suggested that, despite ASFA, there is a tremendous

variance in attitude and approach among judges and juvenile court districts around permanency. In some service areas, the courts are willing to move children quickly to permanency while in other areas judges and county attorneys are less willing to move to TPR.

QSR findings suggest that more attention should be given to the permanency needs of older children, especially those who are less likely to be adopted because of their extreme behaviors. These children could benefit from improved access to appropriate mental health and substance abuse treatment and transitional living experiences that can be individualized to meet their needs.

Transition planning and after care services for children in foster care need to be improved and expanded. This is especially true for children aging out of foster care transitioning into adult mental health services. Transition Planners involved in a focus group told us they believe that successful independent living skills are best accomplished through a "hands-on" approach in the most natural setting possible. Often, group home staff, foster and adoptive parents are best equipped to teach life skills. Providers also expressed concern that many of the youth referred to independent living are not appropriate for this service because they are not yet ready for self-sufficiency. For many of these youth, the duration and intensity of services authorized is not adequate to meet their needs. Both providers and department staff would like to see some type of sheltered assisted living program developed to meet the needs of youth who are on the verge of aging out of foster care.

Juvenile Court Officers report that the Independent Living Skills program is often used as the aftercare component of a group care or institutional placement. The program is particularly important for youth who will not be returning home and need a supportive living arrangement until they are able to demonstrate the maturity to make decisions on their own. Concern was expressed that despite the services available, many youth are unable to handle themselves well in scattered apartment living and, for some, a long-term group home placement may be more effective. There is also a need for more clustered site independent living placements.

Summary

Strengths

- DHS has collaborated with public and private agencies to increase adoptions
- Adoption efforts are viewed as successful by stakeholders and the public in general
- Iowa has a strong foster and adoptive family association, IFAPA
- Iowa has a strong Independent Living program to assist youth with transition to self-sufficiency
- Joint Treatment Planning with DHS, Magellan staff, mental health providers and county staff has been used to develop transitional plans and could be expanded

Opportunities for Improvement

- There is a continued need for subsidized guardianship as a permanency option for children. DHS developed a Subsidized Guardianship program for which funding for 250 slots was appropriated but then deappropriated due to budget deficits
- Expanded post adoption services are needed
- Tracking of disrupted adoptions needs to be improved
- Improve access to mental health services, substance abuse treatment, and transitional living experiences for older children who will not be adopted

Four

4. **Describe the extent to which all services in item 1-3 are accessible to families and children on a statewide basis.**

Description

Iowa has a wide array of services available to families. Funding for services is allocated to each of the eight service areas to ensure that core services such as foster care, adoption and in-home services are available in all service areas. Many communities have worked collaboratively through Decategorization Committees and other community partnerships to develop new programs and services that address the particular needs in their local area.

Although services are often well developed at the local level, many services are not equally available throughout the state. Iowa's population density makes service delivery particularly problematic in rural areas where the distance and travel time between clients limits the cost effectiveness of service delivery for providers. Services such as day treatment, specialized treatment and group care are not available in every community, resulting in children being placed outside of their communities and neighborhood schools. Also, services provided at mental health facilities often require children be placed a distance from home.

Budget reductions for both in-home and out-of-home services have resulted in a reduction in services to help children safely and appropriately return to families from which they have been removed, pre-placement preventive services designed to help children at risk of foster care remain safely with their families, out-of-home placement services, and permanency services. In August 2002, a waiting list for group care placement was established. Children are placed into group care on a first-come first served basis, based on the date of the court order.

Statewide Assessment

Participants in focus groups, interviews and surveys told us the following regarding the accessibility of services across the state.

- Department staff generally agreed that services to preserve families, reunite families, and promote permanency for children were available in all Service Areas
- According to department caseworkers, there is a great variance in the quality and availability of services around the state
- There are also noted differences in the service array that exists between urban and rural locations. In many areas, providers were applauded as being 'strong, excellent, and knowledgeable' while in other areas department staff was generally dissatisfied with the available services and the knowledge and experience of provider staff. Department staff often expressed a willingness to refer a family to an agency only if a particular staff member was available
- Both providers and department staff expressed a desire for better cooperation, more teamwork and enhanced mutual trust
- DHS caseworkers said a wide array of services is available in most area, but they are often too categorical, and authorized for too limited a duration to meet the family's needs
- Both the department and providers feel the need for more and stable funding in order to assure statewide access to quality services. Department staff indicated there is a need for greater availability of substance abuse treatment, and mental health services for children

and adults. Even in areas where these services are available, the accessibility for adults is limited because funding is directed at treating the rehabilitative needs of the child, not the entire family

- Since early 2000, agency and provider staff focus groups have consistently identified voluntary cases - which frequently have the most trackable and treatable problems - as the most likely to have successful outcomes and not re-enter the system. Case managers and private providers have consistently expressed concern that if the state makes the decision to no longer accept voluntary cases, many more children may be put at risk

Summary

Strengths

- Iowa has many excellent service providers and a wide array of available services for children and families
- The availability of flexible funds and support for initiatives like the Community Partnership for Protecting Children, Decategorization and Empowerment has allowed many communities to develop programs that effectively address the particular needs in their local area
- Attention to ASFA has improved permanency outcomes – especially for younger children, and for children 'new' to the child welfare system

Opportunities for Improvement

- There is uneven service quality and availability of services throughout the state
- There is a need for development and support of after-care programs for children returning home from placement
- There is a need for greater availability of specialized mental health service for children such as day treatment, and group care so that children need not be placed outside of their communities and neighborhood schools to receive necessary treatment. This is especially true of treatment provided only at mental health facilities
- Iowa's service delivery system is highly privatized requiring increased coordination between the service providers and the department to prevent fragmentation
- Services aren't as accessible in rural areas. Access problems may relate to the facts that providers come from offices in other communities, experience travel and time problems maintaining contact with rural families, and may have less knowledge of community resources

Agency Responsiveness to Community

One

1. Discuss how effective the State has been in meeting the requirement to consult and coordinate with external community stakeholders in the development of the State's Child and Family Services Plan (CFSP). In responding, discuss how the concerns of stakeholders are addressed in the agency's planning and operations and their involvement in evaluating and reporting progress on the agency's goals.

Description

The Division of Behavioral, Developmental, and Protective Services (DBDPS) within the Iowa Department of Human Services (DHS) is the organizational unit within the state agency responsible for the development, operation, and implementation of the Child and Family Services Plan. This division works in partnership with the DHS Office of Field Operations and Service Delivery Areas to administer both Title IV-E and IV-B programs under the Social Security Act and to develop and deliver a wide array of services designed to protect children, enhance their well-being, and enable them to achieve successful permanency. DBDPS routinely employs partnerships with Juvenile Court Services, Court Improvement Project, other state agencies such as Public Health, Education, Human Rights- Division of Criminal and Juvenile Justice Planning, Inspections and Appeals – licensing staff and Child Advocacy Board, private provider agencies, and other stakeholder groups in efforts to most effectively plan, implement, and monitor programs serving children and families.

These partnerships are carried out through specific committees and workgroups at both the state and local level, as well as through ad hoc committees developed to address specific issues/programs. Information on the major focus, structure, and activities of these various groups is provided in the following portion of this assessment.

Outlined below is information on various state and local level groups that DBDPS consults with to gather information and feedback to use in program development, implementation, and monitoring. This information is organized by state and local level.

State Level Groups

Child Welfare Partnership Committee
The Child Welfare Partnership Committee is a diverse and broad-based group of stakeholders from around Iowa who have an active investment in service issues for children and families. This group was convened by the department in 2001 to provide input and feedback on a wide variety of service and public policy issues that would be useful in development of the state Child and Family Service Plan. In addition, this group has played an active role in providing input, through its various members, into the Statewide Assessment. The Committee is also reviewing and offering feedback on drafts of the Statewide Assessment report.

Court Improvement Project Oversight Committee (CIP)
Administrators and staff from the DBDPS routinely interface with the courts through the Iowa Court Improvement Project (CIP). The Division Administrator, Mary Nelson, serves on the CIP Oversight Committee. She and Evan Klenk, Waterloo Service Area Manager, meet regularly with state Juvenile Court Judges to discuss emerging child welfare/juvenile justice service and practice issues. CIP and its director, Gail Barber, have been involved as key partners in Iowa's effort to provider information for DHS/JCS staff, judges, provider agencies, and other community stakeholders on requirements mandated under the Adoption and Safe Families Act (ASFA). Using this partnership has been a key strategy of our agency and the courts to increase compliance with the ASFA provisions. Ms. Barber represented the CIP in Iowa's September 2001 Federal IV-E Foster Care review and has actively participated in the subsequent program improvement and training plan. She is also serving on the oversight committee of the Child and Family Service Review Preparation Team. In addition, CIP and DHS collaborated in developing a booklet and training on ASFA, IV-E, and CFSR for the Juvenile Court Services staff. CIP produces a regular electronic newsletter of key activities and updated information, providing an excellent means of coordinating communication between DHS and court officials on topics such as ASFA regulations and IV-E foster care eligibility requirement.

Child Protection Council (CPC)
The Child Protection Council (CPC) is a multidisciplinary group of professionals with knowledge and experience relating to the criminal justice system and the issues of child physical abuse, child neglect, child sexual abuse and exploitation, and child maltreatment related fatalities. The CPC serves as the State Task Force as described in the federal Child Abuse Prevention and Treatment Act. Sec. 107[42 U. S. C. 5106c]. The CPC includes individuals representing the law enforcement community, judges and attorneys involved in both civil and criminal proceedings related to child abuse and neglect (including individuals involved with the defense as well as the prosecution of such cases); child advocates, including both attorneys for children and where such programs are in operation; court appointed special advocates, health and mental health professionals, individuals representing child protective service agencies, individuals working with children with disabilities and representatives of parents' groups.

At three year intervals, the CPC reviews and evaluates state investigative, administrative, and both civil and criminal judicial handling of child abuse and neglect cases, particularly child sexual abuse and exploitation cases, as well as cases involving child maltreatment related fatalities and cases involving a combination of jurisdictions, such as interstate, federal-state, and state-tribal situation. Other activities supported by the work of the CPC include:

- **Multidisciplinary Teams (MDT)**
  Iowa Code Section 232.71 B mandates that counties or multi-counties where more than 50 reports of child abuse are made annually must develop multidisciplinary teams. These teams function as an advisory group to aid DHS workers in resolving issues related to a case both at the assessment stage and during ongoing treatment. Cases. In addition, these teams also provide a valuable source of feedback to department staff concerning the overall quality and effectiveness of our intervention activities in child welfare cases.

- **Citizen Review Panels**
  There are three Citizen Review Panels in Iowa, one statewide panel and two multi-county panels. The membership of the panels is broadly representative of the community and includes members with expertise in the prevention and treatment of child abuse. Current citizen review teams have worked on specific issues such as: the consistency of the child

protection case assessment process, the concept of creating a State Central Intake Unit for child protection cases, and issues around reentry into the foster care system. All three panels help prepare an annual report with their recommendations which is submitted to Health and Human Services (HHS).

Child Death Review Team (CDRT)
In 1995, the Iowa Legislature authorized legislation to create the CDRT. The Department of Public Health convenes the team to review all child deaths. The law establishing the CDRT is found in Iowa Code Chapter 135.43 and the Iowa Administrative Code 641-90 governing the team's operation. The Child Death Review Team is composed of 14 members and 7 state government liaisons. Each member represents a different profession or medical specialty.

This multidisciplinary team conducts case reviews, makes recommendations to agencies represented on the CDRT for changes in policy and practice that may prevent child deaths and makes recommendations to the Governor and General Assembly for interventions to prevent deaths of children in Iowa. As an example, the CDRT made a recommendation to DHS that all registered in-home child care providers be required to learn and be certified in child and infant CPR and receive training on appropriate sleep practices and positioning for infants to prevent Sudden Infant Death Syndrome [SIDS] fatalities. DHS has implemented regulations for in-home childcare providers in these areas as a result of the CDRT recommendations. In addition, the CDRT recommended that registered in-home childcare homes be assessed for the risk of secondhand smoke. DHS has implemented regulations that prohibit smoking within areas where children receive care in registered in-home childcare settings as well as in vehicles in which these children are transported by the provider.

Prevent Child Abuse Iowa
This is the Iowa chapter of the Prevent Child Abuse organization. PCA-Iowa administers child abuse prevention activities funded by the Department of Human Services. In addition, this organization makes recommendations to DHS on policies and practices that could lead to improved safety and well being for children and families. For example, this group has made helpful recommendations to DHS for improvements in training procedures for child abuse mandatory reporters and for improvements in protective responses for children exposed to parental substance abuse.

Juvenile Judges – DHS Liaison Committee
Several years ago, the DHS child welfare director and one of the field managers began meeting quarterly with a group of 8 Juvenile Judges, representing Iowa's 8 judicial districts. Recently, this group was reconstituted to include all 8 Service Area Managers. The group focuses on issues in child welfare and juvenile court practice, with a goal of sharing best practice and improving outcomes for children and families.

DHS Service Area Managers/Chief Juvenile Court Officers (CJCO) Group
For over eight years, DHS local and state child welfare administrators have met, at least quarterly and sometimes monthly, with the Chief Juvenile Court Officers (CJCOs) for each of Iowa's eight judicial districts. The eight CJCOs are responsible for delivery of juvenile court services, including case intake, processing, informal adjustment, probation, and placement/case supervision for juveniles involved in commission of delinquent activities and thus are major stakeholders knowledgeable about service trends and needs within their respective judicial districts.

This group provides a consistent forum in which service trends, budget and funding allocation issues, and program gaps and needs can be identified and dealt with. The work of this group was instrumental several years ago in obtaining funding for Juvenile Justice programs designed specifically to meet the needs of juvenile offenders.

Several years ago, the Legislature assigned responsibility for allocating group care dollars to the DHS SAMs and CJCOs and directed them to use the prior five year's service expenditures, demographics, and other indicators of needs as factors in the funding allocation formula. The SAMs and CJCOs group contracted with the Institute for Economic Development to review factors used by other states in determining funding allocations. They identified the most commonly used factors as: child population and the number of children living in poverty. They also identified related factors such as percentage of minority population and urban density.

The SAMs and CJCO's group first identified a goal of starting with heavy emphasis on historical expenditures, modified by child population, children in poverty, and density; but over time to move away from historical expenditures and toward weighting the demographic factors the highest. However, as child welfare funding has tightened up, the group has returned to giving over a 90% weight to historical expenditures. Generally, the group appoints a subcommittee to annually develop allocation formula alternatives that are then presented to the entire group. The group then reviews, discusses and votes on a final formula. DHS uses this formula to allocate the child welfare appropriation approved by the Legislature and Governor among the 8 service areas.
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The coordination for planning and services between these two groups has allowed more effective deployment of limited resources.

Coalition for Family and Children's Services
This group is a child welfare private provider agency association that represents many of the larger private agencies with whom the department contracts for services to children and families. DHS consults with and seeks input from the Coalition on all policies effecting the provider community. Administrators and staff from the DBDPS regularly discuss general child welfare program and funding issues with the Coalition and also deal with program/service specific concerns on an ongoing basis. Issues often include matters such as: rate setting methodologies, service rule issues around service delivery, documentation, and billing regulations, and issues around staff qualifications and recruitment/retention. Although there is not always consistent agreement between the Coalition's and department's perspectives on issues, ongoing communication provides opportunities to identify concerns as well as enhance areas where consensus exists and better coordinate information sharing and advocacy efforts.

Community Initiative for Native American Children and Families (CINCF)
Both BDPS and the local Service Deliver Area regularly send representatives to meetings of the CINCF group to discuss Native American child welfare issues. The CINCF group consists of several different tribal representatives, tribal ICWA specialists, Native American service providers, and other stakeholders concerned with issues impacting Native children and families, and ICWA. This group has been active in identifying issues and developing proposals for action. The department's ability to participate in their meetings and draw on their expertise has been valuable in increasing awareness of ICWA matters. DHS has also been working with the group over the last year on a proposal for an Iowa Indian Child Welfare Act, which they hope to introduce this year.

Iowa Youth Connections Council
In 2001, the department created a state foster youth advisory council that has current and former foster children among its membership. The purpose of this group is to provide perspectives and information to the department on the needs of children in the foster care systems and how the DHS and other policy makers, and our service providers, can be more effective in helping these children transition to adulthood. DHS supports the Council through a contract with the Youth Policy Institute.

Iowa Foster and Adoptive Parent Association (IFAPA)
The department has contracts with this group to provide foster and adoptive parent recruitment and on-going supports to foster and adoptive families, and retention services on a statewide basis. DHS consults with IFAPA on all policies relative to foster or adoptive parents. IFAPA offers training opportunities and peer support for foster and adoptive families. Ongoing partnerships with this group around providing support for foster/adoptive parents and assisting the department in recruitment/retention activities are perceived as invaluable to the department.

Future Net
The department has a contract with the Iowa Coalition for Adolescent Pregnancy Prevention and Parenting designed to increase public awareness of teen pregnancy and teen parenting issues and to assist in strengthening the collaboration between providers who work with these populations in the state. One significant advantage of this partnership is that the department is able to gain insights from this group concerning interfaces between teen parents and the child welfare system. This information is useful in determining how to address and reduce risk factors that contribute to entry into the child welfare system.

Local Level Collaboration

Child Welfare Decategorization Initiative
Since 1989, the department has administered, and promoted the expansion of, the Child Welfare Decategorization Initiative in which local counties, or clusters of counties, are permitted to develop local planning and governance structures in efforts to modify and redesign child welfare services in order to better address local needs and be more effective in achieving the targeted outcomes of safety, stability/permanency, and well-being. This initiative allows communities to organize planning and governance structures that provide meaningful and ongoing opportunities for community stakeholders to be involved in identifying unmet needs and diverting fiscal resources to needed services. Decategorization has long been a key strategy used by the department to include the voices of external stakeholders in the community level design and operation of the child welfare system. In addition, it has provided a vehicle for moving toward coordinated planning, flexible funding, and more of a focus on prevention/early intervention in the child welfare service arena.

Currently, there are 39 different Decategorization projects in Iowa, encompassing all 99 of Iowa's counties. Each project is required to have a project governance board that must include representatives from county government and Juvenile Court Services, as well as the Department of Human Services and submit an annual Decategorization plan that addresses local needs, collaborative activities with other groups, and focus areas for service or system practice improvement plans.

Projects can add representatives from additional entities to the governance board and some have added representatives from groups such as school superintendents, public health, United Way, or

family/consumer groups. Currently no Native American representatives serve on any of the boards. In areas, such as Woodbury and Tama, counties with higher Native American populations, there have been attempts to reach out to Native American groups to invite their involvement in decat project planning committees but obtaining consistent involvement has been challenging. There are currently some Native American representatives involved in juvenile crime prevention and youth development community planning in Woodbury County. In our PIP we will explore strategies to promote state and local level participation by Native American representatives in child welfare practice and service planning initiatives. .

The state budget problems of the past two years have had a significant impact on the ability of Decategorization boards to make investments in prevention and early intervention programs. In the past, Decategorization projects were able to "carryover" into the next fiscal year unspent money from their child welfare budgets and then use these resources to help support community-based prevention/early intervention programs, enhanced, individualized services for children and families in the child welfare/juvenile justice system, and support for community planning coordinators. During state Fiscal Year 2001 for example, over $17.4 million from "carryover" funds statewide were invested in these types of programs. During the past fiscal year however, most of these "carryover" funds had to be diverted to cover the costs of basic child welfare services and many of the early intervention/prevention programs have been eliminated. In addition, in the past the department has provided significant resources from the child welfare budget for Decategorization projects to hire local community planning/collaboration coordinators. With the current state budget situation and reduced resources for child welfare services, the department can no longer support these positions to the extent that was done in the past. However, in some communities other partners are now sharing in some of the costs of these coordinator positions; DHS is committing a percentage of funding for these positions and the remainder is funded through various community partnership entities.

Community Partnership for Protecting Children - Shared Decision Making Groups
Another example of the department's collaboration at the community level is demonstrated through implementation of the Community Partnership for Protecting Children initiative. Iowa was one of the first four locations selected by the Edna McConnell Clark Foundation in which to support implementation of the Community Partnerships for Protecting Children model. Cedar Rapids was the Iowa location selected by Clark for implementation of this innovative approach to enhancing protection for children. A cornerstone value of this model centers on the creation of a community level shared decision making group, composed of both service providers and community residents, who work together to ensure that needed services and supports are developed to enhance safety for children. This shared decision making group is given real power to evaluate community needs, set service priorities, design and implement plans, and evaluate progress for their child protection enhancement activities.

The Cedar Rapids site has continued their Community Partnerships for Protecting Children initiative and the department has recently expanded the Community Partnerships approach to five additional areas (Spencer, Ames, Waterloo, Charles City, and Ottumwa). In these additional locations, as in the Cedar Rapids site, active shared decision making groups are being formed to develop the vision and action steps for improvements in local systems of protecting children. These groups are building on the energy and local collaborative structures established through both Decategorization and Community Empowerment in order to focus on implementing concrete plans to use community partnerships to enhance protection for children. DHS' goal is to continue to expand to additional sites in the future.

As part of the recent department reorganization, each of the 8 service delivery areas was to have a Community Liaison position that would be responsible for promoting networking and service coordination with other organizations within the area. However, only two service areas filled these positions before funding for staff positions was reduced and the other service areas felt it was more essential to use limited staff funding for front line child welfare positions. While the department recognizes the need to build a stronger sense of community investment and ownership in designing services to achieve safety, permanency, and well-being for children and families, the need to focus reduced resources on the most vulnerable populations places stress on our capacity to participate in community engagement activities.

Statewide Assessment

As noted above, DHS interacts and consults with a diverse variety of stakeholder groups in child welfare program and policy development at both the state and local level. One difficulty over time has been developing a process for better coordinating and organizing these interactions into a more unified and identifiable strategy for stakeholder participation. Many of the identified stakeholder groups have formed and come together to provide input regarding a rather narrow range of issues and programs; the department has not had a consistent process of seeking input from a broad-based stakeholder group on the department's comprehensive IV-B Child and Family Service Plan. It is the hope that by involving the broad-based membership of the Child Welfare Partnership Committee in review of this Statewide Assessment Report for the CFSR, a foundation will be established for their participation in development of Iowa's future Child and Family Service Plans.

During FY02, DHS experienced significant budgetary reductions and staffing disruptions because of the state's economic and budget crisis. Additionally, funding resources for department staff were significantly reduced in FY02, which required initiation of a major department restructuring in December 2001 and January 2002. Major changes were the elimination of the local DHS administrator, Human Service Area Administrator positions, movement from 5 regions to 8 departmental service delivery areas, and the transfers, reassignment, and retirement of numerous staff, as well as significant reductions in staffing at the state level.

These fiscal reductions and reorganization have impacted the recent ability of the department to partner with community stakeholders. Impacts noted in focus groups held by the department this spring and summer, in order to collect information for the CFSR Statewide Assessment Report, have included the following:

- Disruptions in relationships between the department and the community because of staff changes
- Elimination of the local department administrator (HSAA) position has resulted in loss of the recognizable DHS presence for community planning purposes in many communities
- Less time for department involvement in community planning and collaboration meetings Some department staff have been reassigned to jobs they are unfamiliar with
- Increasing pressure to make rapid decisions in response to the budget reductions has at times resulted in the department having to make decisions independently, without the time for dialogue with external stakeholders that could result in greater community understanding of the reasons for specific departmental decisions

Department administrators and service area managers are very aware of the impact that budget pressures, reduced staffing, and the agency reorganization of December and January 2002 have had on the agency's ability to effectively partner with other groups in program development and

service coordination. Their perception is that the department has had to become more internally focused now on the core service and key statutory activities required of the agency; this internal focus has dramatically reduced the department's presence and interaction within local communities and at times been perceived as a "retreat" from previous partnerships with communities. As a result of the department's diminishing involvement in community collaboration activities, there have been some instances in which there has been less community willingness to collaborate or be supportive of the department and its efforts to obtain additional resources to improve its activities. In other instances, the community has really stepped in to compensate for the fact that DHS is less able to provide staff and financial support to community collaboration.

The department has intensified efforts to maintain connections at both the state and local level with major stakeholders. For example, in the past six months regular monthly meetings with Chief Juvenile Court Officers and quarterly meetings with Judges have been reinstated.

In spite of these recent developments, stakeholders who provided feedback for this Statewide Assessment Report did identify a number of existing strengths concerning the department's responsiveness and collaboration with other stakeholders. These strengths included:

- The department has many committed and experienced staff
- Some department staff are still performing community involvement and collaboration activities on a voluntary basis- even when no longer a part of their formal job description
- The department has established a tradition, through Decategorization, Community Empowerment and other similar programs, of supporting community planning for early intervention/prevention programs. While the current constricted budget has reduced the department's capacity to fund programs, a commitment to prevention/early intervention planning, and funding to the extent possible, remains within the organization
- Many department staff have been creative in working with other stakeholders to maximize the limited available funding to continue programs and provide services to meet client needs
- The department is not being complacent or stagnant in its self-evaluation, external input has been sought and the department is aware of areas where limitations exist and is open to exploring innovative solutions to improve performance

Juvenile Court judges noted the positive contributions resulting from this partnership in a May 2002 focus group conducted by the department in preparation for the CFSR. The judges felt that the partnership between CJCOs and DHS SAMs had allowed for greater focus and communication around reasonable efforts requirements and also around issues related to what is required for children to be eligible for federal IV-E Foster Care funding; thus maximizing Iowa's ability to appropriately obtain federal funding.

Summary

Strengths

- Iowa has a history of consultation and coordination with external community stakeholders at both the state and local levels. DHS has remained committed to these partnerships/collaborations, even in times of staff reductions and funding cuts
- Creation of a broad-based stakeholders group, the Child Welfare Partnership, has allowed for comprehensive input for our Child and Family Services Plan
- External community stakeholders have been involved throughout the CFSR statewide assessment process

Opportunities for Improvement
- Increase communication with external stakeholders regarding specific DHS decisions, especially when quick decision have to be made due to budget cuts
- Acknowledge the importance of community consultation and coordination by encouraging staff to participate at all levels and provide the time to do so

Two

2. Discuss how effective the State has been in meeting the State plan requirement to coordinate its services with the services and benefits of other public and private agencies serving the same general populations of children and families.

Description

DHS is routinely involved in partnerships at both the state and local levels with other state agencies and statewide entities that serve the same general population of children and families in order to most effectively coordinate service development, delivery, and program monitoring to achieve the optimum safety, permanency, and well-being results for children and families. Examples of these ongoing partnerships include initiatives with: Department of Education, Department of Public Health, including its Division of Substance Abuse; Department of Human Rights, Division of Criminal and Juvenile Justice Planning; Department of Management, Community Empowerment Office; and Inspections and Appeals, State Foster Care Board and CASA Program.

Department of Education
DHS partners actively with the Department of Education on the "Early ACCESS" Program which is an initiative designed to have multiple state agencies coordinate together their efforts to identify, coordinate, and effectively provide services and supportive resources to families of children birth to 3 years old who may have a health condition that may effect their growth or development, or children that may have delays in their ability to play, think, talk, and move. DHS collaborates with the other involved agencies, which include the Departments of Education and Public Health as well as the University of Iowa Child Health Specialty Clinics, to operate this program. The Division of BDPS provides physical space for the program coordinator and fully supports efforts to train department staff on the benefits and mechanics of this program. The Chief of the Bureau of Community Services serves on the State Steering Committee for the Early Access Program. The Department of Education provides DHS with funding for the coordinator position and the coordinator is part of a six person team that provides education, coordination, and technical assistance for the "Early Access" initiative. The major focus of this team is to enhance local level awareness of the early access program and the numerous benefits it can provide to support children and families. In particular, the program coordinator has been extensively involved in effectively educating DHS child welfare field staff about the program and working to establish local networking patterns so that families in the child welfare system can be linked to this program when appropriate.

Though Iowa's extensive investment in the local planning and collaboration required under Decategorization, DHS has worked to collaborate with local level educational systems. In addition, DHS has promoted collaboration with schools in planning for specific children through innovative practices such as family team meetings and use of the "wraparound" concept of case planning and flexible funding. However, one of the challenge to collaboration in service planning

and case coordination has been Iowa's localized school district structure; there are over 360 different school districts in Iowa that range from very urban districts to rural ones where there are less than 200 students in grades K-12. Each district has its own unique level of resources, especially for children with special needs, and establishing the interagency connections necessary to maximize opportunities for community-based child welfare and educational plans can be time consuming for both school and DHS staff.

Attorney General's Office Juvenile Justice Task Force
The Administrator of the Division of Behavioral, Developmental, and Protective Services serves on the Juvenile Justice Task Force sponsored by the Iowa Attorney General's Office. This task force brings together representatives from the various stakeholder groups involved with juvenile justice issues (Juvenile Court Services, The Department of Human Services, private child welfare/juvenile justice provider agencies, county attorneys, law enforcement, etc) to discuss how intervention coordination and service delivery can be improved in cases involving juveniles in order to achieve better outcomes.

Children's Mental Health Redesign
In FY 2001, the directors of the departments of Health, Education, and Human Services pulled together broad groups of stakeholders to develop recommendations for a comprehensive mental health system for Iowa's children. The group published a report in FY2002 entitled, "Creating a System of Mental Health Services for Children in Iowa." In the report, the group identified recommendations regarding the building public understanding of children's mental health needs, increasing resources for problem detection and treatment, addressing fragmentation in the existing mental health system, and identifying or developing a single point of system accountability. Although redesigning Iowa's children's mental health system will be a multi-year undertaking; an important foundation for this effort has been established through the partnership of state agencies.

Community Empowerment Initiative
The department collaborates with and coordinates programs and services provided under the Community Empowerment early childhood care and education initiative. Community Empowerment is an early childhood care and education initiative authorized by the Iowa Legislature in 1998 in which the focus is on creating partnerships between local communities and state government to improve the school-readiness, health and safety, and stability of children 0-5 years of age and their families. Under the program, state funding for school-readiness programs made available to the state Department of Education is combined with funding from our department for early childhood programs and then distributed in grants to 58 local designated Community Empowerment Areas who use these funds to implement approved local plans to achieve better outcomes for young children and their families. Key features of the program include:

- Technical assistance to the local areas from the Office of Community Empowerment created within the Department of Management and from each of the participating state agencies
- A State Community Empowerment Board that includes private citizens as well as the directors of the Departments of Education, Human Services, Public Health, and Human Rights that provided oversight and direction to the project
- Coordination of local level community planning, service delivery, and program monitoring/reporting between the Community Empowerment and DHS Child Welfare Decategorization programs

Iowa Child Advocacy Board - Department of Inspections and Appeals
The Department of Human Services cooperates at the state and local level with the State Child Advocacy Board which is the entity appointed by the Governor to provide oversight for both the Citizen Foster Care Review Board and the Court Appointed Special Advocate (CASA) programs. At the local level, the work of these two programs provides an additional means of performing child welfare case review (more information about these systemic functions is provided in the Case Review portion of this Statewide Assessment Report). At the state level, the partnership between the department and these entities provides a means of assessing system performance, identifying gaps in services and policies, and an opportunity to work together to improve performance.

Criminal and Juvenile Justice Planning Division - Department of Human Rights
The DBDPS has for over 3 years collaborated and partnered with the Criminal and Juvenile Justice Planning Division [CJJP] - Department of Human Rights in a funding allocation, service enhancement project whereby 3 separate state and federal juvenile crime prevention funding streams controlled and administered by CJJP are allocated to each of the department's 39 different Decategorization projects based on a formula of % of state child population. This funding allows for the coordination of program development, funding, and service delivery among programs focusing on juvenile crime prevention and juvenile intervention issues. This close working partnership has allowed for more effective access for the department's Decategorization projects to federally provided technical assistance available through CJJP on topics such as: community needs assessment, juvenile crime prevention model programs, disproportionate minority confinement issues, and juvenile community restitution and restorative justice programs.

The Division of Criminal and Juvenile Justice Planning also sponsors and staffs a Task Force on Disproportionate Minority Confinement of Minority Youth in the Juvenile Justice System. DHS is represented on this Task Force which is focused on examining the reasons why African American, Latino, Native American, and other minority youth are over-represented in certain levels of Iowa's juvenile justice system. The focus of the group is examining data, exploring causative factors that may contribute to minority overrepresentation, and developing strategies and securing technical assistance to help communities and systems address this issue. Through participation in this initiative, DHS is beginning to examine how Iowa can best address issues minority overrepresentation in the child welfare system; Iowa child welfare data indicates that black and Native American children in some areas are over represented in the child welfare system.

Iowa Collaboration for Youth Development
DHS participates in activities of the Iowa Collaboration for Youth Development project. Iowa is one of 9 states that were awarded five-year discretionary grants in 1998 from the federal HHS-Family and Youth Services Bureau agency to support state level collaboration and community capacity-building activities. The lead state agency for this initiative is the Division of Criminal and Juvenile Justice Planning- Department of Human Rights. More than one dozen state agencies, representatives from several community and statewide youth organizations, local agencies, and research institutions are part of this Collaboration. The goals of the Collaboration are to better align state policies and programs, and to encourage collaboration among multiple state and community agencies on youth-related issues. Additional goals are to promote the use of positive youth development principles in state policies and programs and facilitate the use of effective youth development practices in communities throughout Iowa.

Department of Public Health - Division of Substance Abuse Prevention
DHS works hard to collaborate and coordinate programs and service delivery with the Division of Substance Abuse Prevention of the Department of Public Health. For example, the DBDPS, in collaboration with the Substance Abuse Prevention Division, made funding available to local DHS offices, from the Promoting Safe and Stable Families program, for improving our capacity as the public child welfare agency to better recognize signs and symptoms of parental substance abuse and more effectively respond and develop safety and treatment plans for families in which there is co-occurrence of substance abuse and child welfare issues. This effort was very well received at the local level and has been the catalyst for local child welfare/substance abuse partnerships that have endured even after the funding has been exhausted.

In addition, the department is currently involved in working with Public Health on the following issues specific to substance abuse and child welfare cases:
- Drug testing guidelines (i.e. how frequently to test, which drugs to test for, which types of drug tests are indicated for various drugs)
- Cost efficiencies that could result from joint purchasing of drug testing across Public Health, DHS, and the Department of Corrections
- Confidentiality and information sharing issues between child welfare and substance abuse agencies

The Department of Human Services through the DBDPS has also been involved in an advisory capacity with the Substance Abuse Prevention Division on the State Incentive Grant (SIG) substance abuse program grant award recently received by the DPH from the federal government, which provides funding to local communities to implement evidence- based practices in substance abuse prevention with adolescents. Local grant recipients must demonstrate how community stakeholders across service systems were involved in development of the grant proposal and how grant funded services will be coordinated with other community service systems.

Gender Specific Services Task Force - Iowa Commission on the Status of Women-Department of Human Rights
The DBDPS participates in and supports the efforts of this group to expand the array of adequate gender specific programs for adolescent woman served by or at risk of involvement in, the child welfare/juvenile justice system. This interdisciplinary task force meets on a monthly basis to share best practice resources, provide networking opportunities, help plan an annual Gender Specific Services Conference, and identify how to provide technical assistance to communities interested in improving their Gender Services system. This collaborative partnership has allowed department staff to access additional information and training opportunities on gender specific issues and gender effective services.

Domestic Violence Program Activities
The department is keenly aware of the strong interrelationship between cases of domestic violence and child maltreatment/child welfare issues. To provide for better coordination of programs and services between the child welfare and domestic violence systems, the department has provided training for staff on co-occurrence of domestic violence and child abuse, recognizing the signs and impacts of domestic abuse, and "model protocols" for improving service delivery. Developing greater community awareness about the impact of domestic violence on children is also a major focus of the department's Community Partnership for Protecting Children (CPPC) initiative and the local sites that are currently implementing this approach. As part of the CPPC, the Coalition Against Domestic Violence has set up the capacity

in each CPPC site for a DV advocate to participate in case consultation with DHS workers on child welfare cases. Our goal is to expand this capacity statewide.

In addition, the department participates in the Lt. Governor's Commission To Stop Violence Against Women which is designed to promote awareness and protective activities around this issue.

Transition and Aftercare Services for Youth Exiting Foster Care
The department's Transition Planning Program assists older youth in foster care make the transition to self-sufficiency (adulthood) in addition to overseeing the Aftercare program for former foster care youth between the ages of 18 and 21. Services for youth expected to "age out" of foster care typically begin around the age of 16, with an assessment of the youth's life skills; an individualized transition plan is developed based upon the results of the assessment. Aftercare services, including a case management component and emergency funding for such things as housing and transportation, are available for those youth who have left foster care or a psychiatric medical institution for children at the age of 18 or older, or between the age of 17 ½ and 18 and who had been in care for at least the past 6 months. Coordination with various other agencies, including Iowa Workforce Development (linking youth to the state's Workforce Investment Act program), Iowa College Student Aid Commission (providing financial aid assistance in addition to funding for college scholarships specifically for former foster care youth), county-based mental health service programs for adults, and local community agencies (such as food banks, housing authorities, civic organizations) is vital in linking youth to appropriate resources in addition to assisting with life skills training for youth.

Statewide Assessment

DHS has an extensive tradition of collaborating at the state and local level with other agencies whose program impact the same general population of children and families. This collaborative involvement has increased over the past several years as DHS has recognized the increasingly multi-problem nature of child welfare cases and the need for improved coordination and enhanced comprehensiveness of service responses. Results from focus groups conducted as part of the Statewide Assessment process indicated that the majority of respondents felt DHS made numerous efforts at the state and local level to collaborate in order to achieve more effective program coordination. While the recent DHS restructuring and resultant staffing disruptions have diminished capacity for collaboration, the tradition and priority given to these activities remains within the organization. On the DHS staff survey undertaken as part of the Statewide Assessment, questions 21-39 asked respondents to rate whether they thought DHS worked effectively with specific systems, such as schools, Juvenile Court, law enforcement, substance abuse, domestic violence, housing, etc. within their areas. The response rate for this survey was 496 DHS child welfare workers. On a 5 point scale, with 1 being strongly disagree, 2 being disagree, 3 being neutral, 4 being somewhat agree, and 5 being strongly agree; the overall average ranking for these collaboration questions was 3.54 which indicates that staff feel above average collaboration is occurring. Staff rated the effectiveness of their collaboration with the following groups higher than the average of 3.54 – Juvenile Court, law enforcement, county attorneys, medical community, mental health, and substance abuse providers. Based on the survey results, areas where improved coordination needs to occur include with the public housing system, child abuse prevention programs, economic assistance, and the Native American community.

Summary

DHS engages in a wide variety of partnerships and collaborative planning activities designed to coordinate its services with those of other agencies serving the same general populations of children and families. The needs of the children and families served by the Iowa child welfare system have become increasingly complex and often require supports and interventions from multiple organizations. Given the department's recent reorganization, diminished staff capacity, and reduced funding for services, it will be challenging to maintain prior levels of service coordination and inter-organizational planning.

Strengths

- Iowa has a long history of coordinating services with other public and private agencies, which has facilitate increased communication, shared program development and service coordination
- DHS currently coordinates services with other state agencies, including:
  - Department of Education
  - Attorney General's Office
  - Department of Human Rights- Criminal and Juvenile Justice Planning
  - Department of Public Health
  - Department of Workforce Development
  - Department of Management
  - Department of Inspections and Appeals– Child Advocacy Board
  - Judiciary Department
  - DHS successfully coordinated efforts with other state departments (e.g., Education, College Aid Commission and Iowa Workforce Development) and with counties to develop our programming for transitioning older youth from foster care and for providing aftercare services for youth who have left foster care
  - Coordination occurs at both the planning level and at the individual case level

Opportunities for Improvement

- Increase coordination of services and benefits of other public and private agencies whenever possible – especially Public Housing, and adult mental health services
- Fill Community Liaison positions in all eight Service Delivery Areas when sufficient staff funding is available
- Provide staff time and resources to fully participate in planning and implementation of coordinated services

Three

3. Does the agency have agreements in place with other public or private agencies or contractors, such as juvenile justice or managed care entities, to perform title IV-E or IV-B functions? If so, how are services provided under the agreements or contracts monitored for compliance with State plan requirements or other program requirements and accurate eligibility determinations made, where applicable?

Description

The Department of Human Services has partnership agreements in place with the State Judiciary Department- State Court Administrator's Office to provide the protections mandated by 42USC622(B)(10) . This partnership agreement also provides that the Juvenile Court Services is responsible for obtaining, transmitting and making available to the DHS information sufficient to allow the department to determine client eligibility for the IV-E foster care program and to monitor compliance with state and federal policies for children placed in foster care. The department in this agreement maintains the responsibility of determining the IV-E eligibility of any client being served by Juvenile Court Services who enters placement. Since Juvenile Court Services often maintain case management and planning responsibility for child in the juvenile justice system who access foster care placement services funded by the department, this agreement serves a valuable function within our service system. This agreement was renegotiated in FY 2001.

Under the interagency agreement Juvenile Court Services stipulates that it will provide case management services which include case plans and case reviews compliant with 42USC675(1). Children and families who are being served by Juvenile Court Services are also tracked in the states SACWIS system and the funding for many of the services received by these children is provided by DHS

Juvenile Court Services staff also have access to the same training opportunities as department staff; they also may participate in case reviews and other quality assurance processes available to department staff.

Juvenile Court Services staff have participated with department staff in numerous IV-E related trainings over the past year designed to raise the awareness of IV-E regulations and the importance of the IV-E program. The resulting improvements in communication between Juvenile Court staff and department staff have improved the efficiency and effectiveness of interaction between Juvenile Court and department staff at the local and state level. Juvenile Court Services representatives have been involved in the recent federal review of the Iowa IV-E program and have provided input into the development of the department's IV-E Program Improvement Plan.

The department also has an agreement in place with the Department of Inspections and Appeals (DIA) under which DIA, through the Foster Care Review Boards, conducts foster care review staffings for children in care in approximately 41 of Iowa's 99 counties. Department staff cooperate with the Foster Care Review Boards so that these reviews can be conducted and meet with local and state board officials on a regular basis to assess the effectiveness of the review process.

Statewide Assessment

The collaboration between the department and Juvenile Court Services around the IV-E program, and the focus on the two systems working together to make more accurate eligibility determinations and maintain effective record keeping, was cited as a positive recent development

by Juvenile Court Judges in a May 2002 focus group conducted by the department in preparation for the CFSR Statewide Assessment Report.

Summary

Strengths

- The current agreement between DHS and the State Court Administrator's Office pertaining to IV-E determination is perceived as a positive partnership by staff at all levels in both departments and other key stakeholders. Among other things, it has resulted in increased training and case review activities which have in turn resulted in improved accuracy in IV-E determinations
- Agreements with other public and private agencies have allowed maximization of available funding streams
- Agreements have resulted in increased communication between participating agencies

Opportunities for Improvement

- Continue to use agreements to monitor compliance with state and federal policies for children placed in foster care

Four

4. Citing any data available, discuss how effective the state has been in meeting State plan requirements for determining whether children are American Indian and ensuring compliance with the Indian Child Welfare Act.

Description

Iowa does not have a large Native American population within either the general state population or the child welfare system. Of Iowa's 2,926,324 residents, according to the 2000 Census, only 8989 or .03 % identified themselves as Native American. Major tribes represented in Iowa are the Sac/Fox, Sioux, Santee, Ponca, and Winnebago. Iowa does not have any Native American reservations within its borders.

The department promotes collaboration with the Native American community at both the state and local level.

Several of the Indian tribes do maintain their own social service agency infrastructure and DHS does maintain collaborative working relationships with these tribal agencies. Specifically, the Ponca Tribe of Nebraska operates its own social service and child welfare agency. DHS collaborates with this agency through the tribe's Indian Child Welfare Act Specialist who is active in the Community Initiative for Native Children and Families (CINCF) and also serves as a member of the department's Child Welfare Partnership Committee. The Winnebago and Omaha Tribes in Nebraska also maintain social service agencies and we collaborate with them. These two tribes both have tribal courts that can, and often do, request assumption of jurisdiction of a Native American child's case. The Santee Sioux, Rosebud Sioux, and Oglala Sioux in South Dakota all have tribal social service agencies with which the department works upon their request. These tribes do not have established tribal courts that can assume case jurisdiction. The Sac and Fox Tribe, based at the Meskwaki Settlement in Tama, operate their own social services agency called Meskwaki Family Services. The tribe also operates its own health clinic, Healthy

Start program, youth development program, alcohol and drug abuse services, and senior services at the Meskwaki Settlement. The department, through the Ames Service Area administrator, meets with tribal representatives periodically and coordinates case specific service provisions as necessary. Woodbury County has an office of the Bureau of Indian Education and there is also a Native American Resource Center in Woodbury County. The resource center that provides information and referral services is a private agency, not affiliated with any specific tribe.

In addition, the department has continued to provide, in partnership with the Iowa State Bar Association, University of Northern Iowa, and Native American Issues Committee, training for child welfare service staff on provisions of the Indian Child Welfare Act as well as provide information on Native American cultures. The department has also worked in partnership with Juvenile Court Services and Iowa's Juvenile Judges to build training on the Indian Child Welfare Act into their respective ongoing training activities so that judges and juvenile court officers receive additional information concerning Native American issues within the child welfare and juvenile justice system. The department has also produced and provided to staff an Employees Manual specific to the Indian Child Welfare Act.

More specific information about activities under the Indian Child Welfare Act is outlined in the next section.

Statewide Assessment

Of the total children in out of home placement settings, less than 2% (128 children) are identified as Native American. Of the children receiving home-based child welfare services, approximately 1% are identified as Native American. Of the Native American children in foster family care during the 4/1/02 to 9/30/02 period, 17.2% were placed in Native American homes, most in the foster/pre-adoptive category. As this data indicates, although the numbers are small, Native American children are disproportionately represented in the child welfare system, especially in out-of-home care.

We are not prepared to say we know the answer to the over-representation issues, especially in Woodbury County. We are hopeful of using the information from this statewide assessment, the CFSR onsite visit, and other information gathering opportunities about this issue as a springboard to better address this issue in our Program Improvement Plan. We do believe the following factors may play a role in the Native American representation, especially in Woodbury County:

1) Parental substance abuse issues present within the Native American community
2) Extreme poverty within the Woodbury County Native American community that reduces the resources available for these families and makes them more vulnerable to disruption. The median income for Native American families in Woodbury County is roughly half of that for Caucasians, $20,727 compared to $39,146
3) Patterns of disrupted or distrustful interaction and communication between Native Americans and the child welfare system in Woodbury county may make some families reluctant to trust and engage with the service system
4) Possible isolation from tribal support systems for some Native Americans residing in Woodbury County. In some cases, the tribal support systems are in South Dakota or Nebraska and are not accessible to Woodbury County Native Americans
5) A shortage of Native American child welfare service providers and lack of awareness of Native American issues and legal requirements of the ICWA by non-Native providers.
6) Predominantly single parent families

There are several counties and communities in Iowa that do have a larger Native American population and thus are more focused on the ICWA and other Native American child welfare issues. For example, Sioux City, in Woodbury County. Woodbury County is bordered by South Dakota and Nebraska and Sioux City is an urban community of approximately 80,000 people. This area houses many manufacturing and industrial concerns and attracts Native Americans from various tribes who have relocated to the area for employment purposes. In addition, Iowa is home to the Sac/Fox Tribe of Iowa located at the Meskwaki Settlement in Tama County. The department has worked with Sac/Fox tribal officials to partner more effectively to respond to the needs of the Settlement.

In order to prepare this portion of the Statewide Assessment Report, the department conducted two focus groups with representatives of Iowa's Native American community. Representatives belong to a group consisting of several different tribal representatives called the Community Initiative for Native American Children and Families (CINCF) that meets regularly in Sioux City, Iowa to discuss Native American child welfare issues. The department also offered the Sac and Fox Tribe in Tama County the opportunity to participate in a focus group. A summary of themes from these focus groups follows:

- Iowa's child welfare system has many good staff people who care about Native American issues but the recent DHS reorganization has disrupted many partnering relationships that had been established over time
- Child welfare staff should receive more training on the Indian Child Welfare act and the philosophy behind this legislation as well as the specific requirements under the law
- The Department of Human Services should be more proactive in taking advantage of opportunities to partner with Native American groups
- It is extremely important to the Native American community that there be active efforts undertaken by the child welfare system to preserve the cultural and tribal connections of Native children who are placed within the child welfare system
- There is an urgent need for the department to make active efforts to recruit more Native American service providers into the child welfare system. This need for additional Native American providers is necessary at every level of the child welfare service delivery system, from foster and adoptive parents, to child welfare agency staff, and contracted service providers of both home-based and out- of- home child welfare services

The two Native American focus groups identified several issues around identifying children and notifying their respective tribes. These included:

- The department should improve its ability to provide tribal notification within the first 24 hours of placement as this is the most critical period in which intensive crisis intervention services could possibly be provided to prevent continued placement
- The department should develop and deepen partnerships with tribes to promote access to intensive front–end services that might prevent placement (perhaps something like specialized family preservation type programs for Native American families) and do the up-front assessment work that could identify potential kinship placement options for the children at risk of placement
- The department should develop culturally specific materials that inform Native American families at the time of their child's removal of their rights under state and federal law-specifically under the ICWA

Department staff provided feedback on barriers to identifying and providing tribal notification for Native American children placed in care. These comments were:

- The department should work with the Bureau of Indian Affairs to clarify the process, and also improve the timeliness of responses, in situations where department staff place a Native child but do not have adequate information from the child's parents concerning their tribal affiliation to promptly make tribal notification. In these situations, the department has to send the information we have to the Bureau of Indian Affairs and ask for their assistance. This process is often slow and not responsive to the permanency needs of children in the child welfare system
- Obtaining prompt responses from tribes concerning their decisions on intervening in a family situation is necessary; delays in these decisions present barriers to stability and permanency for children
- The focus group identifies the following suggestions for improving compliance with the Indian Child Welfare Act
- Improve partnerships with Native American agencies to provide crisis intervention services to stabilize families before placement is necessary
- Use tribal connections more effectively to identify and support kinship care arrangements that may be appropriate for Native American children
- Evaluate the causative factors behind the disproportionate number of Native American children in foster care and develop strategies to improve these statistics
- Attend more effectively to the safety of Native children placed in foster care settings
- Support efforts by law enforcement agencies to improve relationships and trust between law enforcement and the Native American community. Improving this connection could increase the effectiveness of child welfare early intervention/crisis intervention services. The bulk of activities to achieve this improvement will need to originate from law enforcement agency planning efforts
- Plan to address barriers to increasing the number of Native American foster homes, such as: Native distrust of the DHS system, foster care licensing standards that may discourage Native families from consideration, and perceptions that the department may remove children without provocation. Specialized recruitment efforts for Native American families are important and should be enhanced
- Work with the State Judiciary system and others to improve the quality of legal representation for Native American children and parents within the child welfare system
- Promote the development of a Tribal Court system for Native American children in Iowa
- Provide more data on the number and characteristics of Native American children in the child welfare system to tribal representatives
- Work with the Native American community to develop agreed upon model service protocols and performance accountability standards regarding Native American children
- Examine the requirements of both the ICWA and ASFA and work to resolve any conflicting demands and requirements
- Embrace a policy under which the child welfare system develops and implements a Cultural Connection plan for Native children in out-of –home care

496 DHS child welfare workers were asked to respond to 2 survey questions related to working with Native children and ICWA compliance. The percentage of those responding to each of the 5 available responses is indicated on the following chart.

Survey Questions				
I routinely ask children and families if they have Indian heritage or are affiliated with a tribe.				
Strongly disagree	Disagree	Neutral	Agree	Strongly agree
13.1%	19.5%	26.2%	26%	15.2%

I understand the requirements of the ICWA.				
Strongly disagree	Disagree	Neutral	Agree	Strongly agree
14.2%	0%	28.7%	44.9%	12.3%

People working in my Service Area work effectively with the Tribal community.				
Strongly disagree	Disagree	Neutral	Agree	Strongly agree
16.2%	15.7%	53.3%	12.7%	2.0%

The focus group identified the following suggestions for strategies to increase state child welfare system responsiveness to Native American communities.

- The department should prepare regular special mailings concerning major child welfare issues to the two active Native American groups working on modifying ICWA legislation
- The department should actively partner with tribal ICWA specialists in efforts to: design, implement, operate, and evaluate programs for Native American children and families
- Develop effective procedures for promoting involvement of ICWA tribal specialists in case specific service teams and service plan development
- Continue the pattern of participation by department administrative staff, or their designees, in quarterly meetings of the active Native American child welfare committees
- Promote the development of a stronger Court Appointed Special Advocate (CASA) program to advocate for the best interests of Native American children in the Juvenile Court system
- Partner with Native American groups and current Native service providers to develop strategies to recruit more Native American child welfare service providers

Summary

We do not have a good overall picture of our current child welfare practice related to Native American children. We have begun the process of getting a better handle on what our practice is by continuing dialogue with Native American groups and soliciting their input and ideas for improvement, making some changes to improve practice, and are open to using technical assistance on this topic to guide improvement. From reviewing focus group comments and talking to specialists in field support, we have some basic assumptions on our practice that will be reviewed as we develop our Program Improvement Plan. These assumptions are:

1) DHS staff have become more aware of ICWA through trainings put on through DHS over the past few years.
2) We can improve our identification of Native American children entering care by making this a higher priority for staff. For example, the new case plan for out-of-home cases asks staff to document the date they asked the child and parents if they are a member of a federally recognized tribe.
3) DHS staff experience some frustration and confusion in carrying out the tribal notification process for children claiming Indian heritage, especially when the family may have limited information and the appropriate tribal contact information may be hard to locate. The new case plan will require documentation of tribal notification efforts.

4) There has not been a consistent practice of making diligent efforts to seek relative placement options for Indian children or efforts to place them with members of their tribe. No QA process exists to review this or other ICWA requirements in our practice. Again, the new case plan will require workers to document efforts, justifications if not placed with extended family or tribal community, and information on strategies to maintain the child's connections with family and tribe.
5) Our ability to effectively follow ICWA is impaired by our shortage of Native American foster families and child welfare service providers. In addition, our staff's ability to obtain specific consultation on Native American cultural issues is not readily available.

Strengths

- DHS promotes and participates in collaboration with the Native American community at the state and local level
- DHS has provided staff an Employees Manual specific to the Indian Child Welfare Act
- The BDPS Division Administrator participates in quarterly meetings of the Community Initiative for Native American Children and Families (CINCF)
- DHS plans to use feedback received through the CFSR process to develop plans for specific actions to broaden partnerships and improve the service system for Native American children and families
- Targeted recruitment efforts, especially in Woodbury County, to increase the pool of Native American foster and adoptive parents
- Supported ICWA tribal specialists in their efforts to develop a Native American Foster Parent Licensing Handbook
- Developed and delivered training on the ICWA for judges, Juvenile Court Services staff, DHS workers, and others through a collaboration with the University of Northern Iowa, and the Iowa State Bar Association
- Worked with the Community Initiative for Native Children and Families (CINCF) organization to incorporate the federal ICWA provisions into a proposed Iowa child welfare statute. A legislative proposal to incorporate these provisions was introduced in the 2002 Iowa legislative session but not adopted. CINCF plans to introduce this proposal again in the 2003 legislative session.
- Explored developing the capacity to institute tribal-state agreements that would enable tribes to provide some child welfare services under contract with the department

Opportunities for Improvement

- Provide more in-depth training on the Indian Child Welfare Act (ICWA) to DHS and Juvenile Court Services staff and judges
- Reduce staff turnover to allow for optimum awareness and implementation of the ICWA
- Work to restore partnerships that were disrupted by DHS reorganization

Increase effort to recruit Native American providers, including foster and adoptive parents, child welfare staff, and contracted service providers

- Work to preserve the cultural and tribal connections of Native children in the child welfare system
- Develop culturally specific materials to inform Native American families of their rights under state and federal law and the ICWA
- Consider re-establishing targeted recruitment efforts, especially in Woodbury County, to increase the pool of Native American foster and adoptive parents - the foster care recruiter position dedicated to working with Native groups was eliminated over a year ago as a result of budget cuts

Foster and Adoptive Home Licensing, Approval, and Recruitment

One

1. Discuss how effective the State has been in meeting the requirement to establish and maintain standards for foster family homes, adoptive homes, and child care institutions in which children served by the agency are placed.

Description

Since 1981 Iowa has maintained comprehensive standards for foster family homes, adoptive homes, and group care facilities. The Code of Iowa provides the legal framework for DHS administrative rules and policy that is maintained in the Employees' Manual. These standards are enforced to ensure the safety and well-being of children in care.

Foster family home licensing rules, adoption home approval rules and residential care facility licensing rules are routinely developed and revised, as needed, to reflect changes in state and federal laws and to implement policy initiatives. Following is a brief summary of DHS standards and procedures.

Foster Family Home Licensure : DHS has the overall responsibility for and is the licensing agent for all foster families within the state. Foster family foster care applicants are assisted with the licensing process (completion of licensing home study) by DHS staff or staff from licensed child placing agencies that have purchase of service contracts with DHS to provide foster care services.

Initial foster home licensure requires:

- A completed home study. The home study contains an assessment of the prospective foster parent's personal characteristics, child rearing attitudes and understanding of adjustment issues that may affect the family when a placement is made. Information for the licensing home study is obtained from:
  - Personal interviews with the applicant in the applicant's home
  - Reference checks (solicited and unsolicited)
  - Criminal record checks, child abuse record checks, adult abuse record checks, sex offender registry checks
  - Physicals
  - Home health and safety inspections. The safety inspection includes a review of water quality (private water supply) firearm locks, ventilation, fire safety, written safety plans (fire, tornado, blizzard) etc.
- 30 hours of approved pre service training (effective January 1, 2003) using PS-MAPP (Partnering for Safety and Permanence: Model Approach to Partnerships in Parenting). A more detailed description of the initial pre-service training curriculum is found in section D of this report
- Completion of the one-hour individual study module, "Universal Precautions in Foster and adoptive Homes

A foster family foster home license is issued for one year. A provisional foster home license may be issued for up to 90 days when lack of access to pre-service training delays the

issuance of a full license and the home is needed for placement. A provisional license may also be issued to allow applicants to correct unmet licensing standards that do not impact child safety. Inspection and evaluation prior to issuance of a full license ensure standards for child safety are met. Foster parents licensed at the basic level are required to complete 6 hours of in-service training prior to each annual renewal. Before re-certification as a treatment foster home, licensed foster parents are required to complete 6 additional hours of training in addition to the basic in-service 6-hour requirement.

During the licensing period, prior to renewal, at least one unannounced visit is made by the licensing worker to assess the quality of the living situation and to determine compliance with licensing standards. If concerns are identified during the unannounced visit that do not compromise the safety of the child, the licensing worker may develop a written plan with the foster parents to remedy deficiencies.

If it is determined at any time that the home presents an immediate threat to a foster child placed in the home, procedures are in place to notify workers who have children in placement, as well as to immediately suspend, and revoke the license.

Information regarding DHS foster care policies and procedures is available for foster parents, DHS licensing workers and private staff in **Employees' Manual, 12- B, Foster Family Home Licensing and Appendix.** Policy and procedural information is also available in the **Foster Parent Handbook**. The Iowa Foster and Adoptive Parents Association distributes the Handbook, to each new foster parent when the license is issued. Employees' Manuals and the handbook are also available on-line at www.dhs.state.ia.us.

Relative Care Homes: Relatives and non-relatives are subject to the same licensing requirements described above. Currently, DHS policy does not provide for placement in unlicensed relative homes if DHS has custody, although it does occur is some situations, e.g. in emergency removals or pre-adjudicatory situations. Relatives must be licensed to receive foster care payment. If the Juvenile Court wants to place a child with an unlicensed relative the Court usually gives custody to the relative and directs DHS to supervise the placement. To ensure safety in unlicensed relative placements, relatives may be required to meet child protection standards prior to placement. Such standards include the completion of a criminal record check, child abuse record check, adult abuse registry check and sex offender registry check. In some situations the Juvenile Court may request DHS to complete a relative home study. A relative home study is not mandated in state statute. Guidelines for identifying potential relative placements are included in the Employees' Manual.

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Adoptive Homes: A pre-placement investigation is required for all individuals and families interested in being approved for adoption**. Code of Iowa, Chapter 600.8, Placement Investigations and Reports**, specifies the areas that must be addressed in the investigation. The purpose of the investigation is to determine whether the prospective adoptive individual or family is suitable for placement of a minor child. Mandated areas of investigation include criminal record checks, child abuse registry checks and adult abuse registry checks. The pre-placement investigation is also required by the Code of Iowa to include an assessment of prospective adoptive individual and families' emotional maturity, health, finances and relationships. The results of the investigation are documented in a written report, the Adoptive Home Study and Adoptive Home Study Update. The home study format used for the adoptive home study is the same format used for foster home licensing.

The Court may waive the preplacement report for relatives within the fourth degree of biological relationship. Adoptive home studies must be updated annually until the individual or family receives a placement.

State statutes specify that the preplacement investigation and adoptive home study can be completed only by DHS staff, licensed child placing agencies staff, or an adoption investigator certified by the State.

Group Care Facilities: Group care facilities in Iowa are categorized as follows:

- Shelter Facilities
- Community Residential Facilities
- Comprehensive Residential Facilities
- Community Residentialfor Mentally Retarded Children

DHS is the licensing agent for all group care facilities. Effective July 1, 2002, state law provided that licenses for all facilities are issued for three years except shelter facilities that are licensed for one year.

The standards for the group care facilities listed above are located in respective DHS administrative rule chapters. Group care facility rules specify administration, staff qualifications, staff ratio requirements and corrective action procedures. Other mandatory requirements include abuse and neglect reporting, discipline, education, physical care, nutrition, privacy, religion, confidentiality, culture, environmental safety, guns and ammunition restrictions.

State statutes and department rules that regulate the licensing and approval of foster care facilities and adoptive homes are listed below.

Code of Iowa References:

- Chapter 237, Child Foster Care Facilities
- Chapter 600, Adoption

Iowa Administrative Code References, Section 441:

- Chapter 105, County and Multi-county Juvenile Detention Homes and County and Multi-county Juvenile Shelter Care Homes
- Chapter 108, Licensing and Regulation of Child Placing Agencies
- Chapter 112, Licensing and Regulation of Child Foster Care Facilities
- Chapter 113, Licensing and Regulation of Foster Family Homes
- Chapter 114, Licensing and Regulation of all Group Living Foster Care Facilities for Children
- Chapter 115, Licensing and Regulation of Comprehensive Residential Facilities for Children
- Chapter 116, Licensing and Regulation of Residential Facilities for Mentally Retarded Children
- Chapter 200, Adoption Services
- Chapter 202, Foster Care Services

Statewide Assessment

Iowa has effectively established and maintained standards for foster homes and group care facilities. These standards reflect welfare practices that have been shown to improve the outcomes for children and families.

A December 2002 Iowa Child Death Review Team Report to the Governor and General Assembly recommended that all foster care parents be required to learn and be certified in child and infant CPR and that they be required to be re-certified in this procedure annually. In addition, they recommended that foster parents should be required to have extensive education regarding appropriate sleep practices and environment for infants; and that their homes should be assessed for secondhand smoke exposure and safety before they are accepted into the foster care program.

Foster and adoptive parents that participated in focus groups indicated that DHS has rules and regulations in place to ensure the safety of children in placement.

Under a Governor's Executive Order, all state agencies reviewed their administrative rules in the last two years against five criteria – need for the rule, clarity, intent and statutory authority, cost to implement, and fairness. As part of the process, each department solicited feedback from providers, other state agencies, and constituents. In general the feedback indicated that the rules effectively met the criteria. Revisions to specific chapters were recommended to improve clarity and program consistency. State departments are required to conduct this review process every 5 years. DHS received 7 responses to our request for feedback on our foster family licensing rules, and 9 responses to our request for feedback on our group care licensing rules.

Following are the specific suggestions that were received with respect to family foster care licensing standards.

- Modify physical standards to require separate beds and available bedroom space for older adolescents
- Enhance fire safety requirements
- Raise the minimum age for applying to be a foster parents
- Add a requirement that firearms and ammunition needs to be locked and stored separately
- Enhance training and add more required courses. Require mandatory reporter training prior to licensure (As noted above, DHS is implementing expanded pre-service training for foster and adoptive parents)
- Fingerprint all applicants and check all out-of-state records for persons who have moved to Iowa and apply to be foster parents
- Allow more flexibility for DHS employees to be foster parents, and provide more specific guidelines for when private agency staff can be foster parents
- Review license capacity (DHS has recently revised rules to delegate decision-making about increasing license capacity in individual case situations to Service Area Managers)
- Mandate orientation at the beginning of the licensure process
- Standardize forms for reference checks
- Enhance rules for treatment foster homes
- Revise rules to allow closure of homes that are not positive resources

Following are the specific suggestions that were received with respect to group care licensing standards.

- Combine the chapters for different levels of group care into one chapter
- Separate out basic safety standards for licensing, and move requirements related to programming and services under another chapter
- Clarify policy regarding sending medications home from facilities when children leave or go for home visits
- Revise fire safety standards to reflect new information and technology
- Provide more time (up to 30 days) for facilities to get children to dentists and doctors for physicals

Summary

Strengths

- Iowa has established and maintained standards for foster family homes, adoptive homes, and child care institutions in which children served by the agency are placed
- The same home study format is used for prospective foster and adoptive parents. This avoids duplication of effort for workers and foster and adoptive applicants and this practice is especially beneficial to applicants who are interested in being both foster and adoptive parents
- Beginning January 1, 2003, prospective foster and adoptive applicants will be prepared (trained) together using the 30-hour PS-MAPP curriculum. It is anticipated that this practice will lead to the development of resource families that are available to meet the needs of children who are a various points on the child welfare continuum

Opportunities for Improvement

DHS does not currently have consistent standards for non-licensed relative placements. There is variance in different areas of the state as to how relative placements are initiated and managed. This has an impact on the degree to which family connections are maintained and the quality of care children receive in non-licensed relative placements. A preliminary DHS work group was formed in September 2002 to study relative care issues and develop a plan to address the issue

- Implementation of PS-MAPP provides an optimal opportunity to move toward a system of dual licensure for prospective foster and adoptive applicants. Even though the same home study format is used for family foster care and adoption there is a difference in other requirements i.e. space and safety requirements
- Implement CDRT recommendation to require foster parents to:
  - learn and be certified in CPR
  - have education regarding appropriate sleep practices
  - have education regarding appropriate environment for infants
  - have their homes assessed for secondhand smoke exposure and safety

Two

2. Citing any data available to the State, discuss how effective the State has been in meeting the State plan requirement to assure that the State's licensure standards are applied to all foster and adoptive homes and child care institutions that serve children in the State's care or custody?

Description

Comprehensive standards for family foster family homes, adoptive homes, and group residential care facilities are defined in the Code of Iowa and DHS Administrative Rules. The specific cites are listed in Item #1 of this section.

Family foster home standards are applied equally to all licensed homes. As part of the licensing process, the worker completing the licensing study completes the Foster Family Survey Report, which documents the foster family's compliance with all licensing requirements. This form is submitted to and reviewed by the Service Area Manager (or designee) prior to issuing the initial license or license renewal, in order to ensure that all licensing standards are met.

To document licensing activities, files on all licensed family foster homes (including relatives) are maintained in the DHS office for the county in which the family resides. The file contains:

- a copy of the current license,
- home study,
- home study updates,
- physicals,
- reference reports,
- results of health and safety inspections,
- registry checks,
- record check evaluations (whenever there is a positive record check)
- training reports, and
- unannounced visit reports.

When a family foster home license is issued the DHS licensing worker enters demographic and license status into FACS. The results of the unannounced visits are also entered into FACS. Local offices receive a monthly report of licenses that are coming due for renewal, as well as any licenses that are overdue for renewal. The department also monitors the status of family foster care licenses on a monthly basis at the state level.

Adoption. Adoption approval standards are applied equally to all prospective adoptive individuals and families. Adoption approval activities (home study, physicals, references, record, checks) are maintained in the DHS office for the county in which the family resides. As stated in Item #1 of this section, the Court may waive the pre-placement investigation and report for relatives within the fourth degree of biological relationship.

Group Care Facilities. To ensure that licensing standards are equally applied the Department of Inspections and Appeal (DIA) routinely inspects all group care living facilities licensed by DHS. After inspecting a facility, DIA completes an Evaluation and Recommendation for Foster Group Care License form, which documents whether the facility has met all licensing standards, and provides this form to the Department of Human Services. The Chief of the Bureau of Protective Services makes the licensing decision based on the information in this form and any attached reports.

Approximately three years ago, DHS hired a full time central office staff person to track the status of each group care living facility license and maintain and update the data base. Prior to that time licenses and renewals were not processed timely and consequently many provisional licenses were issued.

Exceptions to Licensing Standards. The department has administrative rules that allow the agency director to make exceptions to specific rules in individual cases. Following are the criteria used to grant an exception.

- Failure to grant the exception will result in undue hardship
- The exception will not substantially affect another person in an adverse manner
- The exception in not prohibited by state or federal law
- The exception will not endanger public health, safety, or welfare
- There are exceptional circumstances justifying an exception to the general rule applicable in otherwise similar circumstances

In calendar 2002, the Director granted a total of 79 exceptions to the licensing rules for family foster care (74 of those related to licensed capacity) and 2 exceptions to the licensing requirements for group care.

In September 2002, there were **2,752** licensed foster homes (including licensed relatives). Each month approximately **50** foster family home licenses are revoked or closed for a variety of reasons. In September 2002 there were **880** approved adoptive homes. At the end of August 2002 there were **150** licensed group care living facilities and **157** licensed child-placing agencies.

DHS administrative rules require mandatory revocation of a foster family home license when the following circumstances exist:

- The foster parents or any person residing in the foster family home (other than a foster child) has been convicted of a crime unless an evaluation of the crime has been made by the department which concludes that the crime does not merit prohibition of licensure
- The foster parent or any person residing in the foster home (other than the foster child) has a record of founded child abuse reports, unless an evaluation of the founded abuse has been made by the department which concludes that the abuse does not merit prohibition of licensure
- The foster parents are operating without due regard for the health, sanitation, hygiene, comfort, or well being of the child in foster care
- The foster parents are misusing the funds furnished by the department

In addition to the mandatory reasons for revoking a license, a foster family home license may be revoked for the following reasons:

- The family fails to continue to comply with all the licensing requirements in both law and regulation
- The foster family fails to meet one or more requirements in the placement agreement
- The foster family fails to notify the licensing worker of a move to another home within 30 days after the date of the move
- The foster family refuses to cooperate with an unannounced visit

IFAPA liaisons attempt to contact families who have exited foster care. Families unable to be reached by the liaisons are mailed a questionnaire to complete regarding why they left the foster care system.

The most recent Foster Family Exit Questionnaire Report was published in December 2002 and covers the time period October 1, 2002 through December 31, 2002. During this time period, 127 foster families exited the foster care system and 30 of those foster parents completed the questionnaire. This sample though limited provides a glimpse of the issues that may affect a foster parent's continued participation in the program.

Personal reasons is the most frequent explanation (13) given on the questionnaire for leaving foster parenting. Personal reasons sited include:

- moved to another state
- adopted children
- illness
- retired
- no more room due to adoptions.

Frustration is the next most frequent reason (6). Sited reasons for frustration include the following:

- biological parents given too long to change
- not contacted for placements
- infrequent contact with DHS worker
- worker was not honest when the child was placed
- reimbursement for child care never received

Statewide Assessment

The standards that DHS established are equally applied to foster family homes, adoptive homes and group care facilities.

Foster and adoptive parents that participated in a focus group indicated that the current training (which was 12 hours in length and based on the NOVA curriculum) does not prepare families for the children that need placement and inappropriate families are not always weeded out. Families expressed hope that PS-MAPP will be an improvement.

Strengths

- Iowa's licensure standards are applied to all foster and adoptive homes and childcare institutions that serve children in the State's care or custody
- Iowa has a process in place to grant exceptions to specific licensing standards in individual situations
- The department monitors that family foster care and group care licenses are up to date

Opportunities for Improvement

- Consideration should be given to implementing dual licensure for foster and adoptive parents. This could create a more seamless process and may lead to the development of "resource families" which is emphasized in PS-MAPP preparation for foster and adoptive families

Three

3. Citing any licensure or safety data available to the State, discuss how effective the State has been in meeting the State plan requirements to conduct criminal background clearances on prospective foster and adoptive families, including those being licensed or approved by private agencies in the State. How does the State address safety considerations with respect to the staff of childcare institutions and foster and adoptive families I if the agency has opted not to conduct criminal background clearances in foster and adoption families?

Description

Iowa requires state criminal record checks, child abuse registry checks, sex offender registry checks and adult abuse registry checks on all prospective foster and adoptive families as well as all individuals over the age of 14 residing in the applicant's home. The checks must be completed before a foster home license is issued or the home study is approved. As stated previously in this section, DHS is the licensing authority for all foster families and is the approval authority for families that adopt special needs children. At the time of application all prospective foster and adoptive families are asked to sign a statement certifying the following:" I have not been convicted of a crime or have a founded abuse report in any other state and do not have knowledge that any other adult living in my home has been convicted of crime or have a founded abuse report another state". Based on information obtained from prospective foster and adoptive families and other sources, record checks may be conducted in other states. National criminal record checks are not required but may be sought when the worker knows families have lived in other states. In Iowa fingerprinting is not required.

In situations where relatives provide care for children and are not licensed foster parents, the Court may request criminal background checks and child abuse checks. When the pre-placement investigation and report is waived for prospective adoptive parents criminal background checks and child abuse checks may be requested by DHS or by the Court.

Foster parents must complete a state criminal record checks, child abuse registry check, and sexual offender registry checks and adult abuse registry check at initial licensure. A repeat of record checks is optional for foster home license renewal. Adoptive parents must undergo:

- a state criminal record check,
- child abuse registry check,
- sexual, offender record check, and
- an adult abuse registry check at initial approval and annually when the home study is updated.

If a foster or adoptive parent, or an individual in the home, has a positive record check, the department evaluates the record check against the following criteria to determine if the abuse or conviction “warrants prohibition of licensure”.

- Nature and seriousness of the crime or abuse in relation to the position
- Time elapsed since the commission of the crime or abuse
- Circumstances under which the crime or abuse was committed
- Likelihood that the person will commit the crime or abuse again

- Number of crimes or abuse committed by the person involved.

To achieve compliance with the Adoption and Safe Families Act of 1997 (ASFA), the General Assembly amended the Code of Iowa to prohibit foster parent applicants and adoptive applicants from being licensed as foster parents or approved for adoption, without recourse, if specific felonies offenses were committed. An evaluation of the criminal record is not allowed in those situations as is the case with other crimes and founded abuse reports.

Documentation regarding the results of criminal background checks and child abuse record checks is located in foster family licensing files and adoptive parent files. In order to ensure that record checks are completed, the Service Areas have implemented procedures requiring that the record checks and findings must be attached to the licensing material in order for the Service Area Manager (or designee) to make the licensing decision.

Group Care Facilities. Criminal record checks and child abuse checks are required, prior to employment, for persons employed at group care living facilities and licensed child placing agencies. The record checks are repeated if the person becomes employed at another facility. If there is a positive check, the department is required by statute to evaluate the crime or abuse according to the same criteria as for foster and adoptive families.

Group care licensing facilities maintain criminal background checks results in agency files that are reviewed regularly by the Department of Inspections and Appeals.

Statewide Assessment

The assessment determined that the process for conducting background clearances on prospective foster and adoptive parents is generally effective. Foster parents that participated in focus groups stated that the scrutiny (background checks, rules and regulations, etc.) foster and adoptive parents experience undergo as part of the licensing and approval process contributes to the safety of children in care. One concern raised by group care providers was around delays in getting the criminal history information for potential employees.

As noted earlier in this section, there have been suggestions that DHS fingerprint all applicants and check all out-of-state records for persons who have moved to Iowa and apply to be foster parents.

Summary

Strengths

- The process for conducting and evaluating child abuse and criminal history background checks is effective

Opportunities for Improvement

- The process could be improved by instituting a policy to require annual criminal background checks and child abuse checks on foster parents in the same manner as for adoptive parents
- Fingerprinting and conducting national check of foster and adoption could improve safety for children, although a lack of funding would be a barrier to implementation

- Group care providers are working with the Department of Public Safety to enable them to access the criminal history records electronically

Four

4. Citing any data available to the State, discuss how effective the State has been in meeting the State plan requirement to recruit and retain foster and adoptive families that represent the ethnic and racial diversity of children in the State for whom foster and adoptive homes are needed, including the effectiveness of the State's official recruitment plan.

Description

In April 2002 DHS entered into a contract with the Iowa Foster and Adoptive Parents Association (IFAPA) to recruit and retain foster and adoptive families state wide. Prior to that time foster and adoptive family recruitment was administered separately through a contact with the Coalition of Family and Children's Services in Iowa for adoption recruitment and a contract with IFAPA for foster family recruitment and retention. **KidSake** is the logo that was initially associated with adoption recruitment, and now represents both foster family and adoptive family recruitment. The contract with IFAPA sets specific targets and ties financial incentives to performance goals related to actual increases in the number of licensed foster families and approved adoptive families.

DHS is actively expanding recruitment efforts through the KidSake Project managed by IFAPA to recruit foster and adoptive families for children with special needs. The contract with IFAPA sets specific targets and ties financial incentives to performance goals related to actual increases in the number of licensed foster families and approved adoptive families.

DHS is actively expanding recruitment efforts through the KidSake Project managed by IFAPA to recruit foster and adoptive families for children with special needs including foster and adoptive families that represent the ethnic and racial diversity of the children for whom foster and adoptive homes are needed. To address the on-going need for minority foster and adoptive families, targeted recruitment campaigns and events have been conducted on a statewide basis and in areas of the state with large minority populations. Recruitment brochures for African American and Hispanic (in Spanish) have been developed, and training leaders are available as needed. Each of the area recruitment teams is expected to develop a plan for minority recruitment to reflect the specific needs of the area.

Efforts are underway to develop and support recruitment teams in each of the DHS service areas to develop recruitment strategies to locate foster and adoptive families in specific geographical areas. The teams are composed of local DHS and private staff and adoption advocates. Each team develops a yearly plan that includes targeted recruitment activities for specific children in the area that need foster and adoptive homes. Following are examples of successful recruitment and retention strategies that are being used:

- Public awareness events - adoption fairs, media events (TV and radio public service announcements, mews releases, newspaper articles), county and state fair booths, mall exhibits

- State and local recognition events (dinners, awards,) for foster and adoptive parents including annual foster care and adoption month activities
- Coordinates with the Governor's Office for a yearly recognition event for foster and adoptive families
- Publication of the foster parents parent newsletter, News and View, six times yearly, and the quarterly newsletter, KidSake Connection, that contains articles about adoption and foster care and photos of waiting children
- Partnerships with the Iowa Citizen's Foster Care Review Board, the Dave Thomas Foundation, Farm Bureau Women's Association, Iowa Photographers Association, Wendy's franchises, and other child advocates
- Publication of the photo listing book, "Iowa's Waiting Children" and a variety of educational materials
- Creation and maintenance of adoption and foster care web sites with links to other adoption and foster care information sources
- Registration of waiting children on the state adoption exchange, state internet photo listing and national photo listing
- Initial orientation and referral to pre-service training and to DHS for the initiation of the home study process. IFAPA and DHS track on a monthly basis the progress of recruited families from initial orientation through placement

In addition to the above activities, DHS contracts with IFAPA to deliver support services to foster and adoptive parents that are designed to improve retention. The support offered by IFAPA include the following:

- Welcome packets are distributed to all newly license foster parents and identified adoptive parents within 30 days of notification from DHS
- Annual grants are provided to foster and adoptive parent support groups that are affiliated with IFAPA and assist in the development of support groups in areas where they do not currently exist
- A toll free information and referral telephone line is maintained for foster and adoptive parents
- Foster and Adoptive Parent Liaison who are IFAPA contract employees are available in each of the state service areas to provide on-going assistance to foster adoptive families about policy issues
- Peer support is provided to foster parents facing allegations of abuse. As part of this process IFAPA provides a preventive parenting training program for foster parents.
- Respite care program for foster and adoptive parents
- Distribution of Foster Parent Handbooks to newly licensed foster parents
- Adoption Information Specia lists (AIS) are IFAPA employees, located in the DHS service areas that contact adoptive families at the point of finalization to provide support, information and referral and minimal crisis intervention to help reduce the disruption potential in the adoptive homes

Statewide Assessment

The number of licensed foster families in Iowa has increased 12% since FY 2000, from 2,434 to 2,725 as of December 2002.

The number of finalized adoptions increased form 226 in state fiscal year 1995 to 781 in state fiscal year 2002. DHS sustained recruitment efforts have contributed significantly to the increase in the number of foster children in Iowa who have been adopted. In 1999 Iowa received a 2002 Adoption Excellence Award from HHS for increasing the number of finalized adoptions. The following chart shows data from the department's SACWIS system on the percentage of children whose parental rights have been terminated who have been adopted. Approximately 75% of the children adopted each year are adopted by their current or former foster parents.

Year child's parental rights were terminated	Number of children adopted as of 12-1-02
1996	554
1997	637
1998	704
1999	710
2000	686
2001	732
2002	403

Although the numbers of minority children in foster care is small, minority children are over-represented in foster care compared to the general population of the state. Foster parents that participated in focus groups indicated that recruitment of minority foster and adoptive families is not effective, " diversity of families is not seen." Iowa currently does not have available on a statewide basis information regarding the number of Native American foster homes, Spanish speaking foster parent, or other diversity information.

Recruitment practices comply with the principles of the Multiethnic Placement Act of 1994 as amended by the Interethnic Adoptions of 1996. In 1998 the Office of Civil Rights, Region VII conducted a review of DHS to determine whether children available for adoption are placed without regard to race, color, or national origin. No violations were found.

Approximately half of the approved families have adopted children and in many instances these families are not available to adopt additional children. The child preference of the remaining approved families often does not match the characteristics of waiting children. Therefore, children available for adoption wait in foster care until an adoptive home is recruited and approved for adoption. In state fiscal year 2002 the average time from placement to termination of parental rights was 12.9 months and the time from termination to adoption finalization was 11 months. When children that are legally free and are not adopted they may remain in foster care (family, group care or independent living) until they reach adulthood (age 18) or graduate for high school or a GED program.

The average length of time families have been foster parents is 3 years. 25% have been foster parents for over five years. Iowa ranks 7th out of 36 states reported by the Child Welfare League of America for the ratio of foster homes available to children in care.

About 55% of licensed foster families have placements during a 12-month time period. Due to the fact that many foster homes remain unused, some homes better suited to meet presenting needs may be over used. Often homes are not available in the areas where placement is needed and families are not always prepared to accept the type of children available for placement. Consequently, some foster homes may be allowed (through Director's exception to policy) to exceed their license capacity. The maximum number of children allowed in a family foster home is five, including the family's own children. In state fiscal year 2002, 60 exceptions or variances to the capacity limit were granted. Area administrators may grant variances to allow sibling to be placed together which also affects the number of child in a home

One of the reasons that some foster homes are not used is that foster parents may not be adequately prepared to accept the type of children that are available for placement. It is hoped that the implementation of PS-MAPP will begin to address this issue. Other reasons that foster homes are unused have not been clearly identified.

IFAPA, through its contract with DHS to recruit and retain foster and adoptive families is expected to contact all currently licensed foster families that have not received placements since the family's last license renewal. The goal of the contact is to inform families about the children who are available for placement and other placement options that could match the families' placement and skill level. It is expected that this process will identify other reasons families believe they are not used.

In a survey, 496 DHS staff were asked on a scale of 1 to 5, with 1 being Strongly Disagree and 5 Strongly Agree, to provide their assessment of the following statements.

Survey Questions				
There are an adequate number of foster families in my Service Area to meet the needs of the children served.				
Strongly disagree	Disagree	Neutral	Agree	Strongly agree
45.3%	39.6%	5.3%	8.9%	1.0%

I understand the requirements of the Multiethnic Placement Act (MEPA).				
Strongly disagree	Disagree	Neutral	Agree	Strongly agree
20%	22.4%	24.4%	25.7%	7.5%

Rural workers reported a higher number of foster families, however both mean scores were low – 1.69 for urban workers and 1.92 for rural workers. Further data analysis is needed to determine the reasons for these responses. With respect to understanding the requirements of the MEPA, supervisors and caseworkers reported higher ratings that did child protective investigators. The mean response for supervisors was 3.33, and for caseworkers 2.84.

Summary

Strengths
- The number of foster families in Iowa has increased over the last 2 years
- The number of adoptions has increased significantly over the last 5 years
- Iowa has a statewide foster family and recruitment project that involves local DHS offices, private agencies and foster and adoptive families

Opportunities for Improvement
- More foster families for adolescents are needed. Foster families are not always available in communities that need them. Staff are often challenged to match the needs of children with a family that can meet those needs
- Foster and adoptive families are needed that match the ethnic and racial background of children that need placement
- Cultural competence knowledge and skills of staff needs to be enhanced

Five

5. Citing any data available to the State, discuss how effective the State has been in meeting the State plan requirement to recruit and use adoptive families for waiting children across the State or other jurisdictional boundaries. In responding, consider relevant agency policies, time frames for initiating recruitment activities and specific methods.

Description

Code of Iowa, Chapter 232.119, requires that a child must be placed on the Iowa Adoption Exchange System (IAES) 60 days after termination of parental and assignment of guardianship to the State if an adoptive home has not been located. If a placement has not been located 90 days after the child is registered on IAES, the Code of Iowa requires that the child must be registered on a national electronic exchange and electronic photo listing system. A deferral of registration to the state and national exchanges may be granted for a limited time period for specific reasons. A report is submitted to the Court every 45 days detailing specific activities that have been made to place a child for adoption in a timely manner after termination of parental rights. The reports must be submitted to the Court every 45 days until the child is placed with an adoptive family.

It is the responsibility of DHS adoption workers to enter children with termination of parental rights into IAES. A monthly printout of children registered on IAES is generated by DHS and sent to KidSake. KidSake uses this information to register children on the National Adoption Exchange, the internet photo listing, AdoptUSKids (formerly, Faces of Adoption) and to identify children that should be included in the photo listing book, Iowa's Waiting Child. Iowa's Waiting Child is often sent to families in all parts of the state and to families in other states.

Inquiries that KidSake receives from families within the state and from other states are immediately referred to the child's worker for follow-up.

Statewide Assessment

From January 1, 2002 through September 31, 2001, 115 children registered on IAES and national exchanges were placed for adoption. Several of these children were placed with adoptive families in other states.

Each month there are approximately 140 children registered on the Iowa Adoption Exchange System. In addition, approximately **65** children are featured in the photo-listing book and approximately **40** children are currently registered on the national website, **AdoptUSKids.**

Iowa is making significant strides toward placing more children with adoptive families in other jurisdictions. This is attributed, in part, to the Code of Iowa mandate to register waiting children on state and national exchanges within specified time frames. In addition, the Internet is used effectively to disseminate information about Iowa's special needs adoption program and the availability of waiting children.

Due to extremely high case loads (50-70 children) adoption workers may not always register children timely on IAES and send referral information to KidSake. As of 10/02 147 children were register on IAES, of which only 60 are listed on the KidSake website and 48 are listed on ADOPT US KIDS. KidSake reports that they have not received information for about 90 children that they know are available for adoption. Updated information and a monthly printout is not being sent to KidSake in a timely way from DHS. These delays could have an impact on the achievement of timely permanence for waiting children.

Summary

Strengths

- Iowa statute and employees' manual provides clear guidance regarding use of the state and national exchanges to place children for adoption across the state or other jurisdictional boundaries

Opportunities for Improvement

- More children need to be registered on the Iowa Adoption Exchange and the national web-site
- Identification of resources and coordination of services when children are placed in other jurisdictions
- More children need to be registered on the Iowa Adoption Exchange and the national website in accordance with mandated timeframes

Section III - Safety and Permanency Data

I. CHILD SAFETY PROFILE	Calendar Year 1999						Calendar Year 2000						Calendar Fiscal Year 2001					
	Reports	%	Duplic. Childn.[2]	%	Unique Childn.[2]	%	Reports	%	Duplic Childn.[2]	%	Unique Childn.[2]	%	Reports	%	Duplic Childn.[2]	%	Unique Childn.[2]	%
I. Total CA/N Reports Disposed[1]							21,276		31,317		25250		25,112		37,704		29,617	
II. Disposition of CA/N Reports[3]																		
Substantiated & Indicated							7,547	35.5	10,822	34.6	9416	37.3	8,712	34.7	12,792	33.9	11,179	37.7
Unsubstantiated							13,729	64.5	20,495	65.4	15834	62.7	16,400	65.3	24,912	66.1	18,438	62.3
Other																		
III. Child Cases Opened for Services[4]									5,140	47.5	4279	45.4			6,024	47.1	5,031	45.0
IV. Children Entering Care Based on CA/N Report[5]									1,445	13.4	1193	12.7			1,812	14.2	1,537	13.7
V. Child Fatalities[6]											10	0.1					15	0.1
STATEWIDE AGGREGATE DATA USED TO DETERMINE SUBSTANTIAL CONFORMITY																		
VI. Recurrence of Maltreatment[7,] [Standard: 6.1% or less)											595 of 5,041	11.8					694 of 6,184	11.2
VII. Incidence of Child Abuse and/or Neglect in Foster Care[8,] (for Jan-Sept) **[Standard: 0.57% or less]**					of 9,127						74 of 9,381	.79					86 of 9,638	.89

FOOTNOTES TO DATA ELEMENTS IN CHILD SAFETY PROFILE

Each maltreatment allegation reported to NCANDS is associated with a disposition or finding that is used to derive the counts provided in this safety profile. The safety profile uses three categories. The various terms that are used in NCANDS reporting have been collapsed into these three groups.

Disposition Category	Safety Profile Disposition	NCANDS Disposition Codes Included
A	Substantiated or Indicated (Maltreatment Victim)	"Substantiated," "Indicated," and "Alternative Response Disposition Victim"
B	Unsubstantiated	"Unsubstantiated," "Unsubstantiated, Other than Intentionally False Reporting " and "Unsubstantiated Due to Intentionally False Reporting"
C	Other	"Closed-No Finding," "Alternative Response Disposition – Not a Victim," "Other," and "Unknown or Missing"

Alternative Response was added starting with the 2000 data year. The two categories of Unsubstantiated were added starting with the 2000 day year. In earlier years there was only the category of Unsubstantiated

1. The data element, "Total CA/N Reports Disposed," is based on the reports received in the State that received a disposition in the reporting period under review. The number shown may include reports received during a previous year that received a disposition in the reporting year. Counts based on "reports," "duplicated counts of children," and "unique counts of children" are provided.

2. The duplicated count of children (report-child pairs) counts a child each time that (s)he was reported. The unique count of children counts a child only once during the reporting period, regardless of how many times the child was reported.

3. For the column labeled "Reports," the data element, "Disposition of CA/N Reports," is based on upon the highest disposition of any child who was the subject of an investigation in a particular report. For example, if a report investigated two children, and one child is found to be neglected and the other child found not to be maltreated, the report disposition will be substantiated (Group A). The disposition for each child is based on the specific finding related to the maltreatment(s). In other words, of the two children above, one is a victim and is counted under "substantiated" (Group A) and the other is not a victim and is counted under "unsubstantiated" (Group B). In determining the unique counts of children, the highest finding is given priority. If a child is found to be a victim in one report (Group A), but not a victim in a second report (Group B), the unique count of children includes the child only as a victim (Group A). The category of "other" (Group C) includes children whose report may have been "closed without a finding," children for whom the allegation disposition is "unknown," and other dispositions that a State is unable to code as substantiated, indicated, alternative response victim, or unsubstantiated.

4. The data element, "Child Cases Opened for Services," is based on the number of victims (Group A) during the reporting period under review. "Opened for Services" refers to post-investigative services. The duplicated number counts each time a victim's report is linked to on-going services; the unique number counts a victim only once regardless of the number of times services are linked to reports of substantiated maltreatment.

Section III - Safety and Permanency Data

5. The data element, "Children Entering Care Based on CA/N Report," is based on the number of victims (Group A) during the reporting period under review. The duplicated number counts each time a victim's report is linked to a foster care removal date. The unique number counts a victim only once regardless of the number of removals that may be reported.

6. The data element "Child Fatalities" counts the numb er of children reported to NCANDS as having died as a result of child abuse and/or neglect. Depending upon State practice, this number may count only those children for whom a case record has been opened either prior to or after the death, or may include a number of children whose deaths have been investigated as possibly related to child maltreatment. For example, some States include neglected-related deaths such as those caused by motor vehicle or boating accidents, house fires or access to firearms, under certain circumstances. The percentage is based on a count of unique victims of maltreatment for the reporting period. The count also includes fatalities that have been reported on the Agency File, which collects non-child welfare information system data.

7. The data element, "Recurrence of Maltreatment," is defined as follows: Of all children associated with a "substantiated," "indicated," or "alternative response victim" finding of maltreatment during the first six months of the reporting period, what percentage had another "substantiated," "indicated," or "alternative response victim" finding of maltreatment within a 6-month period. The number of victims during the first six-month period and the number of these victims who were recurrent victims within six months are provided. This data element is used to determine, in part, the State's substantial conformity with Safety Outcome #1.

8. *The data element, "Incidence of Child Abuse and/or Neglect in Foster Care," is defined as follows: Of all children who were served in foster care during the reporting period, what percentage were found to be victims of maltreatment. A child is counted as having been maltreated in foster care if the perpetrator of the maltreatment was identified as a foster parent or residential facility staff. Counts of children maltreated in foster care are derived from NCANDS, while counts of children placed in foster care are derived from AFCARS. The observation period for these measures is January-September because this is the reporting period jointly addressed by both NCANDS and AFCARS. For both measures, the number of children found to be maltreated in foster care and the percentage of all children in foster care are provided. This data element is used to determine, in part, the State's substantial conformity with Safety Outcome #2.*

Additional Footnotes

(None)

Section III - Safety and Permanency Data

II. POINT-IN-TIME PERMANENCY PROFILE	Federal FY 1999		Federal FY 2000		Federal FY 2001	
	# of Children	% of Children	# of Children	% of Children	# of Children	% of Children
I. Foster Care Population Flow						
Children in foster care on first day of year	4,954		4,862		5,085	
Admissions during year	5,343		5,620		5,829	
Discharges during year	5,443		5,414		5,712	
Children in care on last day of year	4,854		5,068		5,202	
Net change during year	-100		+206		+117	
II. Placement Types for Children in Care						
Pre-Adoptive Homes	85	1.8	286	5.6	381	7.3
Foster Family Homes (Relative)	39	0.8	38	0.7	42	0.8
Foster Family Homes (Non-Relative)	2,555	52.6	2,649	52.3	2,750	52.9
Group Homes	1,413	29.1	1,522	30.0	1,536	29.5
Institutions	408	8.4	433	8.5	400	7.7
Supervised Independent Living	80	1.6	92	1.8	89	1.7
Runaway	0	0	1	0.0	0	0
Trial Home Visit	0	0	0	0	0	0
Missing Placement Information	269	5.5	33	0.7	0	0
Not Applicable (Placement in subsequent year)	5	0.1	14	0.3	4	0.1
III. Permanency Goals for Children in Care						
Reunification	2,038	42.0	2,051	40.5	2,170	41.7
Live with Other Relatives	80	1.6	79	1.6	88	1.7
Adoption	653	13.5	849	16.8	961	18.5
Long Term Foster Care	552	11.4	615	12.1	589	11.3
Emancipation	91	1.9	101	2.0	74	1.4
Guardianship	24	0.5	26	0.5	35	0.7
Case Plan Goal Not Established	1,416	29.2	1,347	26.6	1,285	24.7
Missing Goal Information	0	0	0	0	0	0

Section III - Safety and Permanency Data

II. POINT-IN-TIME PERMANENCY PROFILE (continued)	Federal FY 1999		Federal FY 2000		Federal FY 2001	
	# of Children	% of Children	# of Children	% of Children	# of Children	% of Children
IV. Number of Placement Settings in Current Episode						
One	1,893	39.0	1,919	37.9	1,961	37.7
Two	1,268	26.1	1,359	26.8	1,415	27.2
Three	636	13.1	669	13.2	728	14.0
Four	377	7.8	413	8.1	409	7.9
Five	214	4.4	237	4.7	221	4.2
Six or more	466	9.6	471	9.3	468	9.0
Missing placement settings	0	0	0	0	0	0
V. Number of Removal Episodes						
One	3,378	69.6	3,478	68.6	3,584	68.9
Two	960	19.8	1,019	20.1	1,033	19.9
Three	313	6.4	342	6.7	354	6.8
Four	115	2.4	140	2.8	128	2.5
Five	43	0.9	48	0.9	57	1.1
Six or more	45	0.9	41	0.8	46	0.9
Missing removal episodes	0	0	0	0	0	0
VI. Number of children in care 17 of the most recent 22 months[2] (percent based on cases with sufficient information for computation)	1,157	33.5	1,138	30.2	1,021	27.3
	Number of Months		**Number of Months**		**Number of Months**	
VII. Median Length of Stay in Foster Care (of children in care on last day of FY)	11.7		10.7		10.7	

Section III - Safety and Permanency Data

II. POINT-IN-TIME PERMANENCY PROFILE (continued)	Federal FY 1999		Federal FY 2000		Federal FY 2001	
	# of Children Discharged	Median Months to Discharge	# of Children Discharged	Median Months to Discharge	# of Children Discharged	Median Months to Discharge
VIII. Length of Time to Achieve Perm. Goal						
Reunification	4,176	2.6	4,214	2.5	4,521	2.7
Adoption	767	30.0	718	28.2	660	25.5
Guardianship	0	0	0	0	34	13.8
Other	450	17.4	443	18.7	463	17.9
Missing Discharge Reason	14	9.0	1	14.9	1	29.8
Missing Date of Latest Removal or Date Error [3]	36	NA	0	NA	33	NA
Statewide Aggregate Data Used in Determining Substantial Conformity	**# of Children**	**% of Children**	**# of Children**	**% of Children**	**# of Children**	**% of Children**
IX. Of all children who were reunified with their parents or caretakers at the time of discharge from foster care, what percentage was reunified in less than 12 months from the time of the latest removal for home? (4.1) **[Standard: 76.2% or more]**	3,372	80.2	3,443	81.1	3,686	81.1
X. Of all children who exited care to a finalized adoption, what percentage exited care in less than 24 months from the time of the latest removal from home? (5.1) **[Standard: 32.0% or more]**	288	37.5	284	39.4	305	46.1
XI. Of all children served who have been in foster care less than 12 months from the time of the latest removal from home, what percentage have had no more than two placement settings? (6.1) **[Standard: 86.7% or more]**	5,494	89.2	5,738	88.6	6,024	88.8
XII. Of all children who entered care during the year, what percentage re-entered foster care within 12 months of a prior foster care episode? (4.2) **[Standard: 8.6% or less]**	1,427	26.7 (65% new entry)	1,401	24.9 (66% new entry)	1,425	24.5 (67% new entry)

Section III - Safety and Permanency Data

III. PERMANENCY PROFILE FIRST-TIME ENTRY COHORT GROUP	Federal FY 1999		Federal FY 2000		Federal FY 2001	
	# of Children	% of Children	# of Children	% of Children	# of Children	% of Children
I. Number of children entering care for the first time in cohort group (% = 1st time entry of all entering within first 6 months)	1,533	64.4	1,701	66.4	1,776	68.1
II. Most Recent Placement Types						
Pre-Adoptive Homes	61	4.0	46	2.7	58	3.3
Foster Family Homes (Relative)	4	0.3	10	0.6	9	0.5
Foster Family Homes (Non-Relative)	571	37.2	737	43.3	819	46.1
Group Homes	739	48.2	774	45.5	751	42.3
Institutions	110	7.2	93	5.5	110	6.2
Supervised Independent Living	8	0.5	8	0.5	9	0.5
Runaway	31	2.0	29	1.7	20	1.1
Trial Home Visit	0	0	0	0	0	0
Missing Placement Information	9	.6	4	0.2	0	0
Not Applicable (Placement in subsequent yr)	0	0	0	0	0	0
III. Most Recent Permanency Goal						
Reunification	926	60.4	930	54.7	1,024	57.7
Live with Other Relatives	29	1.9	37	2.2	34	1.9
Adoption	94	6.1	92	5.4	99	5.6
Long-Term Foster Care	31	2.0	22	1.3	24	1.4
Emancipation	8	0.5	14	0.8	8	0.5
Guardianship	5	0.3	5	0.3	5	0.3
Case Plan Goal Not Established	440	28.7	601	35.3	582	32.8
Missing Goal Information	0	0	0	0	0	0

Section III - Safety and Permanency Data

III. PERMANENCY PROFILE FIRST-TIME ENTRY COHORT GROUP (Continued)	Federal FY 1999		Federal FY 2000		Federal FY 2001	
	# of Children	% of Children	# of Children	% of Children	# of Children	% of Children
IV. Number of Placement Settings in Current Episode						
One	1,057	68.9	1,103	64.8	1,153	64.9
Two	306	20.0	368	21.6	383	21.6
Three	107	7.0	144	8.5	146	8.2
Four	36	2.3	53	3.1	54	3.0
Five	15	1.0	24	1.4	23	1.3
Six or more	12	0.8	9	0.5	17	1.0
Missing placement settings	0	0	0	0	0	0
V. Reason for Discharge						
Reunification/Relative Placement	909	90.2	1,001	93.3	1,020	92.6
Adoption	56	5.6	39	3.6	45	4.1
Guardianship	0	0	0	0	6	0.5
Other	37	3.7	33	3.1	30	2.7
Unknown (missing discharge reason or N/A)	6	0.6	0	0	0	0
	Number of Months		Number of Months		Number of Months	
VI. Median Length of Stay in Foster Care	3.3[4]		3.9[5]		4.8[6]	

FOOTNOTES TO DATA ELEMENTS IN THE PERMANENCY PROFILE

[1]The FY99, FY00, and FY 01 counts of children in care at the start of the year exclude 176, 168, and 161 children, respectively. They were excluded to avoid counting them twice. That is, although they were actually in care on the first day, they also qualify as new entries because they left and re-entered again at some point during the same reporting period. To avoid counting them as both "in care on the first day" and "entries," the Children's Bureau selects only the most recent record. That means they get counted as "entries," not "in care on the first day."

[2]We designated the indicator, *17 of the most recent 22 months*, rather than the statutory time frame for initiating termination of parental rights proceedings at *15 of the most 22 months*, since the AFCARS system cannot determine the *date the child is considered to have entered foster care* as defined in the regulation. We used the outside date for determining the *date the child is considered to have entered foster care*, which is 60 days from the actual removal date.

[3]Dates necessary for calculation of length of time in care in these records are chronologically incorrect. N/A = Not Applicable

[4] This First-Time Entry Cohort median length of stay was 3.3 months in FY99. This includes 25 children who entered and exited on the same day (who had a zero length of stay). If these children were excluded, the median length of stay would be 3.6 months.

[5] This First-Time Entry Cohort median length of stay was 3.9 months for FY00. This includes 11 children who entered and exited on the same day (who had a zero length of stay). If these children were excluded, the median length of stay would have been 4.0 months.

6 This First-Time Entry Cohort median length of stay is 4.8 months for FY01. This includes 10 children who entered and exited on the same day (who had a zero length of stay). If these children were excluded, the median length of stay would be 4.9 months.

Narrative Assessment of Child and Family Outcomes

Introduction

Iowa has placed considerable effort into monitoring and informing Iowa's Child Welfare System of the child and family outcomes over the last two years. Among the activities to support this emphasis was working with the National Resource Center for Organization Improvement, revise curriculum to make it compatible with Iowa's Child Welfare System. The curriculum was designed to assist in using data effectively to understand performance and to improve outcomes. Last year Linda Arnold, National Data Analysis System, Child Welfare League of America, made presentations statewide to introduce DHS, Juvenile Court staff, and providers to our data outcome indicators. In order to understand recent performance, Iowa has created the Child Welfare Outcomes reports and measures in house rather than to wait for federal publication. Outcomes are reported by geographic areas -- state; service areas, juvenile court districts, and counties. Further analysis has provided outcomes that are reported by type of case: Juvenile Court cases and DHS cases. Reports have been shared with the Court Improvement Project, with Citizen's Foster Care Review Board, with Juvenile Court Officer Chiefs, and with Juvenile Court Judges.

Data has been used to support performance and compliance with the ASFA requirement that a termination of parental rights petition be filed for children in foster care for 15 of the last 22 months. For the past two years an on-line report has been available to assist DHS and JCS staff in tracking and monitoring their performance. Also for the past two years specific focus on timeliness of adoption has been monitored and reported through another specialized on-line report. In order to keep our partners informed this same administrative data and reporting has been made available to the Citizen's Foster Care Review Board and the Court Improvement Project. Iowa is now focusing on improving child safety, and will soon begin to address the disproportionate representation of minority children in the child welfare populations.

In addition to promoting awareness of our child welfare data outcome indicators, the department has focused on the quality of our data by:

- Conducting two data mapping review projects to assure that our FACS and STAR systems are drawing data elements consistent with federal instructions and data code values
- Conducting two data validation projects where the quality of information in our system was evaluated by comparing it with case file information
- Examining child abuse reports where recurrence was identified to determine if those cases were actual incidents of recurrence

The department has placed emphasis on supporting reunification of children with their families and permanency through adoption. Iowa outcome data reflects strong performance especially in each of those two areas.

Safety

Trends in Child Maltreatment

Iowa recognizes the important role of the public in reporting suspected child abuse. The department uses a variety of public awareness and education strategies about recognizing and reporting suspected child abuse and neglect as important tools for the protection of children. These tools include Public Service Announcements, a web site, training, and a 24 hour Hot Line.

In 2002, the child population in Iowa showed an increase of 2.6% for the first time since 1998 according to Woods and Poole population projections. Child population in Iowa had been decreasing slightly since 1998 through 2001 for a total decrease of 3.6%.

The number of reports of suspected child abuse and neglect have increased annually each of the past three years, as have the number of duplicated and unique child subjects.

In a recent article written by Steve Scott, Director of Prevent Child Abuse Iowa, the author attributes the "real" decline in the extent of child sexual abuse to the fact that:

- Iowa has notably improved its ability to assess allegations of child sexual abuse and take effective action to protect children from further abuse. Critical to this improvement has been the development and growth of child protection centers in Cedar Rapids and Sioux City. These centers have a multi-disciplinary staff skilled at determining whether sexual abuse has occurred and providing evidence to help DHS and law enforcement officials protect children
- Many Iowa school children receive instruction on how to recognize the risk of sexual abuse, protect themselves, and report any threats or harm
- In 2000 DHS expanded its acceptance of reports of suspected abuse, leading to its acceptance of more abuse reports. Furthermore, DHS did not confirm a lower percentage of abuse reports; indeed, its confirmation rate increased from 31% in the early 1990s to 35 percent in 1998-2000
- While the Iowa sexual abuse figures have dropped substantially, those for other forms of abuse have not

Iowa data profile information indicates an increase in the number of reports of suspected child abuse and neglect in 2000 and 2001. The number of total reports increased by 14% in 2000 over 1999, and increased by 18% in 2001 over 2000. There have been similar increases for both total child victims (duplicated) and unique child victims.

Child Maltreatment - Reports of Child Abuse and Neglect

Data profile information indicates the number of reports of abuse, and child abuse victims have been on the increase, the percent of reports substantiated remained constant at 37% with less than a 1% change over the same periods. While Iowa's percent of reports substantiated is slightly higher than the national average of 30%, the national figure does include 16% designated as “Other” while Iowa reports no cases in the “Other” category. The percent of reports not substantiated also remains constant at 62% with less than a 1% change over the same periods.

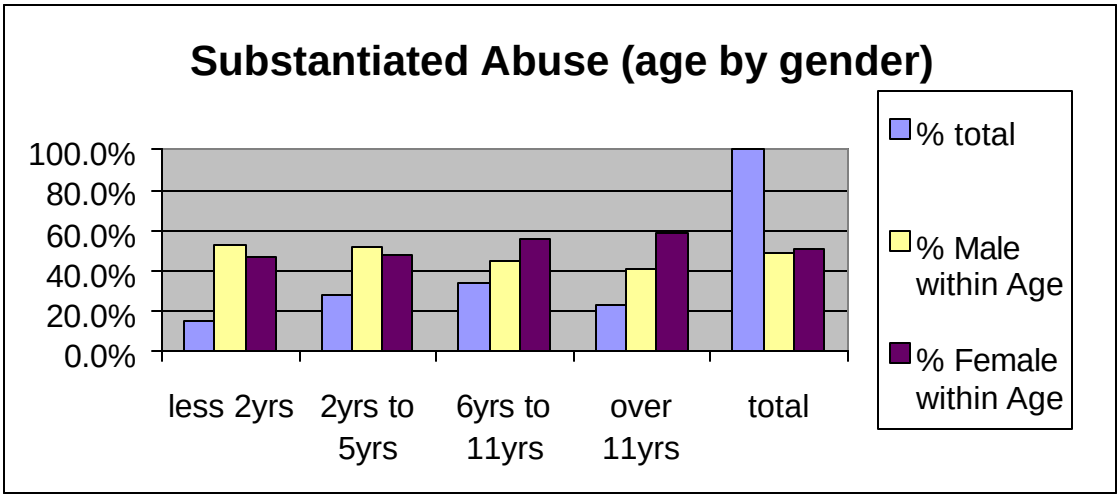
Iowa data is comparable when the national data profile information is adjusted for the “cases” reported as other. Iowa's substantiated reports are 7% over national averages, and unsubstantiated

reports are 6% over national averages, combining for 13% of the 16% nationally reported in the category of "Other".

Of all children who are substantiated victims of abuse, the youngest children (age 0 to 5) account for the greatest percent (43.8%) of all child victims in Iowa's 2001 DCDC report. Children in the middle age ranges (6 to 11) make up 35% of all child victims, and the oldest children (12 to 18) account for the smallest percent of child victims with 22.7%.

Overall, children who are substantiated victims of abuse in Iowa are nearly equally likely to be male (49.2%) or female (50.8%) of children reported in 2001 DCDC data. Grouping children by age shows that females account for a larger percent of the total cases for older children. Females make up 60% of the 12 to 18 age range. (See figure 4.1 below)

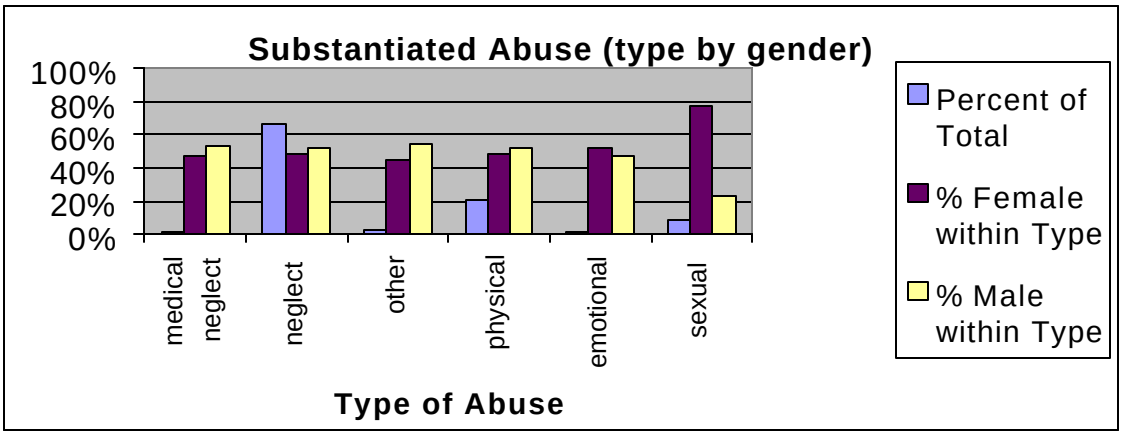
Figure 4.1



Child neglect is by far the most common category of substantiated abuse accounting for 66% of all substantiated abuse. The next most common occurring abuse types are physical (20%) and sexual (8%). The distribution of abuse by type of abuse in Iowa is similar to national trends for type of abuse. When looking at gender by type of abuse only very small differences are seen except for sexual abuse where females account for 76% of all sexual abuse victims. (See figure 4.2)

While sexual abuse occurs for victims of all age groups, it is substantiated most for pre-teen and teenage females. This corresponds to the slight over representation of females in the older age groups represented in illustrated above in figure 4.1.

Figure 4.2



Child Cases Open for Services

The most recent DCDC data (2001) shows that 45% of cases in Iowa are opened for services. This is lower than the national average of 58%. The state self-assessment probed deeper into this issue to explore who was and wasn't receiving services and what was the status of child safety relative to provision of services. Through the Statewide Assessment analysis it was determined that Iowa was under reporting the number of cases that are referred for services. The analysis and re-mapping of the DCDC file has been completed and we are awaiting implementation of correction for children referred to services.

As the state began looking into the data we had to keep in mind that when a report involves more than one child in a family, but the needs are family focused, a service case may be opened in the name of one identified child. The service delivered however would be intended to benefit the overall family need. When individual children have unique needs in addition to the family needs, unique cases are established and services provided for each child with unique needs.

Unfortunately, until the programming changes for children referred for service occurs and more accurate data is produced it is difficult to assess if the percent of children receiving services is more comparable to national data trends. The lack of this service data will also reduce the effectiveness of other analysis, which could have assessed the impact of service upon other things such as re-abuse or entry into foster care.

NOTE: In the course of the Statewide Assessment analysis of NCANDS data, it was discovered that Iowa had a programming error, which affected reporting of "entering care" along with "open for services" cases. Programming changes are currently underway, but the exact impact is unknown at the time of this report.

Children Entering Care Based Upon Abuse and Neglect

Iowa 2001 data profile shows a slight increase (1%) in the number and percentage of children entering care as a result of abuse and neglect from the 2000 level. The 2001 percentage of children entering care in Iowa is 13.7%, 7% less than national 2000 average of 20.8%. The proper identification of children referred for service mentioned above also impacts this statistic.

Child Fatalities

There have been about 10 child fatalities related to abuse annually in Iowa between 1998 and 2000, and 15 reported in the 2001 DCDC file. In the process of completing analysis for this state wide assessment, state staff have reconciled the 15 deaths as reported in the 2001 DCDC file with other state data which indicated 13 child fatalities in 2001. What was discovered was that only 10 of the 15 child fatalities reported in the DCDC submission were in fact child fatalities, the other 5 were data entry errors where workers miscoded information. State staff also discovered 3 other data entry errors where children not listed as fatalities which were child fatalities. Additional monitoring of the use of the code indicating "maltreatment resulting or contributing to child fatality" and immediate feedback to staff doing entry has been implemented to assure accuracy of future reporting.

Eleven of the 13 child fatalities in 2001 had no prior involvement with DHS. Of the two with prior involvement, one had a prior confirmed report, and one had two prior unconfirmed reports. None of the 13 had prior services from DHS. Of the 13 child fatalities in 2001, 7 children were male and 6 children were female. Ten of the children were white, one was Hispanic, and 2 reported as missing information for race/ethnicity. Eight of the 13 child fatalities were under age one, four were at least one but less than six and one was age six. A biological parent was the perpetrator in 10 of the 13 fatalities, and 3 were listed as “other." Of the 10 deaths by a parent, 8 were by a female parent and 2 by a male parent.

Since 1995 the multidisciplinary Child Death Review Team has been conducting case reviews, making policy and practice recommendations that may prevent child deaths, and making recommendations to the Governor and General Assembly.

Recurrence of Maltreatment

Iowa’s recurrence rate has been fairly constant around 10%. This exceeds the National Standard of 6.1% or less. One condition, which contributes to Iowa’s rate, is the inclusion of subsequent reports of abuse received and disposed even when the actual incident investigated occurred before the previous report. It is unknown at this time if Iowa’s current (low) rate of referral for services or placement plays any part in recurrence, until following implementation of the correction mentioned in the referral for services section and analysis future corrected data

Data profile information indicates the profile of children re-abused is not significantly different than for other states. In Iowa, 53% of repeat maltreatment occurs for children between age 0 to 5 (slightly higher than the rate for this age in all maltreatment), 31% for children age 6 to 11 (very similar to the 35% of all maltreatment), 15% for children ages 12 to 18 (slightly lower than that for all maltreatment). Repeat maltreatment like maltreatment in general, occurs equally for males and females each accounting for 50% of re-abuse. Gender is distributed nearly equally in all types of repeat maltreatment, the only exception being sexual abuse which occurs more frequently for females than for males. Child neglect accounts for 75% of all repeat maltreatment (slightly greater share than in neglect in all maltreatment at 66%); physical abuse represents 12% of repeat maltreatment, and sexual abuse 7%.

INITIAL ABUSE	Abuse Type	% of all recurrence	# 2nd same type	% 2nd Same type	# 2nd perp same	% 2nd Perp same
neglect or deprivation	536	77%	436	81%	341	64%
medical neglect	9	1%	6	67%	6	67%
physical abuse	89	13%	18	20%	37	42%
sexual abuse	44	6%	16	36%	9	20%
psyc or emotional abuse	5	1%	0	0%	4	80%

Looking at re-abuse by age, 24% are under 3, 50% are under 7, and 77% are under 12 - a pattern nearly identical to confirmed abuse by age. Analysis for the Statewide Assessment as well as collection of additional information should lead to development of focused strategies in the PIP.

Incidence of Child Abuse and or Neglect in Foster Care

The most recent data (2002DCDC) will reflect a drop by one half the prior rates and show Iowa slightly under the national standard for incidence of child abuse and or neglect in foster care. In prior periods there has been annual fluctuations but the percent of abuse in care has varied less than 1 child per 1000 in care over the prior periods. These changes are not perceived as significant.

Fluctuations can be attributed to a variety of issues according to focus groups contacted as part of the state self assessment. Those issues include; facility staff turnover, harder kids in family homes, high state agency caseload reduces frequency of visits with child/family, and budgetary issues delaying access to timely and effective services. The rate of abuse by an out of home care provider in Iowa is higher than the national standard of 0.57%, at 0.97% in 2001. Each previous year of the past four years has seen slight increases, from 0.41% in 1998 to 0.42% in 1999 to 0.79% in 2000.

Evaluation and Summary of Safety Findings

Reports of child abuse and neglect have increased without relationship to child population. The increases in 2000 and 2001 appear to coincide with public awareness created by a recent high profile case in 1999.

Iowa investigates abuse reports, even if the incident itself occurred in the past, no matter to how long ago. Therefore counts of recurrence of abuse will include some future investigation and reports of prior incidents as subsequent abuse. While the state is aware that some other states also report in this manner, not all do. Attributing "prior" incidents as "recurrence" does not accurately represent a child's current safety status. Iowa would like to be able to identify true incidents of subsequent abuse regardless of report dates. Iowa is also aware that reporting differences alone do not explain all of the difference between Iowa and National re-abuse rates based upon a data validation effort to determine the circumstances around re-abuse. Evaluation of policy, practice and programming necessary to report in this fashion are underway. Regardless of reporting differences, Iowa has re-abuse rates higher than national averages, and the state continues to investigate re-abuse relationships to case management practices, and service issues to determine possible improvement strategies.

AHA Recommendations for Improvement

An independent review conducted by the American Humane Association several years ago made the following recommendations for improvement in Iowa's child welfare system:

- Coordinate services between child welfare and substance abuse treatment providers and the Department of Health
- Provide training and expert consultation in the areas of substance abuse and domestic violence
- Improve the mandatory reporting process by improving training of mandatory reporters and improving the skills and consistency of intake workers

Permanency

Trends in Permanency Data

- Iowa data reflects increases in the number of children with permanency goals of adoption
- Iowa data reflects increases in the percent of children in adoptive homes each year
- Iowa data reflects stable reunification rates at or above the national standard.
- Iowa data reflects stable stability of placement rates at or above the national standard.
- Iowa data reflects a record number of children with TPR in the SFY02
- Iowa data reflects a strong trend of annual increases in the number of children adopted
- Iowa data reflects annual decreases in the percentage of children in care 17 of the previous 22 months
- Iowa data reflects a stable but high re-entry into care rate

Foster Care Population Flow

Iowa's data profile indicates that Iowa has seen no significant changes in population flow over the past three years. The maximum fluctuation of any of the flow measures (the first day, last day, admissions and discharges) compared to the 3-year average was only 3.4% or less for each category. While Iowa is not seeing significant net changes or trends in the "flow" of cases entering or leaving the foster care system, that "leveling off " of the foster care population is fairly recent. Iowa's foster care population stabilized in 1998 following annual increases each prior year for many years. Nationally the data represents continuing increases in foster care populations with a possible slowing of the growth nationally in 2000.

One unique feature of Iowa's population flow compared to national data profile information is the difference in the comparison of the number of children in care to the admissions during a reporting period. Iowa data reflects a "turnover rate" (admissions divided by in care on the first day of the period) which is twice that of the national average. Children in Iowa exit care after stays with significantly shorter average stays than national averages. Most of these children return home (reunification rate is 81% which exceeds the national standard). Iowa has a strong array of services to support reunification. In terms of stability, Iowa is mindful of the high rate of re-entry into foster care. Discovering to what extent these services are provided as post-reunification services and why they are not as successful in preventing reentry into care will require additional information. We will continue to work to better understand our issues around re-entry and the need to improve in this area.

Another unique feature of Iowa's population flow is the percentage of children in care for 17 of the last 22 months. Percentage in Iowa is at 27.3% of children in care for 17 of the last 22 months for FFY01, 30% for FFY00, and 33% for FFY99. Iowa's percent of children in care for 17 of the last 22 months is significantly lower than national averages where 53% were in care 17 of last 22 months in FFY01, and 51% in FFY00 and FFY99.

Placement Types for Children in Foster Care

Iowa's population of children in Foster Care includes the child welfare population (approximately 86% of total), and children in juvenile justice population placed in foster care (approximately 14% of total). All of the children adjudicated delinquent are afforded all the same protections as the child welfare population through requirements of an interagency agreement. Included in the

child welfare population are voluntary and CINA cases in foster care due to mental health and behavioral issues when the parent is unwilling or unable to provide the care the child needs.

Iowa data does illustrate more admissions and discharges per year than children in care at either point in time measure, while national averages are about 50% turnover.

Iowa data profile information illustrates a history of high utilization of group care (27-30%), compared to national averages of less than 8% of all children in care in group care. While Iowa has stemmed the growth in the use of group care, the percent of children in group care continues to be high. Part of the explanation of Iowa's use of group care is due to Iowa including juvenile delinquents and children with mental health and behavioral needs in foster care and therefore the AFCARS populations. Delinquents and children with mental health and behavioral needs are more likely to be in group care, than other levels of foster care. Of all admissions during the most recent period whose current placement is group care, delinquents account for approximately one third. Non-delinquents make up the other two thirds of the children in group care.

Iowa has a lower percent of all other placement types due to the 20% higher percentage of children in group care. For instance, Family Foster Care in Iowa represents approximately 53% of the total foster care population, about 16% lower than nation averages. Iowa also used primarily non-relative family foster homes, with less than 1% relative homes compared to nearly 4% nationally. Iowa has not done enough to encourage use of relative or relative foster care placements, including seeking to improve use of kin by pursuing to waivers, which support use of relative homes.

Iowa looks very similar to national averages in terms of use of institutions. Iowa reports children in institutional placement predominately for youth in Psychiatric Medical Institutions for Children for psychiatric care and treatment. These facilities are medical facilities funded by Medicaid and state dollars.

Iowa data shows no children reported with trial home visits. Iowa does not use trial home visits when children return home and are intended to remain home, only for periodic visits during the course of stay in foster care and therefore do not report trial home visit as a placement type.

Iowa has improved data reporting to avoid missing placement information from a high of 6.1% missing data to less than one percent missing data in the most recent period.

Permanency Goals for Children in Foster Care

Iowa reports consistently higher percents of permanency goals not established than national averages. Iowa data reflects 26% of children with Case Plan Goals not established compared to 16% nationally for FFY00. This may be due to a combination of factors including data entry. The field is not required entry at this time but a system change will require entry in the future. In addition, Iowa data reflects cases entering and exiting at twice the national average, which increases the percent of "new" cases at any point in time which may not have yet had a permanency goal established. Additional analysis will be needed to determine the causes of Case Plan Goals reported as Not Established, and then strategies developed to improve the reporting of Case Plan Goals

In spite of 10% more of Iowa's cases being reported with goals not established, Iowa data still shows a slightly higher than average percent of cases with a goal of reunification. Iowa data does

include delinquents, who will be less likely than child welfare cases to have a goal of other than reunification, and which should slightly elevate the percentage of children with a goal of reunification. Iowa data shows the goal of reunification rates dropping slightly (approximately 1%) each year of the past three years. While the trend in reunification is decreasing, Iowa's percentage of children with the goal of adoption is also increasing. Since other permanency goals remain relatively stable, one could surmise that the decrease in reunification is due to the corresponding increase in the use of the permanency goal of adoption relating to Iowa's focus on time limited reunification.

The percent of children in care in Iowa with the goal of adoption has been lower than the national average of 20%, but increasing annually from 11% in 1998 to 18.5% in 2001. Since Iowa also has a somewhat greater percent of children with the goal of reunification, a 2% difference is not unexpected. Of the children with the goal of adoption 2% are under age 1, 36% between age 1 and 5, 28% between age 6 and 10, and 34% age 11 and over.

Age	Children with Goal of Adoption by age	% with goal of Adoption who achieve Adoption
< 1	2%	1%
1 to 5	36%	50%
6 to 10	28%	31%
11+	34%	14%

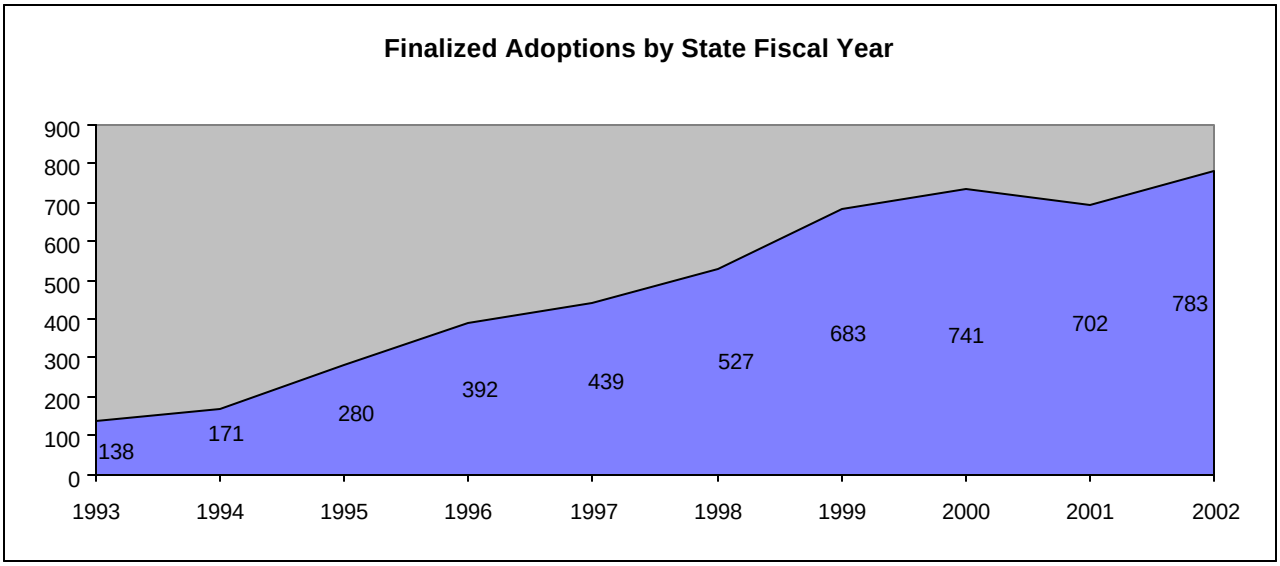
The 2002B AFCARS data shows eleven percent of children have the permanency goal of long term foster care (also known as Another Planned Permanency Living Arrangement) is slightly higher than the national average of 8%. Of the children with the permanency goal of long term foster care only 61% are age 16 or over, 37% are 11 to 15, and 2% are less than age 10. Of the 2% of children under age 10, all but one are in non-relative foster family care which may indicate a need to review the permanency goal for these children.

Achievement of Reunification

Iowa data reflects reunification occurs for approximately 81% of all children exiting foster care. This meets the federal standards of 76.2% or greater. Iowa's reunification rate reflects a strong and widely held value that children are best off with their families. Of all children reunified, 3% are under age 1, 16% age 1 to 5, 11% age 6 to 11, 36% age 11 to 15, and 34% age 16 or older.

Achievement of Adoption

Of all children who exit foster care to adoption, Iowa data reflects approximately 40% do so within 24 months of the latest removal, which exceeds the national standard of 32% or greater. Iowa has also, in the most recent reporting period, seen increase in the percent of children in pre-adoptive placement. Of all children exiting to adoption, 1% were under age 1, 50% between age 1 and 5, 31% between age 6 and 10, 15% between age 11 and 15, and 3% over age 16. The average age of all children who were adopted during the period is 7 years old. The average time these children have spent in care is 27 months.

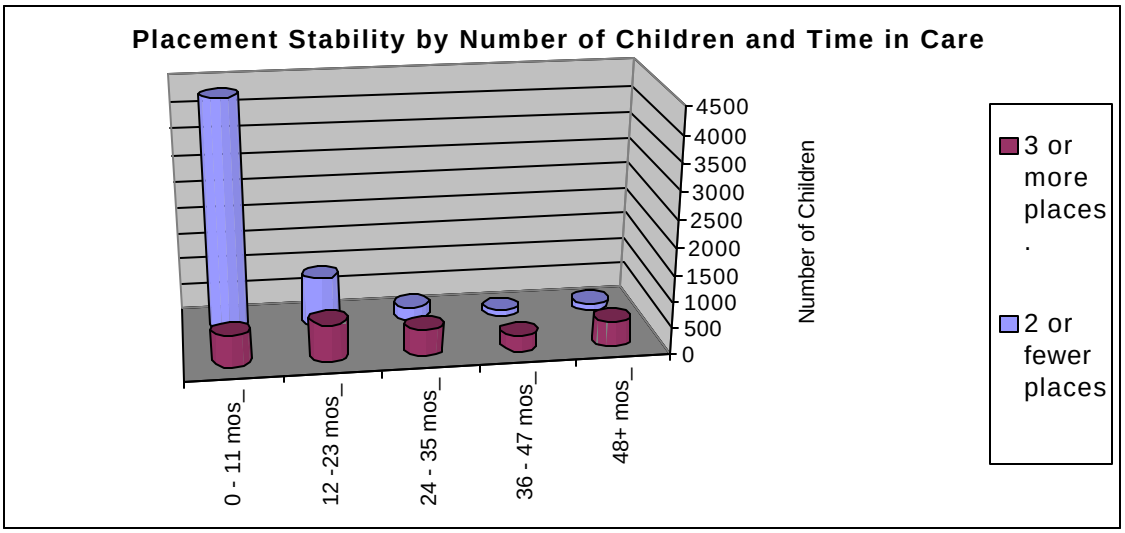


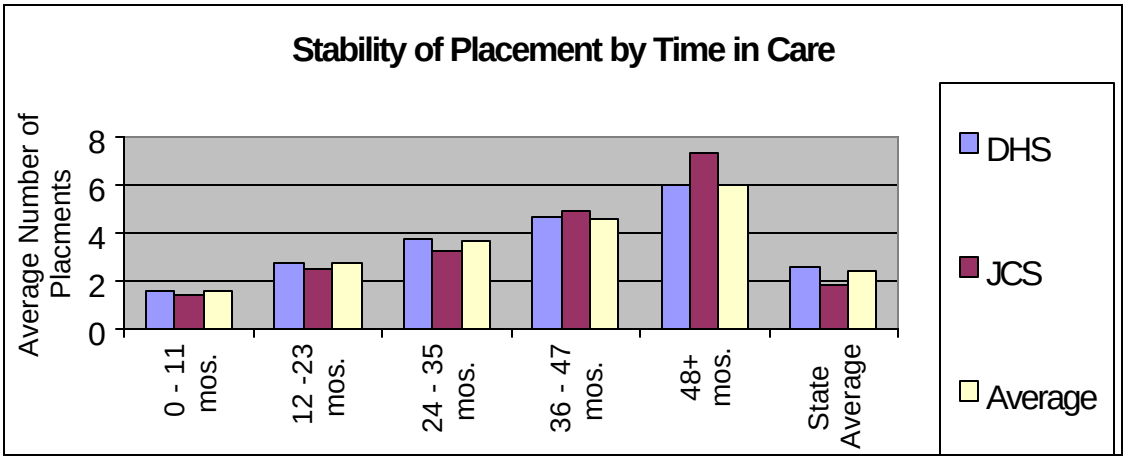
Termination of Parental Rights

Of the children in care on the last day of the period, 19% had termination of parental rights. Of the 19% with TPR 2% were under age 1, 32% were between 1 and 5, 25% were between 6 and 11, 30% were between 12 and 15, and 11% age 16 or older. The average age of all children whose parental rights have been terminated and remain in care is 11 years old. The average time these children have spent in care is 18 months. The majority of these children's current placement setting is reported as non-relative family foster care at 54%, next most frequent is pre-adoptive homes at 32%, followed by group home at 8%, Institution (Psychiatric Medical Institution for Children) at 5%, and Independent Living at 1%. Iowa nearly had a subsidized guardianship program, however legislation was withdrawn shortly after enactment and the program was never implemented due to state budget shortfalls. Such a program could have helped older children attain permanency, and support placement into relative homes when the relatives are unable to meet the child's special needs without assistance.

Stability of Foster Care Placements

The stability of children in care, measured by the percent with two or fewer placement settings, has been about 88% (plus or minus 1%) over the past three years which slightly exceeds the national standard of 86.7% or greater. This measure illustrates that children in care less than a year do not "bounce" from one placement setting to another within a spell of out of home care. Due to Iowa's short length of stay, the majority of children is in care under a year and has less than 3 placements. The children who remain in care longer than a year experience declining stability as illustrated in the charts below.

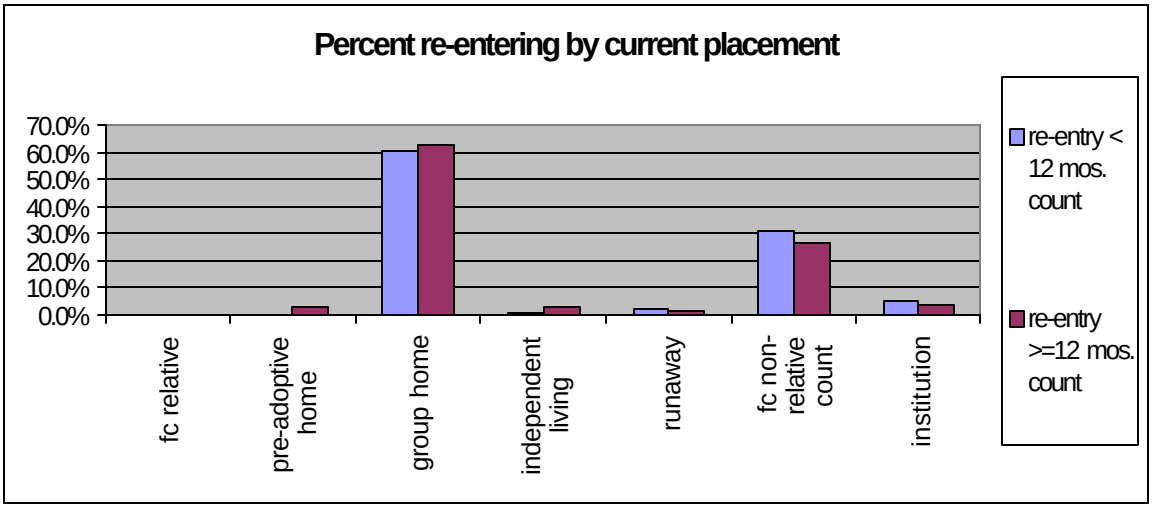




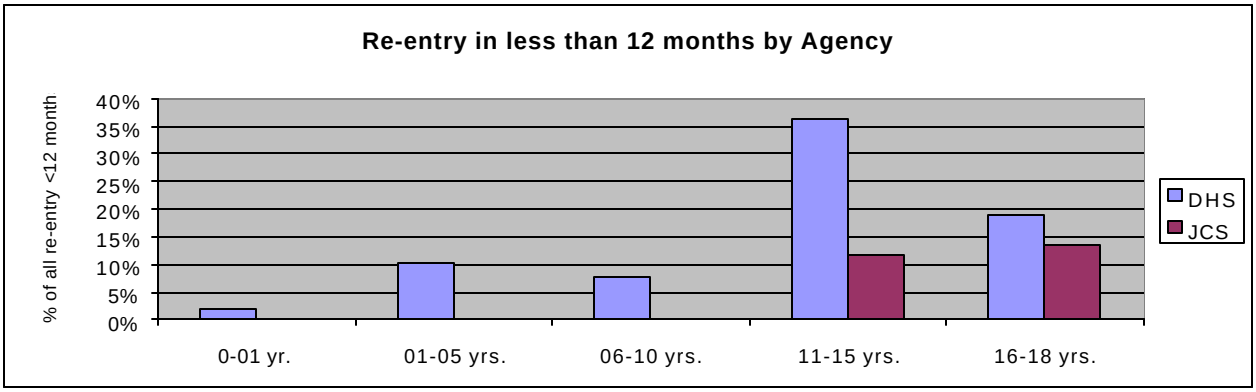
Foster Care Re-Entries

Iowa is not in compliance with the national standard of 8.6 or fewer percent of admissions are re-admissions within 12 months of a prior admission. Children in Iowa re-enter care at a rate of approximately 25% (plus or minus one percent) over the past three reporting periods. While each prior period has seen the rate decrease (by approximately one percent per year) Iowa needs to better understand the underlying factors which contribute to this rate of re-entry which exceeds the national standard of 8.6% or less. Of all children in care during the period, 25% re-entered within 12 months and another 8% enter more than 12 months from a prior episode.

Most of the children re-entering (regardless of the time to re-entry) are placed in group care. Children re-entering care within 12 months are usually (60%) placed in group care, with non-relative family foster care next (30%), and Psychiatric Medical Institutions for Children next (5%), and three other categories contributing the combined remaining five percent.



Of all children re-entering care within 12 months, juvenile justice children account for 6% of the 25% re-entry rate, with DHS accounting for the other 18% (one percent missing data to calculate time to re-entry). Most of the children re-entering within 12 months are age 11 or older (80%). The children age 11 or older are nearly evenly distributed between boys (53%) and girls (47%).



Looking at Iowa's first time entry cohort an increasing percent of all entries are 1st time entries over the past three FFY reporting periods. Each year shows a one- percent increase in first time entries over the prior period.

Length of Stay in Foster Care

The lengths of stay for children in care in Iowa are significantly less than the national averages. Iowa data reflects median lengths of stay of approximately 11 months plus or minus one month, over each of the past three years. National data reflects a median length of stay of 20 months. Iowa has a well developed array of in-home services which is designed to help children safely remain home, and to return home from care as soon as safely possible. Iowa's success with short lengths of stay is offset by high re-entry rates, which illustrates we are not as effective as we would like to be reunifying children.

Other Permanency Issues

The other issues impacting permanency in Iowa include:

- Comprehensive assessment and re-assessment
- Improved family engagement
- Use of family teams and team conferencing
- Primary child welfare services "RTS" focus on behavioral health Vs safety/permanency/wellbeing
- Line staff caseload and supervisor caseload
- Lack of effective concurrent planning
- Lack of effective transition planning (and implementation)
- Loss of permanency resources due to fiscal limitations(e.g. Permanency Planners, Mediation Program, Permanency for Teens Program, Subsidized Guardianship Program, Kinship Care Program)
- Loss of prevention programs due to fiscal limitations (e.g. serving voluntary cases, decategorization programs, reduction of services to at risk youth with disabilities

Evaluation and Summary of Permanency Findings

A number of factors relate to how well children in Iowa fare regarding permanency. The most significant implication for Iowa's system is the high percentage of children who return home (are re-united) following what are typically short (compared to national averages) stays in foster care. Also related to the percent of children who return home after short stays, is the high percentage of children in care under one year with two or fewer placements. Short stays and high reunification rates improve the likelihood that children are not experiencing frequent moves during an episode of out of home care.

Another significant strength of the system in Iowa is the increasing number of children who are attaining permanency through adoption, and who are reaching permanency more quickly than in the past. Children in Iowa who attain permanency through adoption do so at rates that consistently meet the national standards for the length of time to achieve adoption. Iowa initiated reforms to improve permanency long before the passage of the Adoption and Safe Families Act, and leveraged additional focus and public attention on permanency through the Adoption and Safe Families Act.

Iowa has dedicated and competent staff employed in the child welfare system including the courts, private providers, juvenile court staff, and staff of the child welfare agency. The degree to which children in Iowa experience good safety and permanency outcomes is a testament to the resiliency and efforts of all those staff's ability to persevere in the face of economic factors which contribute to increasing demands on the child welfare system coupled with dwindling resources due to declining state budgets.

Concerns that detract from good permanency outcomes are manifest by the high re-entry rates in Iowa. While children are re-united quickly, too often they lack long term permanency in the parental home. Re-entry is especially a problem for youth over age ten, and specific strategies are needed to improve results for this population. Some of the factors which focus groups have indicated influence success of children returning home include:

- Caseloads that are too high to support development and maintenance of a strong therapeutic relationship between family and caseworker. This tends to lead to paying the most attention to crisis and not supporting on-going needs of children and families until things escalate to levels where placements can not be salvaged and this tends to decrease the likelihood that good transition planning occurs and is implemented in a timely way according to plan.
- Iowa's high utilization of group care may also contribute to lack of permanency and high re-entry rates. Group care placements are less likely than other types of care to be near the family home. One effect of that distance may be to compound the existing challenges in the system to design and implement effective transition for children exiting care to return home.

Due to financial constraints, Iowa has seen pressure to provide treatment to children in family foster care who may have been likely to receive group care in the past. These children present a number of challenging behaviors that require different training and new and different supports for foster families to effectively address. In response to permanency well being and abuse while in care, Iowa has developed "Preventative Practices" training for individuals providing family foster care. The intent of the training is to better prepare foster parents to deal with the challenges of parenting foster children. Another change being implemented in Iowa is an increase in pre-service training requirements. This change nearly doubles the amount of training required to become licensed as a foster parent and is intended to better prepare foster families to effectively and safely care for children.

Iowa has specific services, which also contribute to re-entry rates (as well as length of stay data). One such program is an intensive in home treatment program, which includes a very brief out of home component to stabilize crisis situations and extreme behavior and then return the child home. These placements typically last 2 days to 2 weeks while treatment plans and supports are modified to meet the child's needs and prepare for return home. These programs increase both admissions and discharges from care, decrease average length of stay, and increase rates of reunification. It also contributes to higher re-entry rates when more than one course of out of home care is needed. More importantly, it contributes significantly to maintaining connections between the child and family and community, and strategically avoids lengthy stays in out of home care. This does not address all of Iowa's re-entry rate, merely a small portion (about 6 %).

Prior to the state's preparation of the Statewide Self-Assessment an independent review of Iowa's child welfare system occurred initiated following a high profile child protective case. The review, conducted by the American Humane Association made the following recommendations related to Permanency Issues:

- Strengthen the supervisory process by having more supervisors with adequate clinical background
- Use multi-disciplinary teams for information sharing
- Improve the functionality and effectiveness of the case planning process
- Reduce the caseloads of line staff and the span of control of line supervisors
- Employ a form of family team conferencing to help address needed improvements in assessment, family engagement, coordination, case tracking, and provider accountability

Child and Family Well-Being

Child and family well-being in Iowa is generally good for the overall population. Iowa enjoys a very low unemployment rate, and has few areas with severe poverty. Iowa also has one of the highest rates of two parent households with both parents working which can add stress to families working at low wages. As a rural state, Iowa also has a strong sense of family and community connectedness, which is an asset to all families.

As we work on our PIP we will develop better strategies for collection of information to assure the well being of children in our care.

Visitation of Children in Care

Iowa has policy that governs the frequency of visits between the worker and child, worker and service provider and between the child and parents and siblings. Policy directs visits to occur at least every 45 days, and more frequently when the individualized needs of the child and family dictate. Because Iowa's child welfare system purchases most services, the foster care providers and foster parents also contribute significantly to the routine supervision of child safety through their contact with the child and family. The only statewide quality assurance efforts on visit frequency were supervisory case readings that have been lost in the wake of recent budget reductions in the past year. The decision to target supervisory and management staff was strategically made to minimize the impact on line caseworkers that already had caseloads significantly higher than any recommended levels. Line staff who responded to the staff survey developed for the statewide assessment showed an average caseload size of 55 cases per worker. Line staff do have reminders generated in the SACWIS system to help them know when visits are due, and are expected to record visit dates in the system.

Educational Status of Children in Care

Iowa has a strong history of good educational programs, and those strengths benefit children in care. Iowa does lack routine statewide coordination of educational services, so some areas enjoy better coordination and therefore results than others. Iowa's educational system is in fact largely locally managed at the local school district level. While this can be a strength in local responsiveness to unique needs, to do so requires both adequate resource in terms of both educational and child welfare staff to identify needs and coordinate responses. Both systems have suffered in terms of funding and it's impact on resources which presents barriers to achieving the level of cooperation each agency would like to invest to meet the needs of the children in the child welfare system.

Child Health and Mental Health

Children in foster care are enrolled in Medicaid to meet physical and dental health needs, and are enrolled in the state managed mental health plan to address any mental health needs. Physical health screens are required within 30 days of entering foster care and at least annually thereafter. Meeting the health care needs of children in care has not been identified as a major problem. Challenges to meeting the physical health needs of children in foster care however mount when children move from home communities because the continuity of treatment and health history often fall short due to changes in physician with each move. Focus group feedback has also

identified challenges in locating providers willing to accept Medicaid payment especially in rural areas of the state. Anecdotally we know that because reimbursement rates for dental care are low, some Iowa dentists are no longer serving Title XIX patients which can limit accessibility, especially in rural areas. Enrollment in the managed mental health plan assures access to inpatient services when needed, but focus group feedback strongly identifies a lack of available providers of out-patient services and most drastically in rural areas. The state does not have a specific array of mental health services for children, and often attempts to meet mental health needs as possible through the child welfare system.

Focus group comments are not entirely consistent with Iowa Plan data which indicates about 22,000 children receive mental health or substance abuse services in a year. Of those children, about 7,500 also have a child welfare worker assigned so it can be assumed the child is receiving child welfare services. Over $23 Million is spent each year on mental health and substance abuse services on behalf of children. In rural areas, Community Mental Health Centers provide mental health services.

Information indicates that DHS and Magellan need to work more closely to assure that case workers are aware of where mental health services can be accessed in their area and to assist them in assuring that children's mental health needs are being met.

Focus groups and survey respondents clearly indicated a need for better mental health services for all ages. The Mental Health Planning Council has identified mental health services for children in child welfare and juvenile justice as one of the priorities for the upcoming year.

State Assessment of Strengths and Needs

Strengths

1. What specific strengths of the agency's programs has the team identified?

General

A basic tenet of the Iowa child welfare system is strength-based practice with families. As a parallel process, we believe that the strengths in our child welfare system provide us with the foundation upon which we can build for making improvement.

Following is a summary of the major strengths of Iowa's child welfare system that apply across outcomes and systemic factors.

- First, one of our greatest strength is the quality and characteristics of our staff -- human-services workers, juvenile court officers, and supervisors who do their jobs well under what are often extremely difficult working conditions. Despite caseloads that are among some of the highest in the nation, staff demonstrate dedication and commitment to protection of children, assisting families in improving their ability to care for their children.
- Another strength is the contribution of our many child welfare partners, especially Juvenile Judges, our private providers, and our communities. They are our strongest child advocates and consistently strive for excellence even in difficult times. The willingness of our partners to work together with us to assess and improve our child welfare system is an invaluable asset.
- A third, over-riding strength is our history of collaboration at the state and local level, including efforts such as Empowerment, Decategorization, and Community Child Protection Partnerships. This collaboration enhances the department's ability to respond to the changing needs of families.
- A fourth over-riding strength is our strong statutory and policy focus on child safety and strengthening families .
- Finally, Iowa has a strong history of innovation that reflects our commitment to improving outcomes for the children and families we serve.

Child and Family Outcomes

Safety: "Safety first" is demonstrated both in policy and in practice. Policy and practice changes have been made in the last two years as a result of the tragic death of a toddler in our state. Within the resources allocated to the department, DHS has worked to implement the recommendations from the assessment of our child welfare system by the American Humane Association and the Ombudsman's office.

Quality Service Reviews [QSR] of 160 cases in the last 3 years have determined that children are safe and that generally child and family needs are being addressed through the services provided. Safety results from the QSR indicate that 94% of children reviewed had adequate safety status, including 64% having favorable to optimal safety status.

Performance indicators of safety that can be considered strengths include:

- Iowa has one of the highest standards in the nation in terms of time frames for responding to reports of child abuse Staff are expected to have face-to-face contact with a child within 1 hour in cases involving an immediate threat or high risk to the child's safety, 24 hours when the report doesn't involved immediate threat or high risk to the child but the person alleged responsible has access to the child, and 96 hours when the report doesn't involve an immediate threat or high risk to the child and the person alleged responsible clearly does not have access to the child. Child protective supervisors monitor compliance with these timeframes.
- Services are provided to families to protect children in their home and prevent removal. There is generally intensive provider contact in most of the home-based cases, which helps identify evolving safety risks and assure ongoing safety where there are known and manageable risks of harm.
- Assessment of risk of harm is being enhanced by implementation of a risk assessment tool piloted 10/02 with fully implementation planned for 09/01/03.
- Case planning focuses on safety. Case plans provide a format for documenting safety plans and statewide training has been provided in assessing risk of harm and developing safety plans.

Iowa has chosen to implement Community Partnerships for Protecting Children [CPPC] as one of our key strategies for improving the safety of children. CPPC focuses on individualized services to meet the specific needs of children at risk. In CPPC sites, CPS workers convene a family team meeting with families, neighbors and local service providers that results in tailor-made plans designed to support the family and ensure the safety and well being of the children. These plans identify the specific activities to be carried out by parents, friends, extended families and other formal and informal supports. Each Community Partnership creates a network of agencies, neighborhood and community supports that are available to families before, during and after crisis occurs. Citizens and community members are directly involved in shaping the types of services and supports that are made available to families through shared decision making. Localities shape their own strategies and develop a range of services based on their own resources, needs and cultures. Community partnerships influence the development and adoption of new policies, practice, roles and responsibilities. Currently, 5 sites involving 12 of Iowa's 99 counties are implementing the CPPC model. DHS is committed to working with communities across the state to expand CPPC to other counties.

Permanency: Iowa maintains a firm foundation for permanency in policy and practice despite funding cuts in the last three years that have significantly decreased or eliminated specific permanency programs, i.e. mediation for permanency, permanency for teens project, subsidized guardianship, permanency specialists, and the parent-child visitation project.

Iowa's adoption program continues to be strength, and was recognized nationally with the Adoption 2002 Excellence Award in 1999. As noted in earlier sections, DHS has increased the number of adoptions consistently over the last 9 years.

In Iowa, we have the strong value to maintain children in their family of origin when safe, possible, and appropriate. This value is demonstrated in our practice and reflected in our high re-unification rate. Our court system and juvenile court judges are strong advocates for permanency and monitor to assure that there is progress towards permanency for children.

Performance indicators of permanency that can be considered strengths:
- Stability of foster care placement

- Length of time to achieve reunification
- Adoptions completed within 24 months from placement

Well-being: Generally families have enhanced capacity to provide for their children's needs. Family and children's need are assessed. When social workers are unable to maintain at least monthly contact with families and foster children, there is provider contact and communication with the worker.

Quality Service Reviews indicate that 68% of the cases reviewed had excellent to fair assessments of need. Where improvement or refinement was indicated, a current functional assessment that focuses on underlying needs rather than symptoms, as well as improved information sharing among team members were the primary practice improvements recommended.

Generally children's educational, health, and mental health needs are being met. QSR reviews indicate:

- 81% of children reviewed had their educational needs adequately met, including 43% of children whose education needs were optimally met
- 100% of children reviewed had their health needs adequately met, including 83% of children whose health needs were optimally met
- 87% of children reviewed had their mental health needs adequately met, including 23% of children whose mental health were optimally me.

Key strategies for improving well being for children are the use of family team meetings, development of a functional assessment guide, and planned implementation of a standardized assessment tool [North Carolina Children and Family Assessment Scale (NCFAS)]. These strategies will:

- Enhance our ability to adequately assess the identified and underlying needs of the child and family within timeframes
- Promote information sharing with the family team and provide the family team with a common view of strengths and concerns
- Provide mental health needs screening for children in foster care
- Involve the family and the team in case planning

Systemic Factors

Based on our statewide assessment, the following strengths were identified with respect to CFSR systemic factors.

Statewide Information System Capacity - Iowa has a comprehensive management information system that meets federal reporting requirements, is assessable to all staff and also supports other administrative and fiscal functions.

Case Review System - Iowa's case review system meets or exceeds state and federal requirements for periodic reviews. There is a high level of awareness of ASFA requirements and excellent judicial support. The Iowa Court Improvement Project, court training and the Court Appointed Special Advocate Program strengthen the system.

Quality Assurance System– DHS policy reflects a strong set of standards to ensure that children in foster care are provided quality services that protect their health and safety. DHS has

conducted quality service reviews in a number of counties, and has used the information gained from the reviews to improve policy and practice at the state and local level. DHS has established quality assurance positions in each of the 8 service areas and in the Office of Field Support to support quality assurance and improvement activities. The Division of Results Based Accountability is available to state and local offices for technical assistance and support.

Staff and Provider Training – Despite significant reductions in funding, DHS has been able to maintain new worker training, and has taken training "on the road" to service areas to continue to provide in-service to experienced workers. Joint training with domestic violence and substance abuse providers has also been a positive. Iowa is implementing a new training curriculum for foster and adoptive parents, designed to better prepare them.

Service Array and Resource Development - Iowa has a rich history of innovative programs including strong prevention programs, permanency programs, and reinvestment options. There exists a wide array of statewide services. Iowa has a diverse group of stable non-profit providers who are dedicated to meeting the needs of children and youth. The Coalition for Family and Children Services offers continuing education and ongoing support of their members. They partner with the department on child welfare improvement initiatives and serve on committees at state and community level. The state also encourages community involvement in responding to unique needs of children and families - there is support for local practice improvement strategies. Even though the array of services varies around the state, local efforts like Decategorization, Empowerment, and Community Partnerships for the Protecting Children are assets to develop and advocate for needed local services.

Agency Responsiveness to Community – Iowa has a history of consultation and coordination with community stakeholders at both the state and local levels. Stakeholders have been actively involved throughout the CFSR statewide assessment process. In the last year, DHS has worked particularly on collaboration with the domestic violence community.

Foster and Adoptive Home Licensing, Approval and Recruitment - Iowa has a statewide focused plan to recruit and support the needs of adoptive and foster parents. A strong partnership between state agency staff, the statewide foster and adoptive parents association, and provider agencies enhances the ability to recruit and retain foster and adoptive parents. QSR found that caregiver functioning was a strength statewide. 90% of the cases reviewed showed that caregiver functioning was adequate to optimal. This correlates with the finding that 93% of foster children were placed in adequate settings, with 66% of the children in these cases were placed in favorable to optimal settings.

Critical Needs for Examination

2. What specific needs has the team identified that warrant further examination in the Onsite Review? Note which of these needs are the most critical to the outcomes under safety, permanency, and well being for children and families in the State.

We would like the federal on-site review to focus on the following needs; and on identifying potential strategies to address these needs, including best practices that other states are employing.

- recurrence of abuse for causal factors [safety]
- re-entry into foster care for causal factors [permanence]

- maintaining continuity of family relationships and connections for children in foster care [permanence]
- the quality assurance system ability to evaluate adequacy and quality of practice [systemic factor]
- effectiveness of initial and ongoing training for DHS, providers, foster parents and Judges, etc. [systemic factor]
- effectiveness of the various types of review in Iowa, as well as suggestions for the effective and efficient use of citizen review to achieve safety and permanency [systemic factor]

In addition to evaluating needs, we would like the federal review to focus on promising and innovative practices, such as family team meetings, mediation, and the community partnership initiative. How do these practices impact outcomes for children and families?

General

Caseloads and Supervisory Span of Control: Iowa's caseloads and supervisory span of control are some of the highest in the nation. Adequate numbers of staff are needed to:

- Assess needs
- Engage the family
- Craft timely individualized case plans
- Work collaboratively with family team members and with community partners
- Arrange and monitor provision of effectively designed services with multiple providers
- Provide tracking and adaptation needed to achieve case goals

"When caseloads are high, caseworkers regularly deal with 'today's crisis' leaving little time to effectively plan. Not only do high caseloads limit the time available to be spent with individual families, but the challenge of mastering the complex policies of so many different programs would makes it challenging to perform adequately and accurately on all of them. High caseloads among DHS staff have the greatest negative effect on performance in this area and in the coordination category. Likewise, increasing caseloads for Juvenile Court Officers and reduce dollars for services leave resources stretched and leave families with reduced resources and options. Workers are challenged to find the time to schedule and organize regular team meetings and to attend them, with resultant negative consequences for system performance and child and family outcomes. Caseloads need to be lowered if workers are expected to fully address the complex needs of the families they serve." [Paul Vincent, Quality Service Review Summary 6/20/02]

"Community partners empathize with DHS staff and the demands presented by their workload. There is concern that the reorganization and increased caseload size is forcing DHS staff to be more and more out of touch with what's going on with families." [Paul Vincent, 6/20/02]

Supervisory ratios are also high and present a challenge to effective coaching, mentoring, practice, and on-the-job staff training. Supervisors are challenged to find the time to provide workers with specialized clinical consultation and specialized assistance for difficult or complex cases.

We would like the federal on-site review to evaluate the effect of high caseloads on our ability to protect children and achieve permanency, and to achieve substantial conformity with systemic factors.

Polk and Linn Counties have high staff turnover compared to other parts of the state. We are aware that high staff turnover can present additional challenges in achieving positive outcomes for children and families. We would ask that you examine the high staff turnover for its effect on outcomes for children, casework and partnerships within the community and suggest potential strategies Iowa could take to address this, especially in light of the state's limited fiscal resources.

Child and Family Outcomes

Safety: Our recurrence of maltreatment rate, 11.2%, exceeds the national standard, 6.1%. Of all repeat maltreatment, 68.5% is the same or similar circumstances; 58.3%is the same perpetrator. Our data indicate that:

- Sexual abuse occurs in 7.6% of all abuse cases. Of repeat abuse, 2.3% is repeat sexual abuse
- Physical abuse occurs in 17.8 % of all abuse cases. Of repeat abuse, 2.5% is repeat physical abuse
- Neglect occurs in 68.2% of all abuse cases. Of repeat abuse, 62% is repeat neglect

Further examination of causal factors for repeat maltreatment, especially neglect, needs to be made.

The incident of abuse and/or neglect in foster care, 0.89% exceeds the national standard. An initial examination of the abuse in foster care suggests the following contributing factors:

- Training provided does not adequately prepare foster parents for the challenges of foster parenting
- Matching foster children with foster parents who can meet their needs is inadequate due to the lack of readily available homes to meet need
- Lack of adequate ongoing support or supervision of foster parents
- Over using foster homes or exceeding the recommended capacity.

We would be interested in identifying potential innovative strategies to address these issues, including best practices that other states are employing.

Permanency: Of the 160 cases reviewed in QSR – 26% had optimal or substantial permanency and 58% had minimally adequate permanency.

Iowa meets the federal standard for all but one of four CFSR data indicators of permanency. Our re-entry into foster care rate, 26.6%, exceeds the national standard, 8.6%. QSR indicates that a causal factor is inadequate assessment of the underlying causes that let to the child's placement. QSR has suggested that re-entries that could be prevented occur when the new pressures that a parent may experience when a challenging child is reunified are not understood, and that the system fails to wrap appropriate supports around the family. In cases where transitions were successful, there was evidence of good team planning, including full involvement of the family. Plans and supports were designed to meet needs as they emerged.

Causal factors to be targeted for improvement of this standard include:

- Funding barriers that limit family treatment during a child's placement
- Inadequate transition planning from placement to home
- Availability of after care services that match the needs of the family and the child
- The role of the court in re-entry needs to be assess

Other permanency areas that need targeted improvement include:

- Focusing on the long term view in case planning
- Provision of independent living transition planning and services
- Consistent practice in the following permanency requirements - involving family in case planning, preserving continuity of family relationships and connections, proximity of placement, placement with siblings, family visitation with foster children, diligent search for relative placement

Well-being

Assessment: Standardized assessment tools and scales are being piloted with full implementation expected by September 2003. The addition of these standardized assessment tools and scales is designed to address the following issues. Note: the assessment tool will not be operational in the CFSR sites.

In the sample of 160 cases reviewed by QSR, 14% had optimal functional assessments, 54% had minimally acceptable functional assessments

- In many of the cases reviewed, the functional assessment of the child and family was not sufficient to permit the underlying conditions of need to be addressed
- The assessment of needs did not consistently reflect the family's perspective of its functioning and needs
- Existing assessments tended to focus on symptoms, such as defiant behavior, rather than the underlying causes like attachment and loss issues. As a result, the case plan, anticipation of transitions and long-term view were impaired; and child and family outcomes were not satisfactory

Strengths were identified by QSR in the following areas:

- Many successful cases demonstrated a strong reliance on family strengths as a foundation of the assessment process was lacking
- In many child protection cases, effective team functioning contributed greatly to sound decision-making about removal or reunification by adding expertise and accountability

Challenges to address in our program improvement plan include:

- Often, teams did not have a unified 'big picture view' of the child and family - the effects on coordination were problematic
- There were examples where contributors to the case had developed a good assessment of elements of the case, but it had not been integrated into a single "big picture"
- Cases involving mental health diagnoses, substance abuse and domestic violence were the most challenging to assess, in part because team members with expertise in these areas did not communicate face to face with other team members routinely
- The contributions made by families to their own assessment and identification of needs were limited

Case Planning*:* The case plan is being revised to address many of the following issues. When family team meetings are fully implemented, many more of the following issues will be addressed. QSR reviews indicate that 52% of case planning is adequate; 11% of cases reviewed had good to optimal service planning. Service implementation was found to be 54% adequate and 27% optimal implantation of the service plan. It was found that families were successful when their cases included the following elements:

- a functioning service team
- communication among its members

- family team meetings routinely held
- the family is an involved as a member of the service team
- the family is a part of developing the case plan.

QSR identified a strong correlation between the adequacy of informal supports and a positive child and family outcome. For many of the cases reviewed, permanency could be enhanced with better functional assessments. Many cases lacked clear, concurrent planning. Interventions were not successful when they do not address the emerging need. In the cases reviewed the service team did not routinely anticipate transitions and take steps to manage them. Service teams often found themselves struggling with crises that might have been anticipated. The service teams did not routinely anticipate transitions, and take steps to manage them. Teams need to develop the capacity to see cases with a long-term view is essential to establishing timely permanency. Identifying and addressing transitions will support better integration of a long-term view into the case plan. It was also noted that teachers were often not a part of the child's service team

Case plan issues are being addressed through redesign of the case plan and training. This alone with not be successful unless there are adequate numbers of staff to complete case work functions.

Challenges to address in our program improvement plan include:

- Cases, goals and expectations in the case plan did not always match the unexpressed thoughts and considerations of the service team members (e.g. reunification may have been the expressed goal, yet the team believed that it is unlikely that reunification will be possible and kept raising the conditions for a child's return home.)
- This lack of clarity about intentions further distances families from effective relationships with professionals
- Family strengths were found to be listed in most case narratives, conversations with professionals did not reflect that strengths were consistently influential in the development of plans and provision of services and supports
- The frequent changes in DHS and provider staff and high DHS caseloads made the development of trusting relationships difficult

Worker Visits with Children and Parents: Based on staff survey results, believe we are not in substantial conformity with the federal expectation for worker visits with children and parents. The critical factor in meeting this standard is adequate numbers of trained staff.

As important as frequency of visits is the degree to which our staff engage families in a partnership to address concerns and achieve positive results. Child and family engagement is fundamental to good outcomes. In the 160 case review by QSR, 79% had adequate child and family engagement of which 32% had optimal engagement. QSR related the following issues related to engagement:

- QSR uncovered many examples where workers cared about their families, but because of workload issues, had little direct contact with them and, thus, little opportunity to develop a trusting relationship
- When families were successfully engaged they reported feeling respected by DHS intake, service, income maintenance (IM), and child protection workers and were generally very satisfied with the services they receive(d)
- Good family engagement translates into a willingness on the family's part to benefit from services. These families were less likely to be 'revolving door' consumers of services

- The use of family team meetings is a positive contributor to the effective involvement of families

In cases where family engagement was successful - families were meaningfully involved in case planning and attention was paid to their underlying needs. Families who are successfully engaged report:

- a trusting relationships with DHS
- being involved in case planning produces
- score consistently better safety, permanency and well-being outcomes

We would be interested in identifying potential innovative strategies to address these issues, including best practices that other states are employing.

Systemic Factors

Based on our statewide assessment, the following needs were identified with respect to CFSR systemic factors. As noted above, we would be interested in help identifying potential innovative strategies to address these issues, including best practices that other states are employing.

Quality Assurance System: The department currently lacks a statewide comprehensive, coordinated quality assurance system to assess systemic factors and the effectiveness of the department's service programs. While we have begun to develop a basic infrastructure for quality assurance, we recognize that this is a capacity that we need to more fully develop

Staff and Provider Training: Budget cuts have resulted in limited ongoing training course offerings and staff find it hard to find time to attend training due to high and rising caseloads. Staff also indicate that need more training regarding Indian Child Welfare Act (ICWA) and the Multiethnic Placement Act (MEPA).

Service Array and Resource Development: Iowa's service array has been impacted negatively by budget reductions over the last few years. This is especially true with respect to prevention and early intervention services. Iowa also needs to address gaps in services for mental health and substance abuse treatment. Program rules need to be more flexible and to focus more on the family and not just on the child.

Agency Responsiveness to Community: Iowa has a history of consultation and coordination with external community stakeholders at both the state and local levels. DHS has remained committed to these collaborations, even in times of staff reductions and funding cuts. Collaboration has not always reached a partnership level that is required in addressing the challenges of protecting children in our current circumstances. Recently, reductions in DHS staffing and reorganization have disrupted previously established local partnerships and diminished agency capacity for engaging and supporting communities. Initially, DHS had intended to have one community liaison position within each service area; however continued tight budgets have resulted in only 2 of the service areas being able to fill those positions. DHS promotes and participates in collaboration with the Native American community at the state and local level, but improvement is needed to develop a greater awareness and capacity to respond to a population which is becoming increasingly diverse.

Proposed Sites

3. Which three locations, e.g. counties or regions, in the State are most appropriate for examining the strengths and concerns noted in the Onsite Review?

The Department of Human Services administers services statewide with a central administrative office in Des Moines, 8 Service Area offices across the state, and offices in each of 99 counties either full time or part time. There are 8 counties that can be considered metropolitan [population center of 50,000 or more], 45 counties that can be considered small urban [population center between 5,000 and 50,000], and 46 counties that can be considered rural [no population center of 5,000 or more]. Rural counties have not been considered for site selection because the population would not provide an adequate number of sample cases. Iowa initially considered proposing one small urban site, but has modified the initial proposal after negotiation with HHS to recommend on-site in 3 of Iowa's eight largest counties. Polk is included because it is the largest county. Linn is recommended because of some unique practice related to the Clark Community Partnership which appears to be very promising practice. Woodbury is recommended because of some promising practices, and although Iowa has a small Native American population, Woodbury County is home to the majority of Iowa's Native Americans.

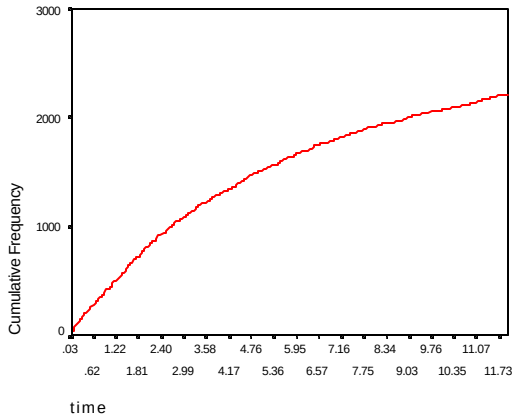
Iowa has a total population of 2,926,324 and a child population of 733,638 according to the 2000 US Census. Iowa has experienced a 5.4% growth in population during the 1990's, which was the second highest rate of increase of all census reports since 1990, however was slower than the growth of other states in the upper Midwest. While Iowa population growth is slower than the Midwest, no state in the upper Midwest is growing as fast as the national rate. Population growth has occurred in urban areas, while populations in rural areas have declined.

The population gain among residents 18 and older was much greater than that for youth between 1990 to 2000. Thus, the percentage of all Iowans 17 years of age or younger declined slightly to a level lower in 2000 than that observed in all previous censuses. In contrast to national averages, Iowa's population of children in foster care has a larger percent age 11 and older. Iowa has 15% fewer children in care than national average that are 10 and under, and 14% of Iowa foster care population is delinquent and more likely to be over age 10.

	Age 10 and under	**Age 11 and over**
National Average FFY 2000	53%	47%
Iowa Foster Care Population from FFY 2002B	38% (3190)	62% (5262)

One area of growth for youth was minority youth. The percent of minority youth has doubled since 1978, reaching 10 percent in 2000. When examining the increases of minority youth, 13 Iowa counties have experienced growth at rates above the state average. The counties with rates above the state average include one projected CFSR site, Des Moines, while Cedar Rapids is not experiencing growth in minority as fast as the state average, nor is the small urban proposed site Woodbury County.

Iowa has a high re-entry into care rate. State wide, youth are somewhat more likely to re-enter care soon after reunification (half re-enter by the third month).



Iowa reports juveniles adjudicated delinquent in foster care in the AFCARS submission. Iowa has an agreement with the courts, which requires that juveniles adjudicated delinquent in foster care be offered the same protections and be subject to the same requirements as child welfare cases. Juveniles adjudicated delinquent make up approximately 12% of all cases reported in AFCARS. 82% of all juvenile cases are reported with a current placement setting of group care, with 6% in family care, and 4% in each institutions and independent living. Another way to look at the frequency of these cases is by the percent of the state child population that juveniles adjudicated delinquent and traditional child welfare case are represented in AFCARS. Child welfare cases reported in AFCARS represent 0.9% (range 2.1% - 0.2% by county) of the total child population, while delinquents reported in AFCARS represent 0.1% (range 0.4% - 0.01% county) of the total child population.

State Level Outcomes Data

The abuse data from DCDC01 submission and Foster Care data from AFCARS2002b submission indicates that the sites selected for the Child and Family Service review are representative of Iowa's performance reflected by the data indicators. Race of children in the child welfare system is similar to child population except for black children who are significantly over-represented (and corresponding decrease for white children). Juveniles and CW cases black children nearly equally represented (slightly higher JCS). Black children are represented nearly equally between Juveniles and CW cases (slightly higher JCS).

1.1 Reoccurrence Rate **[<6.1%]**	2.1 Abuse in care Rate **[<0.57%]**	4.1 Percent reunified in <12 months **[>76.2%]**	4.2 ReEntry <12 months **[<8.6%]**	5.1 Percent adopted <24 months **[>32%]**	6.1 Percent in care <12 months with 2 or fewer placements **[>86.7]**	2002b Total Cases	2002b New Entries	In home cases '02	In home last quarter '02
11.2%	0.89%	82.3%	26.6%	60.2%	88.4%	8452	3021	14,370	8,423

Race - Ethnicity	American Indian or Alaskan Native	Asian or Pacific Islander	Black	Hispanic	Unknown or unable to determine	White
Total Pop	0.3%	1.3%	2.1%	2.8%	1.3%	92.3%
IA Child Pop	0.4%	1.4%	2.9%	4.3%	1.9%	89.1%
IA CW Pop	2.1%	0.7%	11.5%	4.5%	11.3%	69.9%

The counties of Polk, Linn, and Woodbury are seen as representative for examining the strengths and concerns of the child welfare system in our state. These locations are open to examining practice and acting as leaders in implementing the Program Improvement Plan (PIP).

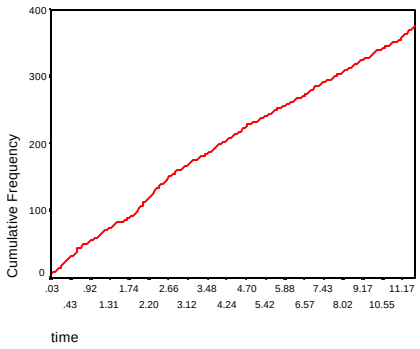
Polk County

Polk County is the largest metropolitan community in Iowa, is centrally located, and includes Iowa's capitol city, Des Moines. According to the 2000 US Census, Polk County has a total population of 374,601 and a child population of 96,300. From the AFCARS report, Polk County child welfare cases represent 1.5% of the county total child population (higher than state average). From the AFCARS report, juvenile adjudicated delinquent cases represent 0.2% of the total child population (higher than the state average).

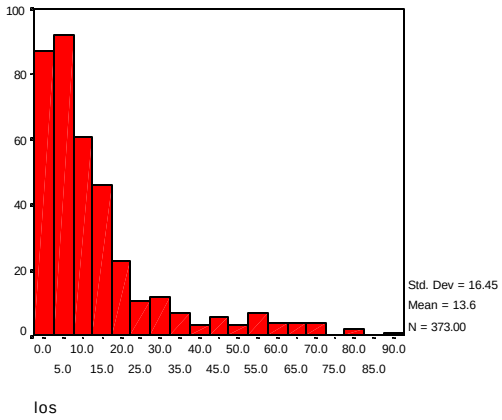
Polk County reflects the state average for age of children in care over and under age 10

	Age 10 and under	**Age 11 and over**
Iowa Foster Care Population from FFY 2002B	38% (3190)	62% (5262)
Polk County from FFY 2002B	40% (634)	60% (951)

Polk County re-entry data reflects the state average but unlike the state, children are as likely to re-enter day 1 as day 364 of placement (half reenter by sixth month, somewhat less likely re-enter as soon as rest of state).



When we look at 'length of stay time in months,' of the 373 cases from Polk who re-entered within 12 months of a prior spell (148 re-entered during the period), the average time in care was 13.6 months. Over one third of these cases are group care, and they are mostly child welfare cases.



As indicated in the chart below Polk County's data indicators reflect:

- The recurrence of maltreatment rate is lower than state average
- Abuse in foster care rate lower than state average
- The number of youth in care higher percentage than the state average
- The reunification rate is higher than the state average
- Re-entry rate similar to the state average
- Percent of children adopted within 24 months higher than state average
- Percent with 2 or fewer placements similar to state average

1.1 Reoccurrence Rate **[<6.1%]**	**2.1** Abuse in care Rate **[<0.57%]**	**4.1** Percent reunified in <12 months **[>76.2%]**	**4.2** ReEntry <12 months **[<8.6%]**	**5.1** Percent adopted <24 Months **[>32%]**	**6.1** Percent in care <12 months with 2 or fewer placements **[>86.7]**	2002b Total Cases	2002b New Entries	In home cases '02	In home last quarter '02
POLK									
9.7%	0.0%	85.3%	24.5%	67.1%	89.5%	1585	604	1552	897
IOWA									
11.2%	0.89%	82.3%	26.6%	60.2%	88.4%	8452	3021	14,370	8,423

In Polk County, black children are over represented in foster care, similar to the state.

Race - Ethnicity	American Indian or Alaskan Native	Asian or Pacific Islander	Black	Hispanic	Unknown or unable to determine	White
Polk Total Pop	0.3%	2.6%	4.7%	4.4%	2.2%	85.9%
Polk Child Pop	0.5%	2.5%	6.1%	6.5%	3.3%	80.9%
Polk CW Pop	0.4%	1.3%	17.7%	5.7%	21.3%	53.7%

Like many other urban and rural areas, Polk County has a demand for services to treat substance abuse problems involving both juveniles and their parents. It also has a strong, active juvenile court system that is focused on substance abuse issues.

Linn County

Linn County is located in Eastern Iowa, the geographical area of the state with the largest population concentration. This county affords an opportunity to examine a blend of urban and rural issues. According to the 2000 US Census, Linn County has a total population of 191,701 and a child population of 48,422. From the AFCARS report, Linn County child welfare cases represent 1.2% of the county total child population. From the AFCARS report, juvenile adjudicated delinquent cases represent 0.2% of the total child population (higher than the state average).

Linn County has a long history of community partnerships as well as use of innovative practices. Most notably, Linn is the site of an Edna McConnell Clark Foundation in Community Partnerships for Protection of Children Project (CPPC), one of four national sites for this initiative.

As indicated in the chart below Linn County's data indicators reflect:

- The recurrence of maltreatment rate is significantly lower than the state average
- Abuse in foster care rate lower than state average
- The reunification rate is slightly lower than the state average
- The re-entry rate is similar to the state average
- The percent of children adopted within 24 months is considerably lower than the state average
- Percent with 2 or fewer placements is similar to the state average

1.1 Reoccurrence Rate **[<6.1%]**	**2.1** Abuse in care Rate **[<0.57%]**	**4.1** Percent reunified in <12 months **[>76.2%]**	**4.2** ReEntry <12 months **[<8.6%]**	**5.1** Percent adopted <24 Months **[>32%]**	**6.1** Percent in care <12 months with 2 or fewer placements **[>86.7]**	2002b Total Cases	2002b New Entries	In home cases '02	In home last quarter '02
LINN									
5.8%	0.0%	81.7%	23.6%	26.1%	90.0%	644	220	841	475
IOWA									
11.2%	0.89%	82.3%	26.6%	60.2%	88.4%	8452	3021	14,370	8,423

Linn County is experiencing a slower growth rate in minorities than the state. Black and Native American children are over represented in foster care, similar to the state.

Race – Ethnicity	American Indian or Alaskan Native	Asian or Pacific Islander	Black	Hispanic	Unknown or unable to determine	White
Linn Total Pop	0.2%	1.4%	2.4%	1.5%	0.5%	94.0%
Linn Child Pop	0.2%	1.6%	3.8%	1.1%	4.2%	89.1%
Linn CW Pop	1.2%	0.8%	19.4%	2.4%	9.9%	66.3%

Linn County reflects the state average for age of children in care over and under age 10.

	Age 10 and under	**Age 11 and over**
Iowa Foster Care Population from FFY 2002B	38% (3190)	62% (5262)
Linn County from FFY 2002B	36% (239)	64% (425)

Woodbury County

Woodbury County is a larger metropolitan county in Northwest Iowa. Woodbury County is representative of the state's metro area including: having a good array of services available locally, transportation, and a full time office. According to the 2000 US Census, Woodbury County has a total population of 103,877 and a child population of 28,390. From the AFCARS report, Woodbury County child welfare cases represent 2.2% of the county total child population (higher than state average). From the AFCARS report, juveniles adjudicated delinquent represent 0.19% of the total child population (higher than the state average). This county is experiencing a growing population diversity that will present new challenges.

Because Native Americans make up a larger percent of the population in Woodbury County than any other Iowa county, the additional following background is provided to help illustrate some unique issues and needs of the Native American population in Iowa. According to 1990 census data, Native American males in Iowa have the highest rate of unemployment at 20.8%, followed by Black males at 14%, and Native American females at 13.5% of the total civilian labor force above age 16. The unemployment rate for white males is at 4.7% and white females at 4.0% (over 92% of Iowa's population is white). In 1989 per capita income (total persons in all age groups) ranged from a high of $12,573 for whites to a low of $6,707 for Native Americans (per 1990 census). The economic condition of Native Americans in Woodbury County is illustrated below with data from the 2000 census on median household income. These economic data are simple illustrations of the complexity of issues regarding children in the child welfare system, and some systemic challenges to improving the welfare for all children, Native Americans and others.

	Native American	White	Black	Hispanic	other	Total
Woodbury Median Income 1999 (2000 census)	$20,727	$39,146	$24,500	$34,432	$36,282	$38,509
% of Median County Income	54%	102%	64%	89%	94%	

As indicated in the chart below Woodbury County's data indicators reflect:

- The recurrence of maltreatment rate is lower than the state average
- Abuse in foster care rate significantly higher than the state average
- The reunification rate is slightly lower than the state average
- The re-entry rate is lower than the state average
- The percent of children adopted within 24 months is slightly higher than the state average
- Percent with 2 or fewer placements is slightly higher than the state average

1.1 Reoccurrence Rate **[<6.1%]**	**2.1** Abuse in care Rate **[<0.57%]**	**4.1** Percent reunified in <12 months **[>76.2%]**	**4.2** ReEntry <12 months **[<8.6%]**	**5.1** Percent adopted <24 Months **[>32%]**	**6.1** Percent in care <12 months with 2 or fewer placements **[>86.7]**	2002b Total Cases	2002b New Entries	In home cases '02	In home last quarter '02
WOODBURY									
3.7%	0.42%	78.1%	16.6%	65.4%	89.3%	708	443	588	304
IOWA									
11.2%	0.89%	82.3%	26.6%	60.2%	88.4%	8452	3021	14,370	8,423

Race – Ethnicity	American Indian or Alaskan Native	Asian or Pacific Islander	Black	Hispanic	Unknown or unable to determine	White
Woodbury Total Pop	0.3%	0.8%	2.3%	2.1%	0.8%	93.7%
Woodbury Child Pop	2.8%	2.6%	2.9%	5.9%	10.9%	74.9%
Woodbury CW pop	14.7	1.4	6.2	11.6	15.5	50.6

Woodbury County data reflects a greater percent of children in care under age 10 than the state average, and is much more similar to national averages than the other two proposed sites.

	Age 10 and under	Age 11 and over
Iowa Foster Care Population from FFY 2002B	38% (3190)	62% (5262)
Woodbury County from FFY 2002B	49.7% (352)	50.3% (356)

Summary

The following chart demonstrates that the three proposed sites reflect the data outcomes for the state. The exceptions are issues that we would like examined during the on-site review.

Standard	National	Iowa	Polk	Linn	Woodbury
1.1 Reoccurrence	< 6.1 %	11.2 %	9.7 %	**5.8%**	**3.7%**
2.1 Abuse in foster care	< 0.57 %	0.89 %	**0.12 %**	**0.15%**	**0.42%**
4.1 Reunification	> 76.2 %	**82.3 %**	**85.3 %**	**81.7%**	**78.1%**
4.2 Re-entry	< 8.6 %	26.6 %	24.5 %	23.6%	16.6%
5.1 Adoption	> 32 %	**60.2%**	**67.1 %**	26.1%	**65.4%**
6.1 Stability	> 86.7%	**88.4%**	**89.5 %**	**90%**	**89.3**

Note: [**Bold**] in compliance with national standard.

In addition to the outcomes, we believe the three proposed locations are:

- Most appropriate for examining the strengths and concerns noted in the state-wide assessment
- Sufficient to facilitate sampling
- Represent the diversity of Iowa
- Welcome evaluation and are willing to accept the host duties of the onsite review

Comments on Statewide Assessment Process

4. Comments on the statewide assessment process in terms of its usefulness to the State, involvement of the entire review team membership and recommendations for revision.

Quote from Child Welfare Director Mary Nelson: "We embrace this process without reservation. Our job is about outcomes. Are children safe? Are children in foster care moving toward permanent outcomes? Is the well being of children assured by the services we provide? This new approach by the federal government comes at a good time for us. It allows us to brag a little about areas where we excel and it underscores the efforts we have been making in areas where we need improvement."

Quote from Iowa Supreme Court Chief Justice Louis Lavorato: "The Iowa Judicial Branch welcomes the Child and Family Services Review of the state's performance in child abuse and neglect cases, and pledges its full cooperation in the process. This review will help us assess our capacity for protecting the best interests of children in need of assistance, one of the state's most important duties. We regard the federal review as an opportunity to identify and correct any shortcomings, as well as an occasion to promote and build upon our strengths in meeting the permanency needs of abused and neglected children."

Iowa welcomed the opportunity to bring together community stakeholders and staff from all levels of state agencies to have a dialogue focused on safety, permanency and wellbeing outcomes and the systems needed to support these outcomes for Iowa's children and families.

The CFSR statewide assessment process is a useful tool and provides the organization and structure to take a comprehensive and critical look at how families and children are faring in our system. It allows us, though the focus on data, to determine whether we are getting the positive results we want. We recognize the usefulness of the statewide assessment in promoting positive program change.

The statewide assessment process was an opportunity to examine our system and to gather insights about the strengths and needs of the child welfare system. Perhaps most beneficial was the opportunity to gather input from stakeholders in an organized way and to educate them about the data we have. This process definitely helped us develop better partnerships that will serve as a cornerstone for the program improvement plan.

DHS administration and our child welfare partners should be credited with seeing the value in this process and allowing time for staff to participate. This is a time-intensive process and a hardship for our staff and our partners during a time of decreasing resources and increasing demands. Federal funding specifically for the total cost of preparation for the CFSR, state on-site reviewers, and strategies to address program improvement would prevent utilizing funds that would be better directed at the protection and permanency needs of our children and families.

Participants

5. List the names and affiliations of the individuals who participated in the development of the statewide assessment.

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Focus Groups

No.	Group Affiliation	Number of Participants	Date
2	Juvenile Court Judges	6 13	05/31/02 06/19/02
1	Juvenile Court Officers	8	05/31/02
1	Foster Parents	9	06/26/02
1	Community Initiative for Native Children and Families [CINCF], Tribal -State Agreement Committee, Sioux City	7	06/27/02
1	Cedar Rapids Foster Care Review Board [FCRB]	10	08/01/02
1	Iowa Youth Connection Council [foster children ages 16-21]	7	08/10/02
1	DHS Service Area Managers	7	08/13/02
1	Merit Behavioral Corporation & Substance Abuse Providers	12	08/16/02
1	DHS Transition Planning Specialists	9	08/07/02
1	DHS Child Welfare Program Managers	8	08/27/02
1	Child Welfare Consumers [Beyond Welfare]	5	09/05/02
1	Child Protection Oversight Committee	2	09/10/02
1	CASA, Des Moines Service Area	9	09/13/02
1	Polk County Partnership - Decat	11	09/13/02
2	Child Caring and Child Placing Service Providers	7 2	08/28/02 09/15/02
1	Child Advocacy Board [State CASA & FC Review Board]	6	09/20/02
	Total	**138**	

Surveys

Category	Responses	Date
Iowa Department of Human Services: Social Workers and Supervisors Survey	496 [65%]	09/04/02
Juvenile Court Services: Juvenile Court Officers Survey	24	09/10/02
Juvenile Court Judges Survey	13	08/27/02
Service Array Survey	114	06/12/02
Total	**647**	

Structured Interviews

Jessie Rasmussen, Director, Iowa Department of Human Services	12/23/02
Gail Barber, Director, Court Improvement Project	12/18/02
Jerry Foxhoven, Director, Child Advocacy Board [FCRB & CASA]	09/10/02
Sally Titus Cunningham, Deputy Director, Iowa Department of Human Services	10/10/02
Mary Nelson, Division Administrator, Iowa Department of Human Services	09/17/02
Lynhon Stout, Iowa Foster and Adoptive Parents Association	05/23/02

Key Reference Documents

Reference	Author	Date
"A Comprehensive Review of the Child Protective Service System for the State of Iowa Department of Human Services, Final Report, December 7, 2000"	American Humane Association, Children's Services Child Welfare Policy and Practice Group (Project Partner)	12/07/2000
Ombudsman's Child Protection Study Investigative Report 00-0	Iowa Ombudsman's Office	1998 12/14/00
QSR Summary of Findings, QSR 2002 Summary Report QSR Practice Improvement Recommendations	Jane Kieler, from reports prepared by Child Welfare Policy and Practice Group	01/18/01 01/01/02 06/01/02
Iowa Foster Care Review Board Annual Report, Fiscal Year 2001	Iowa Foster Care Review Board	07/01/01
Technical Assistance Summary Report 1996 to 2001	Debra Baker, American Bar Association, Children's Center and the Law	05/21/01

Glossary of Acronyms

AFCARS – Adoption and Foster Care Automated Reporting System
AHA – American Humane Association
AIS – Adoption Information Specialist
ASFA – Adoption and Safe Families Act
BDPS – Behavioral, Developmental and Protective Services
BSW – Bachelor of Social Work
CASA – Court Appointed Special Advocate
CCPPC – Clark Community Partnership for Protecting Children
CDRT – Child Death Review Team
CEU – Continuing Education Unit
CFSR – Child and Family Service Review
CINA – Child in Need of Assistance
CINCF – Community Initiative for Native American Children and Families
CIP – Court Improvement Project
CJCO – Chief Juvenile Court Officers
CJJP – Criminal and Juvenile Justice Planning Division
CPC – Child Protection Council
CPPC – Community Partnership for Protecting Children
CPS – Child Protective Services
CPTA – Child Protective Training Academy
CWIS – Child Welfare Information System
DBDPS – Division of Behavioral, Developmental and Protective Services
DECAT – Decategorization
DHHS – Department of Health and Human Services
DHS – Department of Human Services
DIA – Department of Inspections and Appeals
DPH – Department of Public Health
DV – Domestic Violence
EPSDT – Early Periodic Screening, Diagnosis and Testing
FACS – Family and Children's Services
FIP – Family Investment Program
FOSU – Filed Operations Support Unit
FTDM – Family Team Decision-making Meeting
FY – Fiscal Year
GAL – Guardian Ad Litem
HFI – Healthy Families Iowa
HHS – Health and Human Services
HOPES – Healthy Opportunities for Parents to Experience Success
HSAA – Human Service Area Administrator

IAC – Iowa Administrative Code
IAES – Iowa Adoption Exchange System
ICFCRB – Iowa Citizen Foster Care Review Board
ICN – Iowa Communication Network
ICWA – Indian Child Welfare Act
IDHS – Iowa Department of Human Services
IDPH – Iowa Department of Public Health
IFAPA – Iowa Foster and Adoptive Parents Association
IFMC - Iowa Foundation for Medical Care
IITS – Iowa Interagency Training System
IM – Income Maintenance
ISU – Iowa State University
IT – Information Technology
JCO – Juvenile Court Officer
JCS – Juvenile Court Services
MAPP – Model Approach to Partnerships in Parenting
MDT – Multidisciplinary Teams
MEPA – Multiethnic Placement Act
MSW – Master of Social Work
NCANDS – Neglect and child Abuse National Data System
NCFAS – North Carolina Family Assessment Scale
OJT – On the Job Training
PCA Iowa – Prevent Child Abuse Iowa
PMIC – Psychiatric Medical Institution for Children
QSR – Quality Service Review
RBA – Results Based Accountability
RTS – Rehabilitation Treatment Services
RTSS – Rehabilitation Treatment and Supportive Services
SACWIS – Statewide Automated Child Welfare Information Systems
SAM – Service Area Manager
SDA – Service Delivery Area
SIDS – Sudden Infant Death Syndrome
SIG – State Incentive Grant
STAR – Statewide Tracking Assessment Reports
SW – Social Worker
TANF – Temporary Assistance to Needy Families
TPR – Termination of Parental Rights
U of I – University of Iowa